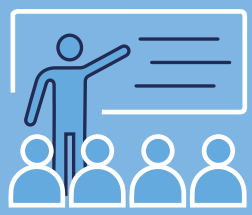
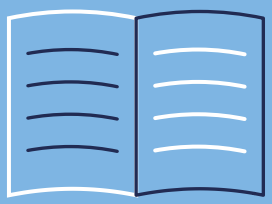
Problem Management Plus (PM+) psychological intervention for individuals

Training manual





World Health Organization

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Problem Management Plus (PM+) psychological intervention for individuals: training manual

ISBN 978-92-4-010992-6 (electronic version)
ISBN 978-92-4-010993-3 (print version)

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Cataloguing-in-Publication (CIP) data. CIP data are available at https://iris.who.int/.

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Acknowledgements

Kenneth Carswell, Georgina Grundy-Campbell, James Underhill (WHO headquarters, Switzerland) coordinated the development and editing of this document, including conceptualisation of the modular format and integration of competency-based assessment into the training programme. Katie Dawson (University of New South Wales, Australia) provided additional content, editing and extensive reviews. The work was supervised by Mark van Ommeren (WHO headquarters, Switzerland).

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This manual is based on several provisional training manuals of Problem Management Plus, primarily written by Katie Dawson (University of New South Wales, Australia). These training manuals include versions developed for WHO and the STRENGTHS Research Consortium. Additional content is based on a provisional version developed by Alison Schafer (World Vision, Kenya). Contributors to these earlier versions include Brian Ogallo, and Edith van't Hof (WHO headquarters, Switzerland), Pernille Hansen, and Martha Bird (International Federation of Red Cross and Red Crescent Societies (IFRC) Reference Centre for Psychosocial Support, Denmark).

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Funding

WHO’s work on the development of this training manual was supported prior to January 2025 by the Bureau for Humanitarian Assistance (BHA), USAID. It was completed using WHO's Assessed Contributions. An earlier version of the materials was developed by the STRENGTHS Research Consortium (led by Vrije Universiteit Amsterdam, Kingdom of the Netherlands), which was funded by the European Commission (Grant Agreement 733337).

Part A.
Introduction, overview and planning tasks

About PM+ training

Overview

Problem Management Plus (PM+) is an evidence-based individual psychological intervention for adults impaired by psychological distress. PM+ was originally designed for communities exposed to adversity (such as chronic poverty, long-term humanitarian emergencies, displacement and similar events and conditions). People in these communities may experience loss of family, friends and livelihoods, and may confront many extreme stressors. People who face such adversities are at a greater risk of developing mental health problems. PM+ is effective in reducing distress indicators – such as symptoms of depression and anxiety – and improving positive mental health outcomes *(1)*.

This training manual is for trainers and supervisors intending to train people to become PM+ helpers. It complements the WHO's *Problem management plus (PM+): individual psychological help for adults impaired by distress and communities exposed to adversity (2)*, referred to in this publication as the PM+ intervention manual which guides trained PM+ helpers as they deliver the intervention (and is also sometimes referred to as the PM+ intervention guide).

Part A. Introduction, overview and planning tasks covers requirements for becoming a PM+ trainer, supervisor or trainee, and outlines how to use the manual and deliver training and competency-based assessments.

Part B. Training modules presents the 12 modules for teaching trainees to be PM+ helpers, broken down into lessons, with step-by-step instructions and cross-references to the PM+ intervention manual.

Part C. Annexes offer checklists, case scenarios and learning activities to accompany the modules.

Approach to development

This training manual was developed to support the training of facilitators delivering PM+. Provisional manuals were developed for use in research trials of the intervention. The final version was developed based on these earlier versions and further adapted to include a modular and competency-based approach.

Note on translation and adaptation of this training manual

WHO has available, on request, a set of materials for translating and adapting this training manual into different languages. To avoid duplication of translations, please consider contacting WHO Press through https://www.who.int/about/policies/publishing/copyright before beginning a new translation.

Planning to provide a psychological intervention and setting up a service

This manual covers the specifics of training people to become helpers who can deliver the PM+ intervention safely and effectively. The manual does not cover the planning process required to establish such a service. Before embarking on training, the implementing organization must have discussed and planned to put into action all the steps shown in Fig. 1.

FIG. 1.

Steps needed to implement psychological interventions

Source: WHO's Psychological interventions implementation manual (Ref. (3))

In addition to following this manual to deliver and adapt PM+ training, other key tasks for the implementing organizations to complete include:

- developing a robust supervision plan specifying frequency and format of supervision to follow immediately after training;
- planning for engaging the community to understand and engage with PM+;
- planning for a functioning and safe referral pathway and processes to support adults and children who may be identified as being at risk (including at risk of suicide, or at risk of harm);
- ensuring that adequate resources are available to support PM+ helpers to deliver PM+ effectively; and
- working with health and social care services, including mental health services, which must be available to support PM+ clients who require more intensive support or who have additional needs.

For more detailed information on the steps required to set up and deliver PM+ and other psychological interventions, see WHO's *Psychological intervention implementation manual (3)*.

Choosing and being a PM+ trainer or supervisor

Your role as a trainer and supervisor is to improve trainees' skills and knowledge so that they can competently deliver PM+ and manage any safety risks (e.g. imminent risk of suicide). There is no formal WHO process or certification to become a PM+ trainer or supervisor. Ultimately, you and your organization must decide whether you are ready to train or supervise. However, it is essential that you are either:

- a person with a background in mental health or psychosocial support (e.g. a mental health professional) who has been trained in, and can demonstrate strong competency in delivering PM+ or the PM+ strategies (e.g. you are trained in cognitive-behaviour therapy [CBT]); or
- a PM+ helper who has had at least 6 months of experience delivering PM+ to clients, can demonstrate strong competency in PM+ delivery and has access to/is supervised by a mental health professional.

In addition, the following qualities are important for delivering effective training. You should:

- be able to manage safety issues (e.g. imminent risk of suicide) according to the procedures established by the organization you work for;
- speak the same language as your trainees or be competent at working with interpreters (see section: Using interpreters);
- be motivated to be a trainer; and
- be able to communicate complex concepts and strategies in simple language, give effective presentations, lead group discussions, demonstrate strategies and skills through role-plays, and give effective feedback to trainees.

Sometimes trainers are also the supervisors of the PM+ helpers they train. If this is not the case, also consider having the future supervisors attend the training. This brings continuity of training, and the supervision sessions will benefit from established trust and rapport.

Becoming a PM+ helper: requirements and expectations of your trainees

Briefly-trained, nonspecialist helpers can deliver PM+. Trainees do not need previous experience of providing mental health interventions. However, they should:

- have strong interpersonal skills;
- have a genuine motivation to help others;
- engage fully with the PM+ competency-based training;
- work in a team; and
- receive continuing support and supervision from a supervisor (ideally someone trained in PM+ or CBT, or someone with a background in mental health and psychosocial support).

Becoming a PM+ helper involves:

Classroom training

- Eighty hours are needed, as detailed in this manual, unless trainees already have expertise and training in providing mental health assistance, in which case the training may potentially be shorter depending on existing competence.
- Trainees must show competency in PM+ skills before proceeding to in-field training.

In-field training

- Trainee PM+ helpers practise delivering the five PM+ sessions to at least two clients with mild symptom presentations (e.g. not with severe depression). You (or the trainee's designated supervisor) should provide close supervision for these sessions (i.e. 1–2 supervision sessions per week).

Ongoing supervision

- Weekly group supervision of at least two hours is a good model. One-to-one supervision may be helpful in different circumstances (e.g. to manage especially difficult help-seekers, initially for newly trained helpers etc.).
- Supervisors should also be available outside of fixed supervision times to respond to suicide or harm risk situations with help-seekers.

Training in other versions of PM+

- Training in one version of PM+ (either Individual PM+ or Group PM+) does not mean you are competent to deliver the other version. To deliver an alternative version, you must complete additional training.
- If you originally trained in Individual PM+ and want to deliver Group PM+, you must complete additional classroom training (approximately four days) to cover group dynamics, additional Group PM+ activities and the Group PM+ case examples.
- If you originally trained in Group PM+ and want to deliver Individual PM+, you must complete additional classroom training (approximately two days) to become familiar with Individual PM+ and learn how to work with individuals.
- In both cases, additional in-field training as described above is then required.

How to deliver effective PM+ training

An outline of the training and planning activities

This training programme consists of 12 modules. The modules should be used for training sequentially because later modules build on earlier ones. However, you can adapt the training for different delivery approaches (see below).

At the beginning of each module there is a summary which outlines the lessons (and proposed timings), the expected learning outcomes and the preparation needed.

Lessons have step-by-step instructions. Key concepts to be communicated are highlighted, and reflective questions are suggested to help your trainees learn. For each module you will need to thoroughly review the corresponding chapter in the PM+ intervention manual and prepare any additional materials. You may wish to reproduce Key concepts boxes as slides or handouts. Other materials that you might need to prepare, such as scenarios and instructions for role-plays, are in the annexes and are cross-referenced in the lessons.

Planning the training

Planning is crucial for effective training. The following sections guide you to prepare to adapt your training to meet the specific needs of your trainees (see section on Adapting the training), work effectively with interpreters (see section on Using interpreters) and integrate a competency-based approach to your training (see section on Planning for competency-based training and assessment in PM+). Other planning tasks include the following:

- **Decide on the class size and trainer to trainee ratio.** It is recommended to use at least two trainers and to limit the class size to a ratio of one trainer to six trainees. This is important for managing your workload, for ensuring that you can give everyone enough support if they need a little extra attention, and for ensuring that you have enough capacity to do competency assessments for all the trainees and to give them individualized feedback.
- **Decide how long the training will take.** The standard timetable (see Annex 2) consists of approximately 80 hours of training over 10 days (including breaks). Consider the unique needs of your trainees and whether this time frame needs to be adjusted (e.g. 20 half days rather than 10 full days).

- **Plan with your co-trainers.** When there is more than one trainer, jointly decide who will lead the training on which modules and what the role of co-trainers will be when they are not leading a module. Prepare and rehearse demonstration role-plays. Decide who is responsible for preparing the pre-prepared flipchart pages, for printing needs such as the case scenarios role-play prompts and PM+ competency assessment tools required for each module.
- **Prepare the training space.** Trainers should consider practical arrangements, such as how your trainees will reach the venue, whether there are enough washroom facilities, how the room/s will be set up, whether you are providing food or drinks, and whether you need additional people to act as clients for Module 11 or others to provide logistical support. A checklist of common set-up tasks is provided in Annex 3.
- **Prepare the trainees.** Plan how you will engage your trainees with the long training process they are undertaking. Experienced trainers report that good engagement is a key element of success. Aim to ensure that trainees fully understand the benefits of all elements of PM+ training and supervision. Full engagement will help to ensure that PM+ is as effective as possible and potentially more rewarding for trainees.

Adapting the training

You can adapt the training, but all PM+ competencies and training information must be covered, keeping true to the original intervention. For example, basic helping skills and all core PM+ strategies must be covered, whether in this training or as part of trainees' previous training. If any section will not be covered, you should still complete competency assessments for the trainees (See Annex 1 for the PM+ competency assessment tool).

You can adapt the scenarios, role-play situations and training activities to reflect the local language, culture and context. In particular, the approach you recommend for supporting clients who report thoughts of suicide may need to be adapted, depending on local procedures and protocols. For instance, you may want to cover services that should be contacted if a help-seeker has a plan to end their life in the near future.

Whenever you choose to adapt the training manual, be certain that the changes will benefit your trainees. Ask yourself, "Why am I making this change?" and "How do I think it will benefit the trainees?" Avoid adaptations only because that is always the way you have trained a particular topic or concept.

Some other possible adaptations

- You might adjust the time spent on a module to meet trainees' needs and existing skills.
- If trainees are highly competent in providing psychological interventions, it may be feasible to shorten the classroom training (although competency assessments are still required).
- You could develop a set of slides to support your training.
- The format of an activity could be changed (e.g. role-plays could be one-on-one rather than in front of a group).

- You might adapt the training approach, using more of some styles (such as presentations, active discussions, role-plays, and group and individual activities) and less of others. Be sure to keep the training approaches sufficiently varied to cater for all types of learners and keep the training active and interesting. Active forms of training, such as role-plays, are recommended as these best support learning.
- You might even consider adapting the training so that it can be delivered remotely. If so, the PM+ remote training manual from The New School and George Washington University may be of use *(4)*.

Using interpreters

Interpretation is a highly demanding job. There are several issues that trainers should consider when working with interpreters. These promote quality interpreting (e.g. accuracy and attention to detail) and interpreter well-being.

- Use more than one interpreter. This allows the interpreters to take breaks and protects their well-being.
- Give interpreters all of the training materials in advance (e.g. the PM+ intervention manual and all accompanying materials and appendices). The interpreters will need to get a sense of the participatory nature of the training and the specific terminology and language used.
- Gently assess what adversity they have been exposed to. Determine whether any material from training may cause them distress and discuss and plan how they will manage this (e.g. avoid interpreting at this time, take a break after this activity, or seek external psychosocial support in some cases). Trainers should offer support to interpreters who feel distressed but trainers are not expected to provide counselling or therapy; this should be sought externally.
- Discuss and jointly decide on the translation of key terms (e.g. mental health symptoms, names of strategies, key terms that describe the rationale of a strategy). Discuss whether multiple dialects are likely to be spoken by helpers and be sure to decide on translation of key terms in all dialects. This process will require back-translation to ensure the accuracy of meanings.
- Practise how to run a role-play, including deciding on how to avoid putting unrealistic expectations on the interpreter, while still ensuring that trainees can learn and receive feedback. Where should the interpreter sit? When should the interpreter talk? If it is a trainee role-play, when will you give feedback without interrupting interpretation?
- During the training, regularly check that interpreters feel supported.
- Make time at the end of each training day to discuss any issues with the interpreter.

Planning for competency-based training and assessment in PM+

The use of competency-based training and assessments in PM+ is designed to help trainees develop the attitudes, knowledge and skills they need to deliver the intervention effectively. This follows the approach of EQUIP (Ensuring Quality in Psychosocial and Mental Health Care).[1] The PM+ competency assessment tool (see Annex 1) lists unhelpful or potentially harmful behaviours and helpful behaviours which you can observe in brief structured role-plays undertaken by trainees during and after training.

Assessments are not used to pass or fail trainees but to monitor progress, identify areas for more practice, and to help you to give the trainees tailored competency-based feedback. Rather than viewing competency as something that can be achieved and assessed once at the end of training, we recognize that a person's competency continually evolves. It is something to be maintained and continuously developed through assessment and support, including through ongoing supervision after training *(6)*.

It is important to plan how you will introduce PM+ competency assessments to trainees. You will need to explain how they work and emphasize that the focus is not on passing or failing. Instead, explain that they help you to understand trainees' strengths and identify areas for improvement. This helps you to give tailored feedback so that trainees can become safe and effective PM+ helpers.

The PM+ competency assessment tool

The PM+ competency assessment tool is used to observe and rate your trainees' competencies in delivering PM+. The PM+ competency assessment tool, used together with the structured role-plays, forms the PM+ competency assessment.

The PM+ competency assessment tool covers seven basic helping skills (referred to as foundational helping skills in EQUIP) that are common across many psychological interventions that are the most relevant to delivering PM+, as well as nine skills specific to delivering PM+. The seven basic helping skills come from EQUIP's Enhancing Assessment of Common Therapeutic factors (ENACT) tool. The PM+ competency assessment tool is for use with both the individual and group version of PM+ and includes two competencies related to delivering Group PM+. These are not included in this manual.

There are some differences between the terms used in the PM+ competency assessment tool and the PM+ Intervention manual, particularly with regard to basic helping skills. For instance, PM+ uses the term "basic helping skills", while the PM+ competency assessment tool which is based on the resources from EQUIP uses "foundational helping skills". This is because EQUIP

1 EQUIP is a WHO and UNICEF project supporting trainers to improve the competence of helpers and the consistency and quality of training and service delivery. See Ref. *(5)*.

takes a more specific approach than was available when PM+ was developed. Despite this difference, all items on the PM+ competency assessment tool are relevant to delivering PM+. For ease of use, each role-play in the training outlines which competencies from the assessment tool are the main focus in that role-play, but many of the foundational helping competencies can be assessed in any role-plays (see Module 3 on Basic helping skills).

The PM+ competency assessment tool is reprinted in Annex 1. It can also be found on the EQUIP website (https://equipcompetency.org) where there is a wide range of other general resources to help trainers and supervisors deliver competency-based training and supervision.

How to use PM+ competency assessments

This manual provides role-plays to cover all PM+ competencies. Most role-plays focus on one or two skills at a time. A few role-plays may cover five or six skills at once (e.g. in Module 3 on Basic helping skills).

Before training (optional)

You can plan to run pre-training competency assessments. These identify trainees' existing competencies and can help you tailor your training. Competency assessments are especially useful if trainees are already trained to deliver other psychological interventions. A pre-training assessment normally covers only competencies in foundational helping skills (items 1–7 on the PM+ competency assessment tool). PM+ specific competencies (items 8–16) would not normally be included. You can assess skills 1–7 through a 10-minute role-play for each trainee (see the EQUIP initiative's *Foundational helping skills training manual* or EQUIP platform for examples of role-plays to assess foundational helping skills: https://equipcompetency.org/en-gb/resources/5051).

During training

At the start of training, prepare a PM+ competency assessment tool for each trainee. Make a paper copy from Annex 1, or use the digital version on the EQUIP platform.

The summary sheets at the beginning of each module indicate the relevant competencies that can be assessed during role plays. During Modules 6–10, the role-plays and corresponding competency assessments focus on skills specific to delivering PM+. Competencies related to basic helping skills (e.g. items 1–6 on the PM+ competency assessment tool) can also be assessed during these role-plays. While some role-plays include step-by-step instructions for competency assessments, any trainee role-play can be used to assess competencies. Generally, the group divides into pairs. One trainee plays a helper, and one a help-seeker (client). Then they swap roles so that everyone gets a chance to be the helper. You will need to ensure that you have enough space, whether in the main training room or separate rooms.

During each role-play, you will move around the room, listening to some trainee pairs and using the PM+ competency assessment tool to assess trainees' competencies. It may not be feasible to observe every trainee but you should aim to observe enough of them to get a sense of common areas where trainees are doing well or may need support.

You may choose to give immediate and specific feedback to individual trainees or role-playing groups; or you may decide to wait and give more general feedback to the whole training group at the end of the exercise. The digital EQUIP platform offers visualizations for analysing and displaying the results. These can provide immediate summaries of how the group is doing.

Aim to assess every trainee on at least a few competencies during the course of training. If you can do more, that will be better for the trainee.

At the end of training

In Module 11, trainees will take part in a full day of skills-building role-plays. Each trainee will have an opportunity to role-play delivering a whole PM+ session. You should plan to observe each trainee for 10–20 minutes, using their PM+ competency assessment tool. At some point during the Module 11 day, take each trainee aside and give them at least 10 minutes of individual feedback based on their PM+ competency assessments and your observations over the course of the training (i.e. all the modules, including Module 11). There are more instructions for Preparing for the extended role-plays in Module 11 below.

During ongoing supervision

Supervisors can also use PM+ role-plays and the competency assessment tool to reinforce trainees' competencies during supervision. This can help in monitoring trainees' progress and tailoring their supervision and practice plans.

You may also use the PM+ competency assessment tool as a supervision tool during live or recorded sessions in which a helper meets real PM+ clients. This can be particularly useful in demonstrating competencies in real-world settings.

Scoring the PM+ competency assessment tool

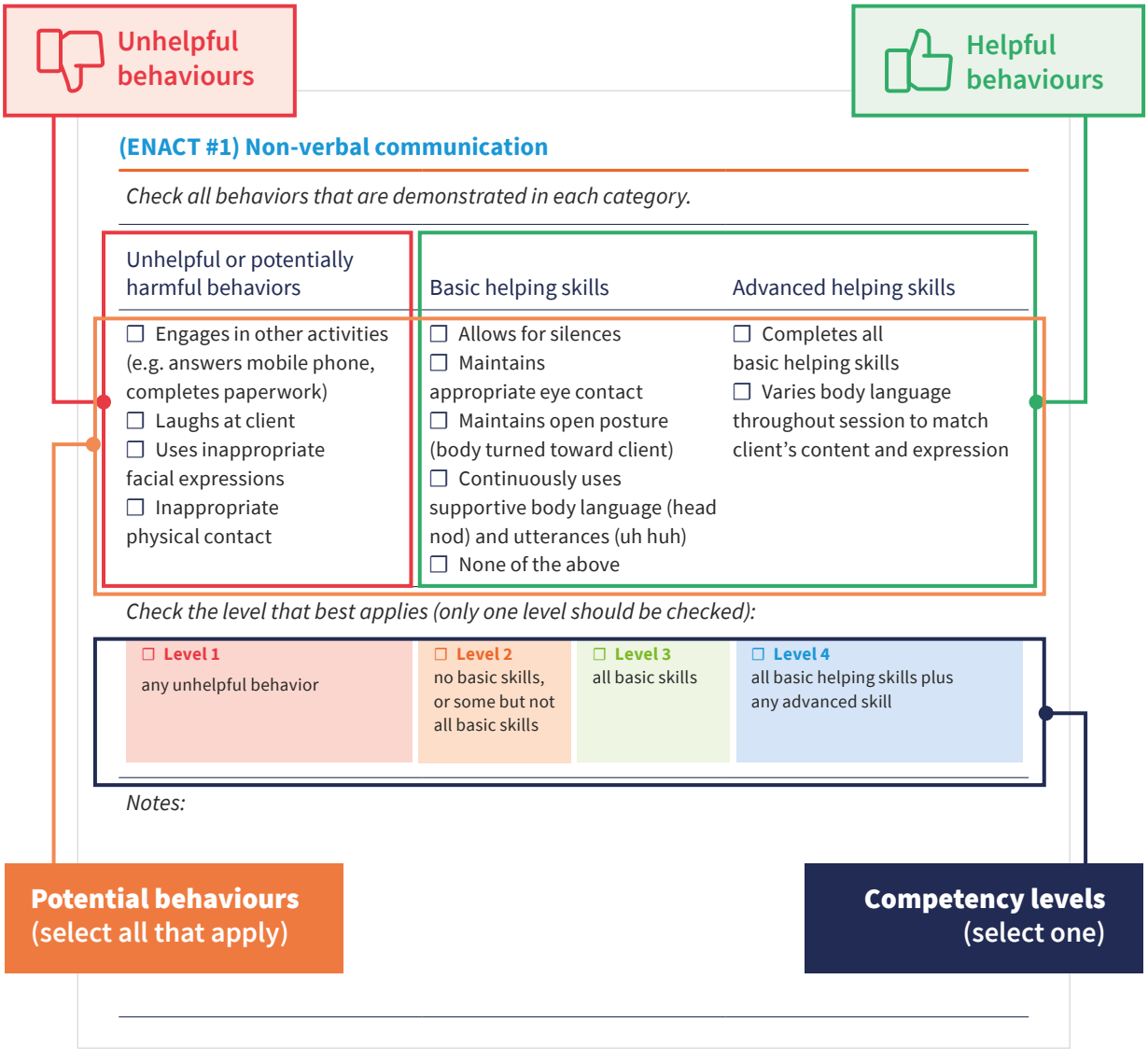
For every skill covered by the PM+ competency assessment tool (e.g. non-verbal communication), there is a checklist of trainee behaviours that are unhelpful or potentially harmful (e.g. doing other things while talking to a client), behaviours that are basic helping skills (e.g. maintaining an open posture), and behaviours that reflect more advanced skills (e.g. varying body language). For each skill, there are typically 2–4 behaviours listed under each of these categories. It is common for trainees initially to display a mix of some unhelpful or potentially harmful and some basic and even some advanced skills.

Once the behaviour checklists are completed, competency can be scored on four levels (Fig. 2.)

- **Level 1.** The trainee shows one or more unhelpful or potentially harmful behaviours. It is particularly important to correct these during training and supervision. Trainees who display any harmful behaviours can score only a Level 1 competency even if they also display basic or advanced helping skills.
- **Level 2.** The trainee shows no harmful behaviours. They may show none or some (but not all) of the basic helping skills. The trainee may or may not show any advanced skills.
- **Level 3.** The trainee shows no harmful behaviours, and all of the basic helping skills. The trainee may or may not show advanced helping skills.
- **Level 4.** The trainee shows no harmful behaviours. The trainee shows all of the basic skills and at least one advanced helping skill.

FIG. 2.

Competency scoring and criteria for each level



Providing feedback

Feedback is central to PM+ competency assessments. To be useful, feedback must provide specific, supportive and personalized information on a trainee's performance or progress so that they can improve and reach their competency goals.

You will need to give feedback to both individuals and groups.

- **Individual feedback.** Focus on what the trainee did well before highlighting anything they could improve on. When discussing areas for improvement, link back to why a particular skill is important and give the trainee concrete suggestions for how to improve, as well as opportunities to practise. Always end feedback positively.
- **Group feedback.** The principles for individual feedback also apply to group feedback, although you should not identify or name specific individuals. Instead, focus on trends observed across the group.

The competency levels are useful for trainers, but are not specific enough to help trainees. For example, rather than saying to a trainee "You are Level 1 on confidentiality", you should say something similar to "You told the client that everything is confidential no matter what they say. There are times, however, when we do not keep all information confidential. What are some of those situations, and how should we explain them to a client?" Similarly, when giving group feedback, it is helpful to point out specifics – e.g. "Many of you showed effective use of open-ended questions to encourage clients to share their experiences. However, many of us are still working on consistently using the skills of paraphrasing and summarizing. Can someone share with the group what paraphrasing and summarizing are?"

In both individual and group feedback, remember to ask trainees for their reflections on the feedback and encourage them to ask questions.

At the end of the training, you can give trainees any completed competency assessment tools. They can then use these assessment tools to develop a training portfolio that they can take with them into supervision to stimulate discussion and further skills-building.

Conducting supervision

In-field training phase: After the classroom training, trainee helpers should be competent, albeit nervous, to start delivering PM+. At this phase, a PM+ helper will deliver the full PM+ intervention to at least two adults with mild presentations (e.g. feelings of low mood and stress). To support helpers' competence and confidence, supervision sessions should happen one or two times a week. One-to-one supervision may be helpful, depending on the skills and confidence of the helper. Supervision lets trainees discuss any questions about PM+, get help with individual cases, and continue to develop their skills.

Regardless of where helpers have reached in their training, they must be able to contact supervisors outside of supervision time to discuss and respond to urgent issues or crises (such as a client at imminent risk of suicide). They may also need some support for their own self-care.

After in-field training: PM+ should be implemented under routine supervision, with weekly sessions of two hours recommended. Supervision can be conducted in small groups (e.g., six people). The frequency of supervision may vary depending on the skill levels of the helpers, which can evolve as they gain experience and confidence. After completing several rounds of PM+ (e.g five clients), supervision frequency may be adjusted from weekly to every two weeks if helpers display competence in all areas. Regardless of experience, routine supervision should always be maintained. Supervisors may also decide to increase the frequency of supervision in some situations (e.g. if helpers begin delivering PM+ to more challenging help-seekers, such as a trauma-exposed population, the helpers may demonstrate loss in confidence or competence).

During supervision, you should ask about clients' progress, and any challenges your trainee helpers are experiencing in their sessions, or more generally, with their clients (such as attendance problems). Active methods, such as role-play, are encouraged when identifying possible solutions to problems – e.g. two helpers can role-play a suggested solution to managing a client who continues to arrive late to sessions. This will increase participation and engagement of all helpers and allows helpers to try out ideas before implementing them with clients. You can also use supervision sessions to continue training. You might use this manual to plan refresher training activities, or you might develop a supervision plan based on a trainee helper's scoring in the competency assessments during their classroom training. Also allow time in supervision to assess helpers' well-being and support them with their own self-care.

You can continue to use the competency assessments with competency-based feedback to support supervision (see the full list of PM+ competencies in Annex 1 or see the EQUIP platform). Supervision is critical to the delivery of psychological interventions. More information on supervision can be found in WHO's *Psychological intervention implementation manual (3)*.

General instructions for delivering the modules

The training modules are based on learning by doing – also called experiential learning. Trainees will use experiences and reflection to connect the training information to real-world situations. Remember to ensure that trainees fully understand the benefits of engaging in all elements of training and supervision. Full engagement helps make PM+ training effective and rewarding for trainees.

To support engagement, vary the flow by introducing a role-play, activity or discussion. However, you can adapt the time spent on activities to your group. For instance, you may have many or a few trainees, talkative or quiet students, quick learners or people who want to practise etc. Remember to use simple words that are easy to understand and not complicated psychological terms that trainees may not know.

PM+ training and experiential learning activities

Reviews

At the beginning of each day the group will review information learned in previous modules. After the first few review sessions, ask trainees to take turns at facilitating these reviews in pairs. This will help ensure that trainees engage with the material. Trainees should use interactive methods to facilitate the review – such as group discussions, quizzes or role-playing-activities. At the end of each day, choose who will facilitate the review and encourage them to ask you questions as they prepare. The review instructions may need to be adapted depending on the timetable. For example, if a module is split across two days.

Home practice

Trainers will lead reviews of any home practice. Setting and encouraging home practice is a key part of PM+. Use the home practice tasks at the end of each module to model how trainees can set home practice tasks with their clients. Home practice can be used for many purposes in the training, including reading/practising parts of the PM+ intervention manual and bringing those experiences to the training sessions. Home practice instructions may need to be adapted depending on the timetable. For example, if a module is split across two days.

Feedback

During training and supervision, trainees will give each other feedback on delivering PM+. This can be used to model the giving of effective and positive feedback to PM+ clients, which is another key part of the intervention.

Role-plays

There are two types of role-play:

Demonstration role-plays

- You and your co-trainer play the roles of PM+ helper and client (unless instructed in the module to identify a volunteer from the trainees). You are demonstrating how to deliver a strategy or technique. Practise these role-plays before you deliver them as part of the training.
- Before the role-play starts, ask the trainees some reflective questions to keep them engaged and help them to reflect on what they will see. Consider using so-called remote control techniques, whereby trainees can signal to request a pause in the demonstration and can ask questions.
- Sometimes it can be helpful to demonstrate a role-play twice to show the differences between poor use of skills and common helper errors (e.g. giving advice to the client) and good use of skills. This can help trainees to learn what to avoid when delivering a strategy. Such role-plays are outlined in the training.
- At the end of the role-play, allow time for reflection and discussion and raise important points if trainees do not mention them.

- Use questions that prompt thought and reflection. For instance, "What did you notice the helper do when the client said XXX?" or "What is another response you would give to this client when they said XXX?" You can use direct reflective questions if trainees do not notice important points, e.g. "Did you notice that the helper asked a question when the client said XXX."

Trainee role-plays

- Role-plays allow trainees to play the roles of helpers, clients and, at times, observers. They are opportunities to practise providing PM+ sessions and strategies. For role-plays representing specific parts of the PM+ intervention, the trainees will always find scripts to use as examples in the PM+ intervention manual.
- When a role-play teaches a specific PM+ competency you will be directed to "Module Role-plays (#)" at the end of the specific modules to use the scenarios and prompts described.
- To save time, decide ahead of time how you will divide trainees into pairs or groups for role-plays and where they will complete their role-plays. Make sure there is adequate space, so role-playing pairs/groups do not disturb each other. Be mindful to keep pairs/groups within range so you can easily go around to all groups to observe them. Be mindful of who you pair together (different personalities, confidence levels and different organizations).
- When starting each role-play, encourage trainees playing the role of the client to imagine they are experiencing the situation and reactions so that they can make the role-play more realistic.
- Both you and the trainees can adapt the scenarios, perhaps using personal experiences of supporting or knowing people living with mental health conditions.
- You should allow the role-play to last approximately 15–20 minutes with at least 15 minutes for reflection, although suggested timings may vary.
- You should move around the room during role-plays, observing trainees and giving immediate feedback if appropriate.
- Use the PM+ competency assessment tool (Annex 1) to help structure your observations and to help you give systematic and detailed feedback to trainees during role-plays or as a group afterwards.

Group discussions

Discussions are important experiential learning tools. You will need to engage all the trainees and their varying opinions and ideas. An engaged trainee is listening, sometimes responding or taking notes or reflecting on ideas. Not all trainees will immediately want to contribute to discussions, as people learn at different speeds and have different preferences. But you can encourage discussion by using the suggested prompt questions highlighted throughout the modules.

Key concepts

Text in *Key concepts* boxes covers ideas you need to share with the group. You can provide these verbally, use a flipchart or create a slide. If you share a set of slides with trainees, remind them that all of the information can also be found in the PM+ intervention manual.

Energizers

Well-chosen energizer activities (3–10 minutes long) can help increase the energy of the group after a break or after covering a difficult topic. Annex 4 has a list of example energizers. Choose activities that you know are appropriate and that you think your trainees will enjoy.

Preparing for the extended role-plays in Module 11

Module 11 is a skills-building session and needs special preparation. It is important to do this before the end of Module 10 (Lesson 4), because that is when you will assign the trainees their roles and ask them to prepare.

There are two parts to Module 11:

- Extended role-plays: the trainees run PM+ sessions 1–3 in groups of two or three. You will need to run several role-plays simultaneously. For example, if there are 12 trainees, there will be four role-plays taking place simultaneously. Within each group, one person will role-play the helper, one will role-play the client and the other will observe.
- To give individual private feedback, plan so that each trainee gets at least 10 minutes of personalized feedback on their overall progress during the training, either from you or from a co-trainer. You may need to take a trainee out of a session that they are observing, or perhaps you could schedule individual meetings during breaks.

When allocating the trainees to groups, mark in your timetable in which session they will role-play the helper. Give some thought to who will work well together, and where you still need to give individual trainees an opportunity to demonstrate their skills. For example, should trainees who work together be in the same or separate groups? Use your observations of the trainees' strengths and weaknesses during the earlier modules to decide.

Use the PM+ competency assessment tool during the skills-building day to support the feedback process. It may not be possible to assess trainees against all of the competencies on the assessment tool during the skills-building day. The idea is that the role-plays observed throughout the training and during this skills-building day should help to build up a good picture of your trainees' strengths and areas for improvement that can continue to be supported during the supervision process.

Instead of using trainees to role-play PM+ clients, you might choose to use independent persons (such as other colleagues who have some experience in working with psychosocial and mental health support, or perhaps local psychology students, or people from the local community).

The advantage of using independent persons to role-play clients is that they are unfamiliar with PM+ and will tend to respond more like an actual client might. For example, they might have difficulties understanding or applying the strategies, and they will not know what is going to happen next in the session. Using independent persons gives you the option to have smaller groups and perhaps to run more simultaneous role-plays. However, bear in mind that you need to plan to spend 10–20 minutes observing and rating each trainee.

If you are using independent persons, plan a meeting where you spend time letting them know what they can expect from the skills-building day and from the trainees. Describe their roles, give them case scenarios from any module role-plays as back-stories and scenarios. Take time to answer any of their questions and concerns and collaboratively decide how to offer support if anyone becomes distressed by the content.

You might also plan to train extra assessors to do competency assessments for a large group of trainees. If you do this, you will need to allocate trainees to your co-trainers before the module begins.

Whatever route you choose, you will need to schedule the day. See Table 1 for an example timetable. Each of the three role-play sessions should last 90 minutes, and should be followed by 15 minutes for the group to reflect, discuss and give each other feedback.

TABLE 1

An example timetable for Module 11 extended role-plays (7 hours 40 minutes, including breaks)

Time	Topic	Trainee names
09:00-09:45 (45 minutes)	**Introduce how and why competency-based assessments will be used**	
09:45–11:15 (90 minutes)	Session 1 role-play (PM+ intervention manual, Appendix G, PM+ intervention protocol	Helper: Client:
11:15–11:30 (15 minutes)	Feedback on Session 1 role-play	Observer:
11:30–11:45	**Break**	
11:45–13:15 (90 minutes)	Session 2 role-play (PM+ intervention manual, Appendix G, PM+ intervention protocol	Helper: Client:
13:15–13:30 (15 minutes)	Feedback on Session 2 role-play	Observer:

Time	Topic	Trainee names
13:30–14:15	**Lunch**	
14:15–15:45 (90 minutes)	Session 3 role-play (PM+ intervention manual, Appendix G, PM+ intervention protocol	Helper: Client:
15:45–16:00 (15 minutes)	Feedback Session 3 role-play	Observer:
16:00–16:40 (40 minutes)	**Trainees reflect on the skills-building day, trainers give group feedback**	

Part B.
PM+ training modules

Module 1

Opening the training 1h40min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Welcome to the training (35 minutes)			
1.1	**Welcome (5 minutes)**	Trainees get to know each other	Ice-breaker (from Annex 4)
1.2	**Introductions (30 minutes)**		Familiarise yourself with the venue
Lesson 2. Setting up the training group (25 minutes)			
2.1	**Establish group rules and guidance on giving helpful feedback (25 minutes)**	Trainees know what is expected within the group, agree to the rules and understand how to give helpful feedback	Blank flipchart paper and pens Print guidance on giving feedback (Annex 5) for sharing
Lesson 3. An overview of the training (40 minutes)			
3.1	**PM+ training structure (20 minutes)**	Trainees know how the training will run	One copy of the PM+ intervention manual per trainee. Prepared flipchart with training schedule Print blank copies of the PM+ competency assessment tool (Annex 1)
3.2	**Training expectations (20 minutes)**	Trainees have examined their training expectations	Prepared dartboard flipchart activity

Lesson 1

Welcome to the training 35min

Prepare in advance

You may wish to choose an ice-breaker activity from Annex 4.

1.1 Welcome (5 minutes)

Tell trainees

- Welcome your trainees and introduce yourself.
- Acknowledge the effort trainees made to attend the training (e.g. taking time away from other work commitments).
- By attending this training, they are making an important commitment towards improving the emotional well-being of their community.
- Cover important practical information such as breaks, toilets, food, etc. Resolve any initial difficulties.

1.2 Introductions (30 minutes)

Learning outcome: your trainees feel at ease.

Organize trainees

- Trainees are going to take part in an introductory activity.
- Ask everyone to find a partner they don't know well.

Tell trainees

- Introduce themselves to each other and find at **least three things** they have in common.
- After three minutes, each pair will introduce each other to the group and **share one of the things they had in common**. Ask trainees to try not to repeat what others have said (e.g. "We want to learn more about PM+").
- Emphasize that everyone in the room brings their own expertise, they will be learning from each other during the training and everyone's skills and experiences are equally valued and important.

Lesson 2

Setting up the training group 25min

Prepare in advance

Have flipchart and pens ready, and print copies of Annex 5 for sharing.

2.1 Establish group rules and guidance on giving helpful feedback (25 minutes)

Learning outcome: your trainees know what is expected within the group, they agree to the rules and understand the steps to giving helpful feedback.

Tell trainees

- It is important to create a safe learning environment.
- Discuss and decide on rules that everyone in the group should adhere to.
- Write the rules on a large piece of paper and keep it displayed throughout the training. Make sure you include the rules in the *Key concepts* box.

KEY CONCEPTS

Some standard group rules

- We keep things disclosed in the training confidential.
- We give everyone an opportunity to talk.
- We commit to being active trainees (including in role-plays).
- We can ask questions – there are no silly ones!
- We can ask trainers to repeat anything that is not understood.
- We will not interrupt each other.
- We are encouraging and thoughtful when providing feedback to each other.
- We are open to making mistakes or being corrected by others.
- We will arrive and finish on time.
- Phones are turned off, or put to silent, but urgent calls can be accepted.
- We aim to learn and have fun!

- Explain that during the training there will be many opportunities for trainees to observe and give feedback on role-plays and demonstrations of PM+ helper skills.
- Distribute and read aloud *Steps to giving helpful feedback* (Annex 5). Be clear that trainees should follow these steps when they give feedback to their colleagues.

Lesson 3

An overview of the training 40min

Prepare in advance

You may want to prepare the dartboard activity for Lesson 3.2 on a large sheet of paper ahead of time.

3.1 PM+ training structure (20 minutes)

Learning outcome: trainees know how the training will run.

Tell trainees (5 minutes)

- Display or share copies of the PM+ training schedule with trainees. Cover the three main stages described in this *Key concepts* box.

KEY CONCEPTS

PM+ training starts with 80 hours of classroom training. Classroom training covers core elements of the PM+ intervention and the skills needed to deliver it. There will be teaching, discussions, group and individual activities and role-plays. The training will review basic helping skills, teach the PM+ intervention, and will include a day of skills-building (Module 11). The skills-building day is an opportunity to practise delivering PM+.

In-field training. Trainees deliver PM+ to at least two real clients with mild symptoms of distress, but stay under close supervision. These sessions take place once or twice a week. They consolidate learning and provide opportunities to practice responding to and managing varied problems and presentations.

Supervision continues even when trainees are trained. Trained helpers still see supervisors who help them focus on more complex and nuanced challenges. Supervisors are also there to support the helper's own emotional well-being.

- Explain that this PM+ training is competency-based and uses the EQUIP PM+ competency assessment resources of WHO and UNICEF.
- Share printed copies of the PM+ competency assessment tool (Annex 1) with trainees. Make sure you provide a copy in a language in which the trainees are fluent.
- Tell trainees that the competency-based assessments are not being used to pass or fail them. Rather, the assessments are used to highlight which elements of the PM+ intervention they are more comfortable delivering (**areas of strength**) and also areas where more practice is needed (**areas for improvement**).

- The competency-based assessments will be used during training role-plays throughout the different modules.
- In Module 11 there will be more extensive role-plays in small groups where trainees will practise delivering different sessions and trainers will give everyone at least 10 minutes of individual feedback.
- Explain to trainees that everything they need to run PM+ is contained in the manual, you will refer to this throughout the training.

Facilitate a discussion (15 minutes)

- Ask for and answer any concerns the trainees may have about the training process.
- Ensure that trainees understand the competency assessment and have not misunderstood their purpose.
- Encourage them to view ongoing assessment as a way to learn about their own strengths and weaknesses, and to refine their skills when needed.
- Expect helpers to be nervous about these assessments. Conclude the discussion by emphasizing that the training will be adequate preparation for the Module 11 assessments and that the purpose is to support trainees to deliver PM+ safely and effectively.

3.2 Training expectations (20 minutes)

Learning outcome: trainees have examined their training expectations.

Tell trainees (5 minutes)

- This "dartboard" activity introduces the core aims and expectations of the PM+ training course and explores their expectations.
- Present and explain the large sheet of paper with a large circle divided into six equal sections. In each section, explain what the underlined statements mean.

<u>I know PM+</u> (I am familiar with PM+ and its strategies).

<u>I can do PM+</u> (I am confident in delivering PM+ strategies to others).

<u>PM+ clients</u> (I know who is and is not suitable for PM+).

<u>PM+ helper</u> (I understand the role of a PM+ helper).

<u>Helping skills</u> (I feel I have the skills to help people living with mental health problems).

<u>Following the manual</u> (I feel confident to be able to provide mental health support by following a manual).

Organize the trainees (5 minutes)

- Ask the trainees to reflect on their thinking and to mark on the poster where they feel they are in relation to each statement.

- Closer towards the centre means they feel increasingly confident. Towards the edge means they feel less confident.

Facilitate a discussion (10 minutes)

- Comment on where most of the marks have been placed. Try to make this reflection positive. For example, say something like: “This is only the beginning of the training so of course you are not feeling very confident about PM+ and how to deliver it. This is good as it means you will be eager to learn, and I can work with you to build this confidence.”
- Ask for the opinions and reflections of the trainees.
- Remind trainees that no one is expected to be confident at this stage – we are here for training! Tell them the activity will be repeated at the end of the training so they can compare how they feel then with how they feel now.

Module 2

Background information and the PM+ intervention 3h10min

No.	Description	Learning outcome	Advance preparation
1. Review and introduction (15 minutes)			
1.1	**Review (10 minutes)**	Trainees will understand how reviews will be used during the training	For ideas to review learning and understanding, see Annex 4 (Additional resources for trainers)
1.2	**Module introduction (5 minutes)**	Trainees know learning objectives/skills to be achieved by the end of the module	
2. Understanding common mental health problems (45 minutes)			
2.1	**Group discussion on identifying common mental health problems (45 minutes)**	Trainees can name and describe common mental health problems	Prepare three flipchart pages with definitions of the common mental health problems
3. Present the PM+ intervention using the PM+ poster (40 minutes)			
3.1	**Overview of PM+ (20 minutes)**	Trainees have an overview of the PM+ intervention	Ensure that a copy of the PM+ structure from **Chapter 2 of the PM+ intervention manual** is displayed prominently as a flipchart (or slide)

No.	Description	Learning outcome	Advance preparation
3.2	**Present the core PM+ strategies (20 minutes)**	Trainees can name the core PM+ strategies	
4. Delivering the PM+ intervention (65 minutes)			
4.1	**Who PM+ is for, and who it is not for (20 minutes)**	Trainees can identify and describe who can participate in PM+ and show that they know who is not suitable for PM+	
4.2	**Group discussion. The role of PM + helpers (30 minutes)**	Trainees can describe the PM+ helper's role, and can recognize and name the characteristics of an effective helper	
4.3	**Including family and friends (15 minutes)**	Trainees understand when and how family and friends should be involved in PM+ interventions	
5. Summary, home practice and setting up the review (25 minutes)			
5.1	**Summary (10 minutes)**		
5.2	**Setting home practice. Considering cultural and local adaptations to the PM+ manual (10 minutes)**	Trainees recognize and demonstrate the adaptation of PM+ to the local and cultural context	

Lesson 1

Review and introduction 15min

Prepare in advance

For ideas to review learning and understanding, see Annex 4 (Additional resources for trainers).

1.1 Review (10 minutes)

Learning outcome: trainees understand how reviews will be used during the training.

Tell trainees

- At the beginning of each day, a brief review of the content from the previous day will be delivered.
- On the first occasion, the trainer will conduct this review session.
- In later modules, you and the trainees in pairs will run these review sessions.
- The trainees will have up to 10 minutes to lead an activity, game or discussion of their choosing.
- The review must include key information from the previous day's training. They do not need to review all the content.
- The trainees should be sure to be engaged in training so they can prepare for delivering a review session to their peers.
- After the review, the trainer will allow time for questions, will clarify any necessary concerns and will then lead a review of the trainees' home practice.

1.2 Module introduction (5 minutes)

Learning outcomes: trainees know the learning objectives/skills to be achieved by the end of the module.

Tell trainees

- This module will give them background information on why PM+ was developed and how it is used with people with common mental health problems. Tell them that by the end of the module they will be able to:
    - name and describe common mental health problems;
    - describe the PM+ intervention and name the four core PM+ strategies;
    - identify who can participate in PM+; and
    - reflect on and describe the role of the PM+ helpers and their relationship with the PM+ client.

Lesson 2

Understanding common mental health problems 45min

Prepare in advance

Before this lesson begins, prepare four flipchart pages. On three of them, write one of the following statements (leaving room on the page to add the group's comments):

(Page 1) Depression. Depression is when someone feels very sad or low for long periods. This could last for weeks, months or even years. People can feel this way without any particular reason. They often stop doing things they used to enjoy (e.g. socializing with others, hobbies) or find it hard to do the things they need to do (e.g. chores or work).

(Page 2) Anxiety. Anxiety is feeling very worried most or all of the time, without necessarily having any particular reason. It can make it very hard for people to cope with daily life. Anxiety can cause strong physical sensations such as tension, headaches and problems with breathing.

(Page 3) Stress. Feeling stressed is often described as feeling overloaded and under-resourced. People can feel wound up tight, tense or worried. Someone very stressed might experience physical discomfort (e.g. tension, headaches, problems with breathing) and may find it hard to perform their normal activities.

(**Page 4)** On the fourth page, draw three columns labelled as Depression, Anxiety, Stress. In the columns, write symptoms from Table 2 and as suggested by trainees during the exercise.

Potential adaptation for people with experience. If trainees have extensive prior mental health experience you may be able to adapt this activity. For instance, you could get trainees to discuss cultural expressions of depression, anxiety and stress and how they relate to PM+.

2.1 Group discussion on identifying common mental health problems (45 minutes)

Learning outcome: trainees can name and describe the common mental health problems.

Tell trainees

- PM+ aims to help people who are experiencing common mental health problems and practical difficulties.

Facilitate a discussion (45 minutes)

- Present the blank pre-prepared fourth flipchart page with three empty columns entitled depression, anxiety and stress.
- Use the following prompts to ask the trainees for their understanding of these common mental health problems.
  - Without saying names, raise your hands if you or someone you know has experienced emotional problems to the point where that person may have been experiencing a mental health problem.
  - How did they behave? How did they present? How did they cope?
- Write down the signs and symptoms that the trainees describe on (blank) flipchart paper.
- If they struggle, draw from Table 2 and/or you could ask:

  "At times we can all feel a little depressed [or anxious or stressed]. Think of a time when you felt that way. What did you notice about your behaviour, your body, etc.?"
- Use the discussion generated to explain that people of all ages, genders, abilities, backgrounds and experiences can live with common mental health problems.
- Now show the trainees the signs and symptoms in Table 2: using the pre-prepared flipchart.
- Highlight the fact that depression, anxiety and stress can share symptoms.
- Explain that PM+ simply tries to support people experiencing emotional distress that interferes with life (also called "common mental health problems or symptoms"), and that helpers do not need to be able to distinguish the problems or symptoms or diagnose mental disorders.
- Finish the discussion by saying that it is normal for everyone to feel depressed, anxious or stressed at certain times. Usually, such feelings pass with time or when circumstances change. However, when people experience these problems almost daily for weeks at a time, and the feelings interfere with their lives, they need some assistance. PM+ is designed to help these people to feel better again.

TABLE 2

Symptoms of depression, anxiety and stress

Depression	Anxiety	Stress
• Low mood, feeling sad most of the time • Lethargy, lack of energy • Feeling tired all the time • Inactive, no longer doing activities or tasks they used to do, not getting pleasure out of activities they enjoyed previously • Feeling worthless or guilty • Irritable • Low self-esteem, self-critical (e.g. feels like a failure) • Poor concentration, unable to make decisions easily • Very slow movements or slow to do things • Loss of appetite or eating more than usual • Suicidal thoughts or attempts	• Worrying a lot (without reason) • Feeling panicked • Physical symptoms not explained by medical conditions can include headaches, stomach aches, body aches, breathing difficulties (rapid, shallow breathing), chest pain, hot flushes • Restlessness (difficulty sitting still) • Avoiding situations due to fear, such as activities, going to places or seeing people, even if those situations are reasonably safe	• Physical symptoms not explained by medical conditions can include tension, frequent headaches, neck or back ache, sweating, dry mouth, light headedness • Unable to relax • Irritable • Easily startled (to sudden noises or movements) • Poor concentration • Over-reacting • Feelings of anxiety and/or depression

Lesson 3

Present the PM+ intervention using the PM+ poster 40min

Prepare in advance

Ensure that a copy of the PM+ structure, from Chapter 2 of the PM+ intervention manual, is displayed prominently as a flipchart (or slide).

3.1 Overview of PM+ (20 minutes)

Learning outcome: trainees have an overview of the PM+ intervention.

Tell trainees (5 minutes)

- Explain that common mental health problems are prevalent in all communities but there are not enough mental health professionals to support the number of people in need.
- PM+ was created so that nonspecialist helpers can deliver evidence-based psychological treatment. If needed, you can briefly give some background:
  - PM+ was first tested with individuals experiencing impairing emotional distress and severe adversity in Pakistan (men and women) *(7)* and Kenya (women) *(8)*, and with groups in the Swat Valley (women) *(9)* and Nepal (men and women) *(10)*, with positive results.
  - Since these original studies, PM+ has been used widely in many places *(11)*.

KEY CONCEPTS

Problem Management Plus (PM+) is a brief intervention, which combines widely used strategies of problem management, stress management, behavioural activation and social support.

It aims to build a client's ability to manage their own emotional distress and practical problems.

PM+ consists of:

- Pre-intervention, during-intervention and post-intervention assessments;
- five weekly treatment sessions, each lasting 90 minutes;
- helpers teaching the client four strategies;
- treatment sessions build on the client's skills to better manage their practical problems;
- ongoing supervision of PM+ helpers.

3.2 Present the core PM+ strategies (20 minutes)

Learning outcome: trainees can name the core PM+ strategies.

Organize trainees (5 minutes)

- Tell trainees to find the definition and aims of PM+ in **Chapter 2 of the PM+ intervention manual.**
- Ask them to read and reflect on the text describing the four core strategies that make up PM+.

Facilitate a discussion (15 minutes)

- Ask for trainees' reflections and questions about the core strategies.
- Explain that throughout this training the trainees will learn and practise how to deliver these core strategies.
- Cover the information in the *Key concepts* box for your trainees.
- Explain that the trainees should become as comfortable as possible using the PM+ intervention manual. Review the manual showing how to find the important parts such as scripts and other materials for sessions.
- This is important because all the information needed to deliver PM+ is in the PM+ intervention manual. If trainees include things that are too different from what is in that manual or if they ignore parts of it, then they are delivering something different from PM+.

KEY CONCEPTS

The four core strategies of PM+

1. **Managing stress.** This helps people to reduce and manage their stress and symptoms of physical tension and panic. It is introduced in the first PM+ treatment session.
2. **Managing problems.** This strategy helps people to manage their practical problems, such as interpersonal conflict, or financial problems. It is introduced in the second PM+ treatment session.
3. **Get going, Keep doing.** This helps people to reduce symptoms of depression and low mood by gradually increasing their activity. It is introduced in the third PM+ treatment session.
4. **Strengthening social support.** This helps people to seek out and strengthen their social supports because these can help people manage their problems and improve overall emotional well-being. The strategy is introduced in the fourth PM+ treatment session.

Education about adversity and PM+ (covered in Session 1) and staying well (in Session 5) are important components of the intervention but are not core strategies. They can be likened to the beginning and end of a story.

- Point out key sections such as the chapters, PM+ protocol and client handouts and give a brief explanation of how these are used to deliver PM+.

Lesson 4

Delivering the PM + intervention 65min

Prepare in advance

Trainees will need flipcharts for their group discussion.

4.1 Who PM+ is for, and who it is not for (20 minutes)

Learning objective: trainees can identify and describe who can participate in PM+ and they know who is not suitable for PM+.

Tell trainees (5 minutes)

- PM+ is designed to help adults who are experiencing emotional distress interfering with their lives (e.g. depression, anxiety or stress) and may be living in communities affected by adversity.
- PM+ is not suitable for the following:
  - anyone who has a plan and/or intention to end his/her life in the near future (someone at imminent risk of suicide) – such persons are likely to require other treatment that helps them stay safe;
  - anyone who has a severe impairment related to a neurological disorder (e.g. an intellectual impairment, dementia, etc.) – such persons will probably not be able to follow the structure of PM+; they would benefit from treatment that allows for more flexibility, and a helper who can tailor treatment to the problems they have;
  - anyone who is experiencing symptoms of a severe mental disorder (e.g. active psychosis, or severe and uncontrolled substance dependence) – such persons need a programme that can be tailored to their mental health problems (how well these persons function can change over time, and a more flexible treatment would be better for them).
- People who are not suitable for PM+ should be referred to services that can provide more tailored treatment. In many cases this may be to a psychiatrist or psychologist, if available.
- If a helper meets a client who has an acute difficulty and/or faces an immediate risk (e.g. a young woman who is at acute risk of being assaulted), helpers should respond initially with psychological first aid and then with referrals to people better able to help. If appropriate, once the acute protection risks have settled, such clients may also receive PM+. Other resources provide more information on dealing with intimate partner or sexual violence *(12, 13, 14)*.

- Point out where in **Chapter 1 of the PM+ Intervention Manual** trainees can find this information.

Facilitate a discussion (15 minutes)

- Use the following prompts to elicit trainees' opinions and to gauge their level of understanding at this point:
  - From what you have learned about PM+ so far, why do you think PM+ strategies are suitable only for people living with stress, depression and anxiety?
  - How do you think the PM+ intervention may help people? Why do you think it might not help other people?
- Finish the discussion by saying that you will now go on to explore the role of the PM+ helper.

4.2 Group discussion: The role of PM+ helpers (30 minutes)

Learning objective: trainees can describe the PM+ helper's role and recognize and name the characteristics of an effective helper.

Tell trainees (5 minutes)

- The next activity will allow them to think about the role of the PM+ helper and how they may feel in that role.

Organize trainees (3 minutes)

- Divide the trainees into two groups. Give each group some flipchart paper and pens.
- **Give group 1** the question "What are the important characteristics of an effective PM+ helper?" Ask members of group 1 to consider how they might behave, what to say or not say, and to think about their attitudes towards clients.
- **Give group 2** the question "What are the helper's roles and responsibilities?". Ask members of the group to consider the roles and responsibilities of being a PM+ helper.
- Allow five minutes for the groups to list their responses. Then hang the posters on the wall and allow each group five minutes to present their lists.

Facilitate a discussion (20 minutes)

- Encourage trainees to contribute ideas to the other group's list. Make sure that the discussions include all the ideas in the *Key concepts* box.

KEY CONCEPTS

A PM+ helper's role

Effective characteristics are:

- be caring and nonjudgemental;
- have good communication skills (speak clearly and use simple language);
- talk in a way that engages the client and earns their respect;
- be passionate about your work.

Roles and responsibilities are:

- closely follow the PM+ manual;
- teach clients the PM+ strategies;
- support and encourage clients as they implement PM+ strategies in their lives;
- attend supervision meetings regularly;
- monitor PM+ clients' welfare and ensure their rights are respected at all times;
- take care of your own emotional well-being (through self-care).

- Highlight characteristics and roles that may not be appropriate for helpers to demonstrate and give reasons why. Examples of inappropriate characteristics are being **highly critical** or **judgemental** towards clients. Examples of inappropriate roles include **delivering strategies that are not part of PM+** or **giving clients advice** (e.g. on how to manage a problem).

Tell trainees (2 minutes)

- Tell the trainees to reflect on the skills and characteristics of an effective PM+ helper. Identify areas where they might struggle, or which they may wish to address.
- Let trainees know that they can come and speak to you or to your co-trainers to discuss this at any time during the training.

4.3 Including family and friends (15 minutes)

Learning objectives: trainees understand when and how family and friends should be involved in PM+ interventions.

Tell trainees

- Go over the following important points on when and how to include a client's family or friends in PM+.
- Emphasize that it is important that PM+ helpers make sure their client understands all of these points:
    - Sometimes it can be useful to involve a family member or friend in PM+ (also called a trusted other).

- Clients must give permission or ask for the trusted other to be involved during the intervention.
- The client can decide at any time that they no longer want the trusted other to be involved.
- The trusted other will learn the PM+ strategies, so they feel better equipped to support the client with any future problem or distress. However, we would not expect this other person to act as the client's PM+ helper.
- Not all of the five PM+ sessions are suitable for involving a trusted other. For example, additional people should not join Session 2 on Managing problems. However, Session 1 (Managing stress) and Session 3 (Get going, Keep doing) can be appropriate times to include a family member or a friend.

- Point out the relevant section of **Chapter 2 of the PM+ Intervention manual** — Presence of family or friends during PM+ sessions.

Lesson 5

Summary, home practice and setting up the review 25min

5.1 Summary (10 minutes)

Tell trainees

- Provide a brief summary of the topics covered in Module 2.
- Provide an opportunity to clarify any concerns or to answer trainees' questions.

5.2 Setting home practice. Considering cultural and local adaptations to the PM+ manual (10 minutes)

Learning outcome: trainees recognize and demonstrate the adaptation of PM+ to the local and cultural context.

Tell trainees

- The PM+ intervention manual in its current form is the generic version of PM+.
- As the first home practice task, ask the trainees to think about and make some notes on cultural and contextual adjustments that might need to be considered before introducing PM+ in their workplace or community. They can review their manual to help with this.

- Tell the trainees they can think broadly about all aspects of the implementation of PM+, or about just one part such as advertising PM+ in the community, recruiting clients, or delivering the PM+ sessions. Trainees might like to consider (but should not feel limited to) the following:
    - language;
    - setting (where PM+ should be offered);
    - stigma associated with seeking mental health support;
    - local expressions or metaphors that could be included.
- Tell trainees that they should be ready to share their reflections at the start of the next training module.

Module 3

Basic helping skills 7h20min

Prepare in advance

If you are training mental health professionals or people with a background in psychological interventions who already have good basic helping skills, this module can be adapted. You might simply review the basic helping skills in PM+ and discuss any special considerations. If you did not run a pre-assessment, you can complete one role-play using the PM+ competency assessment tool to ensure that trainees are competent in these skills.

Note for trainers

The aim of this module is to improve basic helping skills. References to PM+ manual scripts are not included in role-play instructions, as the exercises aim to broadly improve skills relevant for all interactions. You may need to adapt or clarify role-play scenarios, or provide additional direction to trainees (e.g. what to discuss in a role-play), depending on the training group.

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review home practice and introduce the module (25 minutes)			
1.1	**Review home practice (20 minutes)**	Trainees see how to facilitate a review session and feel at ease	Blank flipchart to note reflections
1.2	**Module introduction (5 minutes)**	Trainees know the expected learning outcomes for this module	Consider adaptations if teaching mental health professionals
Lesson 2. Introduction to basic helping skills (135 minutes)			
2.1	**What basic helping skills are (10 minutes)**	Trainees can name the basic helping skills required to deliver PM+	
2.2	**Demonstration role-play. Verbal and non-verbal communication skills (35 minutes)**	Trainees understand why verbal and non-verbal communication skills are important in delivering PM+	

No.	Description	Learning outcome	Advance preparation
2.3	**Trainee role-plays. the importance of basic helping skills (45 minutes)**	Trainees practise using basic helping skills	Prepare slips of paper with 2.3 Trainee role-plays. The importance of basic helping skills (45 minutes)
2.4	**Group role-plays to demonstrate basic helping skills (45 minutes)**	Trainees demonstrate using basic helping skills	Print and share explanations of basic helping skills (Annex 6) Print Module 3 role-play (found at the end of this module) Have trainees' PM+ competency assessment tools ready (paper or online)**. The relevant items are PM+ 1–6**
Lesson 3. The PM+ client–helper relationship (105 minutes)			
3.1	**Role of the PM+ helper from the perspective of the client; group discussion on how to build the client–helper relationship (60 minutes)**	Trainees explore how to use analogies to introduce the role of the PM+ helper to the client	PM+ intervention manual, Chapter 3: Client–helper relationship
3.2	**Trainees role-play how to use basic helping skills to work with a reluctant client (45 minutes)**	Trainees demonstrate how to engage a reluctant client	Have trainee PM+ competency assessment tool ready (paper or online). The relevant item is **PM+ 4**
Lesson 4. Special considerations for delivering PM+ (85 minutes)			
4.1	**Group discussion on the setting and physical contact (35 minutes)**	Trainees understand how to choose a setting, and they discuss cultural norms on physical contact	See *(4)* for an outline of adaptations needed for remote delivery
4.2	**Challenging presentations or contexts (50 minutes)**	Trainees practise how to respond to challenging presentations and contexts	

No.	Description	Learning outcome	Advance preparation
Lesson 5. Giving and receiving helpful and effective feedback (60 minutes)			
5.1	**Giving helpful feedback, and tips for receiving it (10 minutes)**	Trainees hear how to give helpful and effective feedback	Print copies of Annex 5 as handouts, and either prepare scenarios or use a scenario from Annex 7
5.2	**Practising giving and receiving feedback (50 minutes)**	Trainees practise giving and receiving feedback	
Lesson 6. Summary, home practice and setting up the review (30 minutes)			
6.1	**Summary (15 minutes)**	Trainees recap learning	
6.2	**Set home practice (5 minutes)**	Trainees will practise basic helping skills in daily life and observe the effects	
6.3	**Set up the review for next time (10 minutes)**		

Lesson 1

Review home practice and introduce the module 25min

1.1 Review home practice (20 minutes)

Learning outcome: trainees see how to facilitate a review session and feel at ease.

Organize trainees (1 minute)

- Remind trainees that in future they will be running the review sessions but for now they can watch you.

Trainer led review (17 minutes)

- Facilitate a brief review of the information learned in Module 2, including asking trainees to explain what they remember about the PM+ intervention, the training requirements, the characteristics of PM+ helpers and so on.
- Also facilitate a review of their home practice about adapting PM+.
- Ensure that the following points are covered:
  - How might they begin the process of culturally adapting the PM+ manual?
  - What language should PM+ be in?
  - What cultural expressions and metaphors might be included?
  - Should help be offered in someone's home, or at a centre?
  - Should the helper be the same gender as their clients?
  - How should consent be obtained?
  - How should family members be involved?
  - How should the helper discuss taboo topics, such as sexual violence, with clients?
  - Are there any other adaptations that might be needed?
- Make a note of the reflections as these will help inform how PM+ is delivered in the setting.

Tell trainees (1 minute)

- At the end of the review explain that these reflections can continue to be used throughout the training and will help as the trainees begin to learn how to deliver the intervention.

Organize trainees (1 minute)

- Ask for volunteers or nominate two trainees to run the next review session.

1.2 Module introduction (5 minutes)

Learning outcome: trainees know the expected learning outcomes for this module.

Tell trainees:

- They are going to learn about and practise using basic helping skills.
- These are foundational to any intervention, so we want to ensure that everyone is confident in using them. By the end of the module trainees will be able to:
  - know the basic helping skills required to deliver PM+ and why these are such foundational skills;
  - reflect on the PM+ helper's role from the client's point of view, and practise how the use of basic helping skills can build trusting and therapeutic relationships;
  - practise using basic helping skills to work with reluctant clients and those living through challenging and sensitive circumstances.

Lesson 2
Introduction to basic helping skills 135min

Prepare in advance

Basic helping skills cover listening and responding well. There are some differences between the terms used in the PM+ competency assessment tool and the PM+ intervention manual. Despite this difference in terms, all of the skills on the PM+ competency assessment tool are relevant to delivering PM+. Each role-play in the module outlines which items from the PM+ competency assessment tool are relevant.

2.1 What basic helping skills are (10 minutes)

Learning outcome: trainees can name the basic helping skills required to deliver PM+.

Tell trainees:

- Read the material in the *Key concepts* box to the trainees. Consider displaying it as a slide or using it as a handout.
- Answer any questions and discussion points. Explain that this module will cover these skills.

KEY CONCEPTS

Basic helping skills

These are the skills used to build a healthy relationship with clients. They build trust, and offer support and comfort.

Basic helping skills are the foundation of every PM+ session and include the following:

- Verbal communication – we will cover specific techniques for this even though in your manual it is not separated out from other skills.
- Non-verbal communication – this is often called "body language" and we will practise it alongside verbal communication.
- Confidentiality – it is important to explain clearly what will, and what will not, be kept confidential.
- Communicating concern – in other words showing empathy (including using verbal skills to do this).
- Praising openness – this helps clients to engage with you and the PM+ sessions.
- Validating (normalizing) people's experiences – this is the skill of dispelling embarrassment or shame and avoiding belittling the client's feelings.
- Putting aside personal values – remaining nonjudgmental even if you do not agree with your client's beliefs and views.
- Not giving direct advice – our job is to open up possible solutions, not to direct clients to take a specific action.

2.2 Demonstration role-play: Verbal and non-verbal communication skills (35 minutes)

Learning outcome: trainees understand why verbal and non-verbal communication skills are important in delivering PM+.

Tell trainees (1 minute)

- This role-play will mimic a possible first meeting for the local context.

Organize trainees (4 minutes)

- You and your co-trainer will take the roles of helper and client. If you are the only trainer, invite a trainee to play the client.
- Tell the other trainees to observe.

Run the demonstration role-play (5 minutes)

- Introduce yourself as a PM+ helper and begin to talk to the client about why they were referred, the purpose of the assessment session today and a little information about PM+.

- Use poor verbal and non-verbal communication skills but do not make the role-play farcical or silly. You need to be convincing enough that trainees recognize these behaviours as common (e.g. they may have observed a doctor do this with them in the past) but also recognize that these are unhelpful behaviours when establishing a good relationship with a client. Consider any of the following:
    - using a loud voice;
    - not telling the client which organization you are from;
    - sitting too close to the client;
    - keeping too little or too much eye contact;
    - being distracted by your telephone;
    - interrupting;
    - dismissing or minimizing the client's difficulties (e.g. saying "I know someone in a worse situation than you");
    - telling the client that they have to do PM+;
    - correcting them (e.g. saying "I think what you really meant was …." or "That was the wrong thing to do");
    - smiling when the client looks distressed.

Facilitate a discussion (10 minutes)

- Invite trainees to talk about their reactions to the role-play. Emphasize that the demonstration was deliberately wrong to spark discussion.
- Help them think about how they might have felt if they had been the client. How would it feel to talk if no one is listening?
- Invite the client to describe how they felt in that situation. Emphasize the key basic communication skills that were missing from the demonstration, and what effect this would have on things such as trust, relationship, or motivation to join the intervention.

Re-run the demonstration role-play (5 minutes)

- Re-run the role play, this time demonstrating key communication and basic helping skills. The aim of this role-play is to provide a good model of communication skills.

Facilitate a discussion (10 minutes)

- Invite trainees to talk about their reactions to the role-play. Emphasize that the demonstration showed how communication skills are used correctly.
- Ask similar questions to the previous discussion and emphasize the difference between the two role-plays.
- Emphasize the key basic helping skills that were shown in the demonstration, and reflect on how this may positively impact on trust, the relationship, or motivation to join the intervention.

2.3 Trainee role-plays. The importance of basic helping skills (45 minutes)

Learning outcome: trainees practise using basic helping skills.

Prepare in advance

Prepare slips of paper with one behaviour from the list below written on each. Have enough slips for all of the people in your group. Some behaviours might be written several times depending on the size of your group.

- **Posture:** sitting in a stiff and upright position with arms folded.
- **Posture:** sitting in slouched position.
- **Personal space/position:** sit facing the person but too far from them.
- **Personal space/position:** sit facing the person but too close to them.
- **Personal space/position:** sit next to the person so that you are both facing in the same direction.
- **Tone of voice:** speak in a quiet voice.
- **Tone of voice:** speak in a loud voice.
- **Eye contact:** stare at the person for long periods of time.
- **Eye contact:** give minimal eye contact to the person (look away from them).

Organize the trainees (5 minutes)

- Give one slip of paper to each trainee and tell them not to share the content.
- Ask them to form pairs, and decide whether to be person A or person B.
- Give them instructions for how to perform the role-plays (see points below).

Run the trainee role-play (20 minutes)

- In the first half of the role-play, person A plays the helper, and person B plays the client.
- Person B talks about a recent situation (ideally using a made-up problem).
- Person A responds, using the behaviour on their paper.
- Person B should identify and remember their personal reactions to person A's behaviour (e.g. their feelings, thoughts or behavioural reactions) for later discussion.
- After about five minutes ask the pair to swap so that person B plays the helper and person A is the client.
- Then, stop the role-plays and ask the pairs to repeat the process, but this time, not using the behaviours on the paper, but using their basic helping skills as they naturally would.

Facilitate a discussion (20 minutes)

- Ask trainees to contribute their reflections on their personal reactions to their partner's behaviour (e.g. their feelings, thoughts or behavioural reactions) for the first and second role-play.
- Then ask the trainees: "Thinking about the previous activities, why are basic helping skills so important in PM+?"
- Some prompts might include:
  - What behaviour did you notice in your partner?
  - How did it feel to have your partner behave this way when sharing something personal?
  - How did it feel when they were not behaving this way?
- Write the trainees' responses on a flipchart.
- Finish the discussion by using the *Key concepts* to help answer the last question if the trainees do not cover all the issues.

KEY CONCEPTS

Basic helping skills help to:

- build rapport with your client;
- make clients feel more comfortable in a potentially uncomfortable situation (i.e. talking with a stranger about personal matters);
- build trust in the relationship, especially for clients who lack trust due to previous experiences;
- encourage clients who might feel ashamed to disclose their trauma (e.g. intimate forms of violence);
- improve the chances that the client will engage with the PM+ strategies and will be motivated to stay in the programme.

2.4 Group role-plays to demonstrate basic helping skills (45 minutes)

Learning outcome: trainees demonstrate using basic helping skills.

Prepare in advance

Print Annex 6 (handout describing basic helping skills).

Print and cut out multiple copies of the Module 3 role-play instructions (at the end of this module).

Have trainees' PM+ competency assessment tools to hand for the trainee role-play.

Tell trainees (10 minutes)

- Ask trainees to turn to **Chapter 3 of the PM+ intervention manual** and find the section entitled basic helping skills, with definitions of the various skills that are required to deliver PM+.
- Share Annex 6 (handout describing basic helping skills).
- Give the trainees 5 minutes to read the definitions of basic helping skills and respond to any concerns they may have.
- You might want to point out that this training identifies verbal communication as an additional skill to the basic helping skills listed in the PM+ manual.

Organize trainees (5 minutes)

- Divide trainees into groups of three and ask them to decide who will play the role of the PM+ helper, PM+ client and observer.
- Give those trainees playing the role of the PM+ helper a copy of the 'Information for the person playing the role of the PM+ Helper' in the Module 3 role-play and 1-3 different basic helping skills with instructions on how to demonstrate that basic helping skills (Table 3 at the end of this module).
- Give the trainees playing the role of the PM+ client a copy of the Information for the person role-playing the client in the Module 3 role-play instructions, a case scenario to adapt if they want them, and 1-3 corresponding basic helping skills instructions to elicit the basic helping skill from the PM+ helper scenarios (Table 4 at the end of this module).
- Ask the person playing the role of the observer to watch the role-play and at the end offer feedback to their peers.
- If needed, clarify that, since this module focuses on learning basic helping skills applicable across all interactions, the PM+ manual scripts will not always be used. These scripts will be used more regularly in later training modules.

Run the trainee role-plays (15 minutes)

- Explain that during the role-play the PM+ helper aims to establish a trusting relationship with the client and starts to find out what has been happening in their life and why they are seeking help.
- Tell them they have up to 10 minutes to perform the role-play.
- During the role-plays walk from group to group and informally rate trainees' skills using the trainee's PM+ competency assessment tool (digital or paper). The relevant items for this role play are PM+ 1 (non-verbal communication), PM+ 2 (verbal communication), PM+ 3 (explain and promote confidentiality), PM+ 4 (rapport building and self disclosure), PM+ 5 (exploration and normalization of feelings) and PM+ 6 (demonstrate empathy, warmth and genuineness).
- After the role-plays, trainees can give feedback to each other in their small groups.

Facilitate a discussion (15 minutes)

- Bring the group back together and give more general feedback based on what you observed.
- Ask the observers and the rest of their trainees to give their feedback and facilitate a discussion.
- Finish the discussion by reemphasizing why basic helping skills are important and that they will be used throughout PM+ delivery.

Lesson 3
The PM+ client-helper relationship 105min

3.1 Role of the PM+ helper from the perspective of the client and group discussion on how to build the client-helper relationship (60 minutes)

Learning outcome: trainees practise how to use analogies to introduce the role of the PM+ helper to the client.

Tell trainees (5 minutes)

- Basic helping skills build the relationship between the PM + helper and client, and this relationship is vital in delivering an effective PM+ Intervention.
- We defined the role of the PM+ helper in Module 2 but will now reflect on the role of the helper from the perspective of the client.
- Explain that different factors can affect the client-helper relationship such as: the role of the helper, reluctant clients, verbal and non-verbal interactions and the setting where they meet.

Facilitate a discussion (55 minutes)

- The aim of this discussion is to encourage trainees to reflect on how clients may perceive and respond to PM+ helpers.
- The discussion can be facilitated in three steps:

Step 1 (10 minutes): Ask the trainees "What reasons might clients have for seeing a PM+ helper?" Allow five minutes to talk about this and write down suggestions. You can use the following as prompts if necessary:

- They believe PM+ will help them.
- They have tried other options without feeling better.
- They have been encouraged or pressured by a family member to seek help.
- They see the helper as a doctor or traditional healer, and expect to be healed or mended.

- Their doctor recommended it.
- They heard about the programme from a neighbour, who said it was very helpful.
- Rumours about the programme were circulating in the community, so a community leader wanted to join to find out what it was all about.

- Encourage trainees to think of as many different types of reasons as they can.
- Finish this step by saying clients may have many different feelings about seeing a helper. Some may believe that asking for help means they are weak. Therefore, it is important that PM+ helpers normalize the client's feelings and reassure them that they are not weak, and that many people feel strange asking for help.

Step 2 (5 minutes): Direct the trainees to **Chapter 3 in the PM+ intervention manual**, where they will find a section titled *The client-helper relationship*. Give the trainees time to read the suggested script and description of the PM+ helper role.

- The PM+ intervention manual recommends describing the helper's role as similar to that of a teacher, but this may not be appropriate for every setting and can be adapted. For example, explaining to the trainees that it is important clients understand the helper views the client as the expert in their own emotions and experiences and the helper is an expert in PM+.

Step 3 (40 minutes): Get the trainees to form small groups and ask them to discuss analogies for the client-helper relationship that fit with their local context (10 minutes). They should be exploring a culturally adapted way to present the role of the helper to the client and community. This will build better acceptance and understanding of the PM+ intervention.

- Have the small groups rejoin the larger group. Invite one person from each group to present their analogy to the whole class (allocate 2 minutes per person). After all the presentations, ask the trainees to select their preferred analogy for future use. Trainers can decide how trainees will vote for their preferred analogy (e.g., by raising hands, calling out, or voting anonymously).

3.2 Trainees role-play how to use basic helping skills to work with a reluctant client (45 minutes)

Learning outcome: trainees demonstrate how to engage a reluctant client.

Prepare in advance

Have trainees' PM+ competency assessment tools to hand for the trainee role-play. The most relevant item is number four: rapport building and self-disclosure (corresponding to praising openness).

Facilitate a discussion (15 minutes)

- Now ask the trainees, "How would you feel about telling a stranger your emotional difficulties for the first time? Would you feel comfortable?".

- Within their answers, highlight that some clients may hesitate to talk openly with their helper, particularly clients who have been referred for help by a doctor or family member.
- Ask trainees to think about what makes a client hesitant to talk.
- Write their responses on a flipchart and then ask the trainees to compare the answers with the ones provided in **Chapter 3 of the PM+ intervention manual in the section titled B. *The reluctant client.***
- Now ask the trainees what might help a client to feel more comfortable when sharing personal information, and how they might respond to a reluctant client in the initial session.
- Within their answers, highlight consistent use of basic helping skills as a way to help clients relax.

Organize trainees (5 minutes)

- Divide trainees into groups of four or five persons.
- Give each group one reason – from the list just generated – why a client might hesitate to talk.
- Ask the groups to choose someone to play the PM+ helper and someone to play the reluctant client. The other trainees should observe.

Run a trainee role-play (5 minutes)

- Run a short role-play (maximum five minutes).
- The person playing the PM+ helper is introducing PM+ to the client.
- During the role-plays, informally rate one or two helpers (per trainer), using their PM+ competency assessment tool (digital or paper). PM+ 4 (rapport building and self disclosure) is the most directly relevant item to this role-play, although you may want to rate other items too.

Facilitate a discussion (20 minutes)

- Bring the class back together and ask for trainees' reflections.
- Then provide group feedback based on your observations (but do not single out anyone).
- Create a list of points to remember when meeting a reluctant client. Talk through the following approaches.
    - **Always be gentle and respectful.** Remember this when encouraging others to share information with you.
    - **Never pressure clients to talk.** If a client does not want to talk further about a topic, respect this. A helper might say: "I can see that this is upsetting for you to talk about […] and I want to respect that. However, if you do want to return to talking about this topic, I want you to know that I am ready to listen to you at any time during this programme."
    - **Skip unanswered questions and return to them later if relevant.** Note which questions a client does not answer.

- **Let clients know they can pause or stop.** Some clients may believe they are expected to do everything their helper suggests, including talking about sensitive and personal topics. To ease a client's discomfort, the helper could say: "You seem very upset talking about this. I am willing to listen to your story and help you talk about it but I want you to know that you can decide what we talk about. If you need to stop at any point or if you do not want to talk about a particular part of the story, then this is OK."

Lesson 4

Special considerations for delivering PM+ 85min

Prepare in advance

If the trainees may be delivering PM+ by telephone or video, be ready to adapt Lesson 4.1 on arranging the setting. See WHO's *Psychological intervention implementation manual* (3) for an outline of adaptations needed for remote delivery.

4.1 Group discussions: considering the setting and physical contact (35 minutes)

Learning outcome: trainees understand how to choose a setting, and they discuss cultural norms on physical contact.

Tell trainees (5 minutes)

- This discussion is designed to encourage trainees to reflect on the settings that enhance the delivery of PM+ as well as non-verbal communication such as physical contact.

Facilitate a discussion on the setting of PM+ (15 minutes)

- Explain that the setting for the PM+ sessions is an important element in the client–helper relationship.
- Ask trainees why they think this is so.
- Ask them to suggest what is important to consider when choosing a venue.
- Make sure the discussion covers privacy and seating arrangements.
- Highlight that helpers should always try to find a private, comfortable setting for PM+, whether the sessions are face-to-face, online or by telephone.
- Allow a few minutes for trainees to reflect privately on where they will hold their own PM+ sessions and to consider any problems they may encounter.
- If your trainees may deliver PM+ by telephone or video, adapt this exercise, reflecting on how to maintain confidentiality and create a therapeutic relationship.

Facilitate a discussion on physical contact (15 minutes)

- Explain that another important factor affecting the client–helper relationship is the participants' physical interaction.
- For instance, in some cultures the use of physical touch – such as touching someone's shoulder to offer support – is acceptable. In other cultures, this is not appropriate.
- Emphasize that helpers should avoid using physical contact or touch to express support and concern and should respect people's personal space.
- Discuss what physical interaction and behaviour between the helper and client is acceptable in the trainees' cultures. Highlight that trainees should be aware of cultural norms and should respect them.
- Finish the discussion by referring trainees to the **PM+ intervention manual Chapter 3** to read more about the importance of considering the setting and physical contact when delivering PM+.

4.2 Challenging presentations or contexts (50 minutes)

Learning outcome: trainees practise how to respond to challenging presentations and contexts.

Tell trainees (5 minutes)

- Explain that there are situations where some basic helping skills are needed more prominently than others.
- Ask trainees what basic helping skills they think would be particularly important for clients with a history of severe trauma, such as sexual abuse, torture or imprisonment. Examples are:
    - communicating concern;
    - praising openness;
    - validating;
    - keeping confidentiality;
    - non-verbal skills.

Facilitate a discussion (15 minutes)

- Ask trainees "What basic helping skills would be **particularly important** with clients with a history of sexual assault?" Examples are:
    - keeping confidentiality, praising openness, non-verbal skills, validating.
- Ask the group some of these questions to promote discussion:
    - Has anyone experienced working with individuals who have been affected by sexual violence?
    - Have people you have worked with talked openly about these experiences?
    - What does your community think about gender-based violence?

  - Is there shame associated with it? Do people speak openly about it?
  - Are there any fears associated with this experience?

- Next, ask the group, "From this discussion, what difficulties would you expect a client to have if they had experienced this type of adversity?" Ensure that the group identifies these difficulties:
  - trusting others;
  - stigma and shame;
  - fear of partner/perpetrator finding out they have disclosed the information;
  - risking further violence.

Organize the trainees (5 minutes)

- Ask the trainees to get into pairs, and refer to the **PM+ intervention manual Chapter 3** subsection on challenging presentations and context.
- Tell them to review the guidance in the PM+ intervention manual and create a fictional scenario of someone who has experienced sexual assault.

Facilitate a small group discussion (15 minutes)

- Ask trainees to use the fictional case scenarios to practise using and adapting the gentle and respectful phrases from the PM+ intervention manual, familiarizing themselves with how they could respond to this challenging topic.
- Ask them to reflect on what could be difficult or uncomfortable for either the client or the helper, and how they could manage those difficulties.

Facilitate a large group discussion (10 minutes)

- Bring the whole group back together and ask pairs to share their reflections and discuss the ways in which they felt uncomfortable as helpers. Ask for any suggestions they have for making themselves feel more confident.
- Highlight ways they could use basic helping skills to build trust and reduce their client's fear.
- Remind everyone that it is for the client to decide what they disclose.
- Finish the discussion by ensuring the points in the *Key concepts* box were covered in the discussions.

KEY CONCEPTS

Do not force a client

The client must decide whether or not to disclose sensitive or challenging experiences.

No one should be forced to say things they do not want to say. Instead, use basic helping skills as a foundation to build trust.

Clients who feel safe and respected may feel ready to disclose sensitive and challenging topics.

Lesson 5

Giving and receiving helpful and effective feedback 60min

Prepare in advance

For Lesson 5.1 on Giving helpful feedback, you will need printed copies of the How to give feedback handout from Annex 5.

5.1 Giving helpful feedback, and tips for receiving it (10 minutes)

Learning outcome: trainees hear how to give helpful and effective feedback.

Tell trainees

- Tell the trainees about the two steps of giving feedback – i.e. giving positive feedback before dealing with areas for improvement. Explain that this is an important skill both for the training and for use when delivering PM+.
- Give out the accompanying handout and cover the described steps.

Step 1: Give positive feedback first

Be specific with your praise and avoid saying only "It was good".

Instead, describe what was positive, e.g. say:

- "You used the basic helping skills appropriately"; or
- "You had good eye contact, and you were able to reflect back what the client was saying in a way that normalized their feelings".

Give an example of when you noticed something positive. For example, you might say:

- "When the client was reluctant to engage [in Get going, Keep doing], you reflected back her concerns in a gentle and understanding way. Then you repeated the steps for this strategy in a way that made sense to her and helped her see that this strategy may be helpful for her. You then reassured her that you would help her increase her activity slowly."

Describe how the helper's comment or behaviour helped the client. For example, you might say:

- "By demonstrating basic helping skills and going over the steps for [...], the client felt listened to and that their concerns were valid. It also helped them understand and engage in the strategy."
- "What do you think? Do you agree or disagree with this comment?"

Step 2: Provide feedback about areas to improve (only if necessary)

If there is an area where you believe the person needs improvement, or if you need to make a correction of some kind, do so gently and using your basic helping skills.

Be specific, give an example, and discuss how this may have affected the client (or explain that this is not how PM+ should be delivered). For example, you might say:

- "When you told the client what to do, I noticed that she started to close down and stop talking to you so openly. Did you notice that? Why do you think that happens?"

Invite the person to respond. For example, you might say:

- "What do you think? Do you agree or disagree with this comment?"

- Finish the explanation by going through these *Key concepts*.

KEY CONCEPTS

Tips for receiving feedback

Listen well. Focus on what is being said as it will most likely be very helpful to you and can help you improve.

Ask questions for clarification. Feedback is not a passive exercise, asking clarifying questions can be very helpful. But don't interrupt.

Process and acknowledge the feedback. Don't act defensively and don't reject feedback. Think about it and acknowledge the causes: you will immediately see the benefits.

Create an action plan. Good feedback can only help if it is put to good use. Receive the feedback and begin planning immediately how to use it.

Remember that the feedback is about what happened, not about you as a person. Feedback should be factual, and should be based on an action someone took. Always think of how these actions can be improved. Don't take feedback personally. It is about delivering the best service to our beneficiaries.

5.2 Practising giving and receiving feedback (50 minutes)

Learning outcome: trainees practise giving and receiving feedback.

Prepare in advance

Print case scenarios in Annex 7. Alternatively, you might prefer to prepare scenarios that reflect common situations that your trainees could encounter.

Organize trainees (5 minutes)

- Split the trainees into pairs.
- Explain that they will now practise giving helpful feedback, based on a scenario.
- Give each group one of the scenarios from Annex 7 or create your own.

Run a trainee role-play (25 minutes)

- **Step 1:** Ask the pairs to discuss the scenario and how best to give the helper feedback, using their learning and the Annex 5 handout.
- Give them about 10 minutes to create a two-minute role-play that shows how they would give this feedback to the helper. One person acts as the helper in the scenario and another gives feedback.

- **Step 2:** Ask each pair to join with another pair, ensuring they have a different scenario, and to present their role-play to each other (approximately two minutes per pair). The person playing the role of the helper (receiving feedback) can either act out the case scenario or read it to the observing pair.
- The observing pair should reflect and comment on the feedback skills and on anything the trainees can improve in giving feedback (allow five minutes).

Facilitate a discussion (20 minutes)

- Bring the class together again and ask for reflections on this activity.
- Ask trainees what they think will be challenging in giving constructive and helpful feedback and address any issues they raise.
- Ask them to reflect what it was like to receive feedback from their colleagues.
- Highlight useful responses, such as not making excuses for poor performance, not interrupting, thanking colleagues for their feedback, and considering how to use feedback to improve.

Lesson 6

Summary, home practice and setting up the review 30min

6.1 Summary (15 minutes)

Tell trainees

- Provide a brief summary of the topics covered in Module 3. Cover the following points.
- Basic helping skills are foundational to PM+. They start with, but are not limited to:
  - the way we use verbal and non-verbal communication skills to build a healthy relationship with a client;
  - how we establish boundaries and safety by explaining confidentiality;
  - how we consider the environment and setting we are in when we meet a client;
  - how we set up and explain the role of the PM+ helper to the client;
  - how we respond with respect and sensitivity if the client discloses challenging topics and how we can encourage a reluctant client to engage with us.
- Emphasize that these basic helping skills are repeated throughout the PM+ training programme and are the foundations for PM+ sessions. The best way to get to know them is to practise them.

6.2 Set home practice (5 minutes)

Learning outcome: trainees will practise basic helping skills in daily life and observe the effects.

Tell trainees

- Ask the trainees to practise using their basic helping skills in conversations with another person, and to reflect on how using the skills affected the discussion.
- Tell trainees they should come to the next training session prepared to share their experiences with the rest of the group.

6.3 Set up the review for next time (10 minutes)

Organize trainees

- Remind trainees who are to facilitate the review session at the start of the next module.
- Remind them to focus on key information they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run this review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 3 role-play

Basic helping skills

These instructions are for the trainee role-plays from Module 3, Lesson 2.4 on *Group role-plays to demonstrate using basic helping skills*.

The PM+ intervention manual outlines several basic helping skills, and this training adds verbal communication skills to this.

You can choose to give two or three basic helping skills to each role-playing group at a time.

Print out the general instructions to the PM+ helper and some basic helping skills instructions, and give them to the trainee playing the helper. Give to the trainee playing the client one of the scenarios and the corresponding prompts for playing the client role.

Information for the person playing the role of the PM+ helper

The trainer will give you two or three basic helping skills to demonstrate during this role-play. If you feel confident, and if it is relevant to the role-play, please show other skills too. Think back to a time when a close friend or family member was thankful to talk to you about a problem. It is likely that you used these skills while you were listening to them. These skills can be very natural and show clients that you are listening and willing to support them. Use your natural instincts in the role-plays.

In this role-play you are the PM+ helper, you are meeting a possible client for the first time. You are trying to establish a trusting rapport as well as finding out some background information about the client and why they are seeking help.

TABLE 3

Instructions for the person role-playing the helper in the Module 3 role-play

Basic helping skill	Instructions for role-playing the helper
Non-verbal communication and active listening	Your role is to begin the meeting and actively listen to the client's thoughts and feelings as they share some of their experiences throughout the role-play. Use non-verbal communication skills to make the client feel safe and heard.
Verbal communication	Your role is to discuss some of the reasons the client has come to see you. Use good verbal communication skills. Remember to show the client that you are engaged, ask them questions and demonstrate understanding of what they are saying.
Correctly explaining confidentiality	You need to explain and promote confidentiality, making sure the client understands the reasons for your approach. Refer to the **PM+ manual Chapter 4** – *Steps to follow to complete an assessment*, point 3 on confidentiality if needed.
Praising openness	Use the PM+ intervention manual to find key phrases that praise openness.
Validating	Ask the client to explain how they have been feeling, and what they have been experiencing recently. Explore these feelings further, normalizing and validating feelings and concerns in relation to the situation. You will find some key phrases in the PM+ intervention manual.
Communicating concern and putting aside personal values	Your role is to demonstrate empathy and to communicate concern with warmth and genuineness towards the client. Explore the client's feelings and problems without passing judgement or making assumptions about how you think they should be feeling.

Basic helping skill	Instructions for role-playing the helper
Not giving advice	Your role is to help, but not to advise. Offer information that your client might find helpful. Try to rephrase any advice you feel tempted to give so that the client can explore it as an option rather than taking it as an instruction. You might ask what they would advise a friend to do in a similar situation.

Information for the person role-playing the client

You can choose to use the background information in either of these stories to inform your character in the role-plays or to create your own fictional PM+ client.

Female scenario

The client is a 24-year-old mother of two young sons. She is married but her husband lives across the border, leaving her alone to care for the children. He sends money irregularly. She feels frightened by violence near her apartment, struggles to sleep, worries about money and feeding her children, and gets angry with the children when they complain about wanting things. She feels ashamed to ask neighbours for help and fears they think she just wants money. She feels very lonely and has been crying a lot in the past two months. She doesn't really know what help she wants from PM+ or what she expects the helper to do.

Male scenario

The client is a married man with four school-aged children and who works full-time at a local factory. Six months ago he had an anxiety attack he thought was a heart attack, but doctors assured him he is medically healthy. He now feels anxious constantly, worrying about having another attack, especially at work in front of colleagues. He avoids activities like exercise so as to prevent attacks. He argues frequently with his wife. He fears having more anxiety attacks and has anxiety symptoms like racing heart, dizziness and fatigue. He isolates himself, avoiding social and enjoyable activities. He struggles to ask for help and he is not sure what help there might be.

TABLE 4

Prompts for the person role-playing the client in the Module 3 role-play

Basic helping skill	Instructions for role-playing the client
Non-verbal communication and active listening	At times during the interview use culturally appropriate body language for feelings of sadness or worry. When asked, describe why you are seeking help. You might say "I am feeling anxious all the time", "I get so angry and shout at people I love", or "I feel tired and can't stop crying".
Verbal communication	When the PM+ helper uses closed-ended questions such as "Do you, did you, can you…?", respond with short yes/no answers. When the helper uses open-ended questions, such as "Please tell me about …", "Would you like to share with me how…." or "How did that make you feel?", respond with more detailed answers.
Correctly explaining confidentiality	During the role-play, ask the helper "Are you going to tell anyone these things that I tell you?" or say "I am afraid you will tell other people the things I am telling you".
Praising openness	During the role-play, be slow and reluctant to share too much personal information. But if the PM+ helper starts to use phrases that praise your openness, then respond by being more open.
Validating	Do not share feelings or emotions unless the helper asks about them. If the helper does ask questions such as "How are you feeling?" or "Please tell me about anything that has been bothering you or worrying you lately" or "I notice that you seemed sad when you came in, please tell me if something has been upsetting you", then describe difficult feelings relevant to the scenario. For example, you might say, "I am afraid to go out alone, what if I get attacked?" or "What if I have an attack again?"

Basic helping skill	Instructions for role-playing the client
Communicating concern and putting aside personal values	When asked by the PM+ helper, describe feelings and problems such as “I am feeling really tired all of the time” or “I get easily aggravated and fight with people even though they are trying to help me” or “I am so worried all of the time” or “I am worried about what people will think of me”. At times during the role-play, use culturally appropriate body language for feelings of sadness or worry and, if asked, describe sadness. If the PM+ helper uses personal assumptions or makes judgements during the discussion, such as telling you what you should feel or do, then withdraw from the conversation.
Not giving advice	Say to the helper: “I feel so alone and I want to find more people and friends, as I need help and support. what should I do?” If the helper offers resources or information, accept it and show that you will consider it. If the helper corrects you or dismisses and minimizes your concerns, tells you that you are wrong or that what you are feeling is wrong, then react by closing down a little or by saying that you don’t think their suggestion will work. If your helper asks “What would you advise a friend to do in this situation”, then suggest some advice for yourself.

Module 4

Assessing PM + clients 6h

No.	Description	Learning outcomes	Advance preparation
Lesson 1. Review and introduction (20 minutes)			
1.1	**Review home practice (15 minutes)**	Trainees revise their knowledge of basic helping skills and practise facilitating a review session	
1.2	**Introduction to Module 4 (5 minutes)**	Trainees know what to expect in Module 4	Adapt this module to be consistent with the assessment process you will use for PM+ in your setting.
Lesson 2. Using PM+ assessments (85 minutes)			
2.1	**The three types of PM+ assessments (25 minutes)**	Trainees know the assessments that are used to ensure suitability and track a PM+ client's progress	Print multiple copies of assessment questionnaires from the **PM+ intervention manual, Appendix A, B and C**
2.2	**Understanding PM+ exclusion criteria (20 minutes)**	Trainees know when and why PM+ might not suit a client, and what action to take	
2.3	**Demonstration role-play of a PM+ pre-assessment (40 minutes)**	Trainees experience how to carry out a pre-assessment	Create a scenario for the person playing the client (Annex 8) Print blank copies of the **pre-intervention assessment form from the PM+ intervention manual**

No.	Description	Learning outcomes	Advance preparation
Lesson 3. Practising how to do PM+ assessments (60 minutes)			
3.1	**How to do an assessment (10 minutes)**	Trainees understand how to conduct an assessment	Trainees need their **PM+ intervention manual**
3.2	**Trainees role-play using the during-intervention and post-intervention assessments (50 minutes)**	Trainees practise using the PSYCHLOPS assessment tool	Distribute copies of **PSYCHLOPS from the PM+ intervention manual, Appendix B**
Lesson 4. Coping with challenges during an assessment (40 minutes)			
4.1	**Challenges during assessments (40 minutes)**	Trainees can identify why people may feel uncomfortable during assessments, know how to support them, and see two challenging assessments	
Lesson 5. Assessing and monitoring thoughts of suicide (130 minutes)			
5.1	**Group discussion on assessing thoughts of suicide (30 minutes)**	Trainees know when and how to talk to clients about thoughts of suicide	**PM+ intervention manual, especially Appendices A and B.** Annex 9 of this manual
5.2	**Demonstration role-play: trainees explore actions to take if a client has thoughts of suicide (25 minutes)**	Trainees become familiar with actions to take following an assessment of suicidal thoughts	**PM+ intervention manual, Appendix A Part 6**
5.3	**Demonstration role-play exploring reasons to stay alive (35 minutes)**	Trainees learn how to manage clients' thoughts of suicide by exploring reasons to live	**PM+ intervention manual, Appendix D**
5.4	**Trainees role-play monitoring thoughts of suicide using PM+ manuals (40 minutes)**	Trainees become familiar with a script for monitoring clients with thoughts of suicide	Print Module 4 role-play instructions and blank PSYCHLOPS forms Have copies of trainees' PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 7**

No.	Description	Learning outcomes	Advance preparation
Lesson 6. Summary, home practice and setting up the review (25 minutes)			
6.1	**Summary (5 minutes)**	Trainees consolidate their learning on assessing thoughts of suicide	
6.2	**Set home practice (15 minutes)**		
6.3	**Set up the review for next time (5 minutes)**		

Lesson 1

Review and introduction 20min

Prepare in advance

The content you cover in this module will vary, depending on how PM+ clients are going to be selected in the programme you are training for. In some programmes, PM+ helpers will screen potential clients. If that is expected, trainees should receive the full training in this module.

If PM+ helpers will not conduct the pre-assessments, you can focus more on the during-intervention PSYCHLOPS assessment section and explain that other staff will screen potential clients before referring them to a helper. **Assessing and monitoring thoughts of suicide should always be covered.**

1.1 Review home practice (15 minutes)

Learning outcome: trainees revise their knowledge on basic helping skills and practise facilitating a review session.

Organize trainees (10 minutes)

- Nominated trainees lead the review session (up to 10 minutes).
- The trainer answers questions or clarifies misinformation about the previous module's content.
- Either invite two new trainees to volunteer to lead the next review session, or nominate a pair of trainees.

Trainer-led review (5 minutes)

- Review the trainees' home practice and experience of using basic helping skills.
- Ensure you ask the following questions:
  - Can the trainees name the basic helping skills they learned in Module 3?
  - Which skills felt natural to them?
  - Did using basic helping skills improve the connection trainees felt in the conversation?
  - Which skill was the most difficult to use – and what made it difficult?
- You can invite further questions. This gives an opportunity to dispel misunderstandings or clarify explanations.

1.2 Introduction to Module 4 (5 minutes)

Learning outcome: trainees know what to expect in Module 4.

Tell trainees

- Explain that the first part of the module covers the assessments required to deliver PM+ to a client and to monitor their progress.
- Tell trainees they will explore how to conduct a successful client assessment, the steps involved, and ways to cope with challenges.
- Explain that the second part of the module will focus on assessing a client for thoughts of suicide.

Lesson 2
Using PM+ assessments 85min

Prepare in advance

Lesson 2.1: Prepare copies of the PM+ assessments (**Appendix A, B and C in the PM+ intervention manual**) as handouts for trainees. Trainees will need **Appendix A, Part 7 of the PM+ intervention manual** when reviewing exclusion criteria.

2.1 The three types of PM+ assessments (25 minutes)

Learning outcome: trainees know the assessments used to ensure suitability and monitor a PM+ client's progress.

Tell trainees (5 minutes)

- Explain in detail the differences between the three PM+ assessments, particularly noting the differences between the different versions of the PSYCHLOPS.
- Highlight that the pre-assessment is where one first meets the potential PM+ client and can set the tone for the whole intervention. Therefore the use of basic helping skills to build a trusting relationship from the beginning is essential.
- Cover the information in the *Key concepts* box.

KEY CONCEPTS

PM+ assesses clients in three ways:

Pre-intervention assessment. We use this to meet the client, assess the severity of their emotional distress and difficulties in daily functioning and determine if PM+ is suitable.

PM+ during-intervention assessment. A Psychological Outcomes Profile (or PSYCHLOPs) is completed at the beginning of every PM+ session. This helps you to understand and monitor changes during treatment. If clients are not improving, these assessments will allow you to gather information about why and give you and your supervisor information to decide how to adjust the support offered.

Post-intervention assessment. At the end of PM+, re-administer a PSYCHLOPS and revisit the pre-assessment measures in order to record any changes that the client reports. Support any referrals if needed.

Organize trainees (5 minutes)

- Divide the trainees into three groups.
- Give each group copies of the pre-assessment, during -treatment assessment and post-treatment assessments **(Appendix A, B and C in the PM+ intervention manual).**
- Give each group a few minutes to read the questions posed in the assessments.

Facilitate a small group discussion (5 minutes)

- Ask the groups to discuss the purpose of the assessments.
- Make a list of what key information is gathered from each assessment.
- Ask what the similarities and differences are between the assessments?
- Tell them to write their ideas on a flipchart.
- Tell them they have 5 minutes to complete the exercise.

Facilitate a large group discussion (10 minutes)

- Bring the groups of trainees back together.
- Choose one group to explain ONE key piece of information.
- Encourage the other groups to add any other insights.
- Finish the discussion by highlighting again the distinct roles of the different assessments (use the key concepts box again if necessary).

2.2: Understanding PM+ exclusion criteria (20 minutes)

Learning outcome: trainees know when and why PM+ might not suit a client, and what action to take.

Tell trainees

- Explain that after conducting a PM+ pre-assessment the helper will be able to understand whether a person would benefit from the PM+ intervention, or whether the intervention is not suitable for them at this time.
- PM+ has two criteria that indicate that the intervention is not suitable for a client.

- **Exclusion criteria 1. A potential client has thoughts of ending their life in the near future.**
- PM+ is not suitable for this person. The helper should refer to **Appendix D on assessing and responding to thoughts of suicide of the PM+ intervention manual**. This gives clear questions to ask clients to help understand thoughts and plans.

- Reassure trainees that you will cover this in more detail later in this module. Go over the following key points now with the trainees and give them time to ask questions.
  - Many people who experience depression, anxiety or grief will have thoughts of ending their lives at some point.
  - These thoughts are symptoms of the person's emotional problem.
  - Clients should not be made to feel guilty or ashamed of these thoughts.
  - Most people do not want to have these thoughts – they just come into the person's mind – and the thoughts cause them distress.
  - Most PM+ clients will not have thoughts about ending their life.
  - Helpers should explore if the potential client has these thoughts in the first assessment, to ensure that they can get the help they need.
  - PM+ helpers must monitor their clients during the intervention. If clients report thoughts of suicide, helpers should follow the guidelines for responding to suicidal thoughts outlined in **Appendix D of the PM+ intervention manual**.

- **Exclusion criteria 2. The potential client has a severe impairment possibly due to a mental, neurological or substance use disorder.**
  - PM+ is not suitable for individuals experiencing severe impairments — possibly related to mental, neurological or substance use disorders — that significantly affect their ability to understand information and limit participation.
  - People with such difficulties may struggle to engage with PM+ effectively, as their ability to concentrate and retain information may be affected.
  - Explain that trainees are not expected to make a diagnosis, but to use a set of questions to help assess whether a person is suitable for PM+.

- Clarify that if a person appears to be under the influence of a psychoactive substance (e.g. drugs or alcohol), or may need additional support for a substance use problem, the implementing agency should have procedures in place to support the person. This may include referring the person to a specialized service (e.g. a drug and alcohol service), and outlining specific conditions under which the person can participate in PM+ (e.g., when not under the influence). The helper should always contact their supervisor for guidance on these situations.
- For example, a helper may in a sensitive and empathetic way explain that they should not continue with the session as they are under the influence of drugs or alcohol. However, they may be able to attend a future session if they are not under the influence. The helper would also contact their supervisor for guidance on the need for any other support or services.
- Give the trainees a few minutes to review **Appendix A, Part 7 of the PM+ intervention manual**. Explain that the helper should consider excluding someone from PM+ if they answer **"No" to questions 1 or 2, or "Yes" to questions 3 or 4** of this section.
- Answer any questions or concerns that the trainees may have.

2.3 Demonstration role-play of PM+ pre-assessment (40 minutes)

Learning outcome: trainees experience how to carry out a pre-assessment.

Prepare in advance

Print copies of Annex 8 scenarios and share them.

If you do not have a co-trainer, you will need a suitable volunteer to help you deliver the demonstration role-play. It is best to preselect this person.

Organize trainees (5 minutes)

- Work with a co-trainer or volunteer.
- You play the role of the PM+ helper and the co-trainer/volunteer plays the role of the client using the scenario from Annex 8.
- The other trainees will observe the role-play, using blank copies of the **pre-intervention assessment form from Appendix A of the PM+ intervention manual** to score the clients' answers.

Run the demonstration role-play (20 minutes)

- Role-play the whole assessment ensuring that you demonstrate:
  - strong basic helping skills;
  - management of the client's distress (e.g. their crying);
  - management of the client's concern about the personal nature of the questions;

- management of the client when they become overly talkative or go off-topic;
- how to move through the assessment questions as the client discloses relevant information (i.e. not needing to complete the assessment questions in order);
- responding to a client who asks for medications or material compensation.

Facilitate a discussion (15 minutes)

- At the end of the role-play, ask the trainees to choose a partner and compare their assessment scores.
- Address any major differences in scoring and answer any questions/concerns or reflections trainees may have.

Lesson 3

Practising how to do PM+ assessments 60min

Prepare in advance

Be ready to use the PSYCHLOPS assessment tool in **Appendix B of the PM+ intervention manual**.

3.1 How to do an assessment (10 minutes)

Learning outcome: trainees understand how to conduct an assessment.

Organize the trainees

- Ask trainees to look at **Chapter 4: *How to do an assessment* in the PM+ intervention manual**.
- Give them a few minutes to review the steps described in the PM+ intervention manual.
- Answer any questions they may have.

Tell trainees (5 minutes)

- Present a summary of the steps in the *Key concepts* box and ensure that trainees all understand it. Remind trainees that they saw these key concepts in the previous role-play.

KEY CONCEPTS

Steps for conducting an assessment

1. Introduce yourself.
2. Tell the client why they are having an assessment and let them know what to expect. Explain that you will ask questions about their current experiences and psychological difficulties, but they do not have to answer if they are uncomfortable.
3. Tell the client about confidentiality. This will help them feel more comfortable when sharing personal information.
4. Check that the client is in agreement with what they have been told to help the client feel in control of the process.
5. Give brief information about PM+. Be honest about what the client can expect from PM+, what the benefits may be and the time expectations.
6. Begin the assessment with an open question such as "Would you like to begin by telling me some of the main difficulties you are seeking help for?"
7. Ask the specific assessment questions.

3.2 Trainees role-play using the during-intervention PSYCHLOPS and post-intervention assessments (50 minutes)

Learning outcome: trainees practise using the PSYCHLOPS and post-intervention assessment questionnaires.

Organize trainees (5 minutes)

- Explain that the trainees will now practise using the PSYCHLOPS and post-intervention assessment questionnaires.
- Ask the trainees to form pairs and give each pair the scenario from Annex 8.
- They should both read the scenario, but one trainee should prepare to play the PM+ helper and the other the client. The person playing the client can use the scenario for character inspiration.

Run the trainee role-play (10 minutes)

- Tell the trainees that the client is returning for PM+ Session 2. Therefore, a relationship has already been established, and the client has started to engage with PM+.
- Ask the trainees playing the PM+ helper to start the assessment and to ask the questions from the PSYCHLOPS assessment (**Appendix B in the PM+ intervention manual**).

- Tell them that, although they will be reading questions out, they should still pay attention to the client, especially if the client becomes distressed. They should use their basic helping skills and may need to take a break from the script at times.
- Tell them they have 10 minutes for the role-play.

Run the trainee role-play (10 minutes)

- Ask the trainees to swap roles.
- This time, using the same scenario from the previous role-play, the pair should imagine that they are at the end of the PM+ intervention.
- Give trainees a few minutes to familiarize themselves with the PM+ post-intervention assessment questionnaire **(Appendix C of the PM+ intervention manual).**
- They will have 10 minutes to role-play.
- People playing the role of the PM+ helper should ask questions from the post-intervention assessment.

Facilitate a discussion (25 minutes)

- Bring the group back together and discuss the role-plays.
- You can use the following question prompts:

Questions for the helper administering an assessment

- What was your experience asking the series of assessment questions?
- Are there ways you could change the delivery to make it feel more natural?
- How easy or hard did you find scoring the person on their functioning and level of emotional distress?
- What could you do to make yourself more confident using the assessments?

Questions for the person role-playing the client (being assessed)

- How did you feel being asked those questions?
- Learning from that experience, what could you do to improve the experience for your future clients?

Lesson 4

Coping with challenges during an assessment 40min

Prepare in advance

Trainees will need **Chapter 4 of their PM+ intervention manual** for Exercise 4.1.

4.1 Challenges during assessments (40 minutes)

Learning outcomes: trainees can identify why people may feel uncomfortable during assessments, know how to support them, and see two challenging assessments.

Organize trainees (5 minutes)

- Go to **Chapter 4 of the PM+ intervention manual** and specifically the scripts given for *Challenges during assessments*.
- Give the trainees a few minutes to review the scripts.
- Ask for two trainees to volunteer as PM+ helpers for a role-play in front of the group. Their task is to try to complete the pre-intervention assessment or PSYCHLOPS assessment as you role-play one client who talks too much and a second client who is non communicative.

Run the demonstration role-play (25 minutes)

- Tell the volunteers that it is important that they have all the assessment questions answered, but they may decide to leave some until the very end of the session when the client feels more comfortable or has more trust in the PM+ helper.
- Allow the first role-play to run for 5–10 minutes. During this role-play, be a client who is very talkative.
- At the end of this role-play allow 5 minutes for reflections from the trainees.
- Run the second role-play for 5–10 minutes. During this role-play, be a client who does not feel comfortable speaking.

Facilitate a discussion (10 minutes)

- At the end of the second role-play bring the group together and ask the trainees to reflect on how the helper managed the different challenging situations.
- Which skills did they use well and what areas need more support?
- End the discussion by focusing on the strengths of the volunteers and the importance of practice.

Lesson 5

Assessing and monitoring thoughts of suicide 130min

Prepare in advance

Discussion 5.1 and 5.2 needs the trainees to have **Appendix A, B and D from the PM+ intervention manual** and the Annex 9 handout from this manual.

5.1 Group discussion on assessing thoughts of suicide (30 minutes)

Learning outcome: trainees know when and how to ask clients about thoughts of suicide.

Tell trainees (5 minutes)

- There may be times when a client experiences thoughts of suicide.
- They might wish they could fall asleep and never wake up or that their life would be taken from them. Many clients may not share this with you because they feel ashamed of having these kinds of thoughts for various reasons. But as helpers you need to be prepared in case a client does tell you this information.
- Remind the trainees that the pre-intervention assessment questionnaire and weekly PSYCHLOPS require PM+ helpers to ask the client if they have any thoughts of ending their life. Tell them that this discussion will go further into the topic.

Organize trainees (5 minutes)

- Refer the trainees to the guidance in the **PM+ intervention manual for assessing thoughts of suicide (Chapter 4. *Assessment of thoughts of suicide*).**
- Then ask the trainees to review how thoughts of suicide are assessed in the **intervention assessment questionnaires**. Tell trainees to read through all the questions in **Part 6 of Appendix A in the PM+ intervention manual.**
- Next, review and compare how thoughts of suicide are covered within during-intervention assessments. See **Appendix B, questions 4a and b of the PSYCHLOPS in the PM+ intervention manual.**
- Review the instructions that, if a client indicated 4 or 5 on question 4a, or if they have a history of suicidal thoughts or plans while in PM+, the helper should complete an assessment of thoughts of suicide.
- For all other clients, the helper can go to question 5.
- Hand out Annex 9 and review this along with the questions on assessment of thoughts of suicide from the **PM+ intervention manual.**

Facilitate a discussion (20 minutes)

Note for trainers: Trainees may have a range of experiences with, and opinions about, suicide. Trainers should remain thoughtful and try to understand trainees' perspectives while encouraging them to be open to and show empathy to people who are suffering from thoughts of suicide. Be aware that some trainees may also have personal and lived experience (e.g. personal thoughts of suicide in the past, or a family member having attempted or completed suicide).

- Ask how the trainees feel about asking people about thoughts of suicide. If they give only positive responses, ask them what can be difficult about asking people about thoughts of suicide.
- Ask about trainee's experience of working with people who have thoughts of suicide. You can use these prompts:
    - Have any trainees had experience of working with people with thoughts of suicide?
    - How did they cope?
    - What did they do?
    - What is the local perception of suicide?
- Respond to any concerns trainees may have about asking direct, clear questions and responding to a client who has a plan to end their life in the near future.
- Ask trainees to share how they might feel if their client talks to them about thoughts of suicide. Encourage them to share a range of reactions. These might include anxiety, fear, worry, discomfort, anger, or having judgemental thoughts.
- Normalize trainees' responses. You might say, "It's understandable and common for helpers to feel nervous or uncomfortable when their client talks about suicide or wishing they were dead". You could also share the feelings you had when you first encountered a client with thoughts of suicide.
- If you think (or know) your trainees are religious, you may also respectfully acknowledge that each trainee might have strong beliefs about suicide, including suicide being wrong or a sin.
- Emphasize to trainees that a big part of their role as a PM+ helper is to put aside their own thoughts, beliefs and feelings about suicide. They need to respect that suicidal thoughts are a symptom of the difficulties their client is experiencing and ensure they get the right support. This means they are expected to show compassion and support, and not be judgemental towards their client.
- If a trainee is unsure about doing this, it is important for them to feel comfortable telling you. Tell them that, if they have concerns, they can talk to you in private (say when you can do this).

5.2 Trainees explore actions to take if a client has thoughts of suicide (25 minutes)

Learning outcome: trainees become familiar with actions to take following an assessment of suicidal thoughts.

Organize trainees (5 minutes)

- Ask for four trainees to volunteer.
- Assign each volunteer a different possible client response to the assessment of thoughts of suicide questions. For instance:
  - Client 1 answers "No" to question 1: "I do not have any thoughts of ending my life."
  - Client 2 answers "Yes" to question 1 but "No" to question 3: "I do think that it would be easier for everyone else if I were dead, but I do not have an plans or ideas about how I would end my life."
  - Client 3 answers "Yes" to question 1, "No" to question 3 and reports that they have previously tried to end their life: "I am having thoughts about ending my life. I don't really have a plan at the moment but in the past I have tried."
  - Client 4 answers "Yes" to question 1 and "Yes" to question 3: "I am scared as I am having very strong thoughts of ending my life. I also know exactly how and when I can do it."
- Explain that you will ask each volunteer to give their line and then you will demonstrate how a PM+ helper should respond.
- Ask the rest of the trainees to watch the demonstrations while reviewing Annex 9 and scripts in **Appendix A, Part 6 in the PM+ intervention manual.**

Run the demonstration role-play (5 minutes)

- Ensure that you demonstrate the different actions a PM+ helper should take for each response.

Facilitate large group discussion (15 minutes)

- Bring the whole group back together.
- Encourage the trainees to ask any questions they have and problem-solve any problems they may foresee for any of the actions.

5.3 Demonstration role-play exploring reasons to stay alive (35 minutes)

Learning outcome: trainees learn how to manage clients' thoughts of suicide by exploring reasons to live.

Prepare in advance

Role-play 5.3 could be adapted to trainee role-plays instead of a demonstration role-play if your trainees already have mental health expertise.

Tell trainees (5 minutes)

- Emphasize to trainees that, if a client expresses thoughts of suicide, it is important to continue that conversation.
- It is very common that people avoid talking or asking about suicidal thoughts because they fear that talking about the idea could lead to suicide attempts. But talking about suicide does not make suicide more likely.
- If anything, the opposite is true. Avoiding the topic leaves people feeling hopeless and helpless, and hence they are potentially more at risk.
- Tell trainees that, as PM+ helpers, they must get comfortable with talking openly about suicidal thoughts and feelings. Helpers need to show their client that they are not shocked by anything the client may say and that the situation is not hopeless. Explain that one way to do this is to explore the client's reasons for staying alive.

Run a demonstration role-play (15 minutes)

- With your co-trainer, role-play exploring the reasons to stay alive.
- Use Table 5 as prompts for your role-play.
- One trainer is the helper, the other is the client. The client will have thoughts of suicide but no plans to act on these in the near future.

TABLE 5

Questions for exploring reasons for staying alive, and the challenges to staying alive

Reasons to stay alive	Challenges to staying alive
• What is keeping you alive at the moment? • Are there any family members or friends you are staying alive for? • Are there some things that you have enjoyed in life recently? Or long ago? • Have you always felt this way? If not, what did you used to enjoy in life? • What hopes do you have for your future? (Help clients to think about solving their practical problems, reducing their emotional problems, etc.) • What if you did not have the problems you are experiencing at the moment? Would that change your thoughts about not being alive?	• What is making it hard to stay alive at the moment? • What are some of the reasons you wish to end your life?

- Within the role-play, the helper should demonstrate how to give a summary of the discussion.
- After hearing the client's responses, the helper should summarize the client's main reasons to live and not to live, emphasizing their reasons to live. Also repeat the reasons that PM+ may be helpful to the client.

For example, you might say "You are facing a number of problems right now which are making you feel like not wanting to live, but you have also said many good reasons for wanting to stay alive, including [your family, your children, wanting to help your community]. PM+ is designed to help you better manage and reduce these problems. If you stayed in PM+, and if your problems decreased, would this be a good reason to stay alive?"

Facilitate a discussion (15 minutes)

- Review the role-play, the assessment of thoughts of suicide questions and **Appendix D of the PM+ intervention manual** with the trainees. Make sure that the trainees are clear on the responses to the different scenarios.
- Emphasize that if in doubt, trainees should speak to their supervisors.
- Read the remaining parts of Appendix D. Emphasize that the same questions and response actions should be followed at all assessment sessions and throughout the PM+ intervention.
- Respond to and clarify any concerns that trainees may have.
- End the discussion by explaining that in the next activity the trainees will have an opportunity to practise asking about and monitoring thoughts of suicide.

5.4: Trainees role-play monitoring thoughts of suicide using PM+ manuals (40 minutes)

Learning outcome: trainees become familiar with a script to monitor clients with thoughts of suicide.

Prepare in advance

Print and share Module 4 role-play instructions.

Have copies of trainees' PM+ competency assessment tool.

Organize trainees (5 minutes)

- Divide the trainees into pairs and ask them to decide who will play the helper roles and who will be clients.
- One trainer will take the helpers to one side of the room, while another trainer takes the clients to another space. Ensure that the two groups cannot hear the instructions given to each other.
- Persons playing clients should be given instructions and a case scenario from Module 4 role-play instructions (at the end of this module). Tell them not to show this information to their partners playing helpers.
- Persons playing PM+ helpers should be given instructions from **Module 4 role-play** (at the end of this module).
- Direct the trainees playing helpers to use **the during-intervention PSYCHLOPS assessment (Appendix B of the PM+ intervention manual), including question 4.** Also refer all trainees to the box titled *Managing clients with suicidal thoughts in PM+* in **Appendix D of the PM+ intervention manual.**

Run the trainee role-plays (15 minutes)

- Bring the pairs together and run the role-plays.
- Tell them they have 15 minutes to complete the role-play.
- Have trainees' PM+ competency assessment tools (digital or paper) available so that each trainer can informally rate one or two role-play demonstrations. The relevant item for this role play is **PM+ 7** (assessment of harm and developing a response plan).

Facilitate a discussion (20 minutes)

- Bring the class back together and allow for approximately 10 minutes of feedback and reflection on how it felt to ask about and monitor thoughts of suicide. Provide group feedback based on your observations of the role-plays.
- Allow time to ask trainees how they personally feel after having discussed such a challenging topic. Normalize and validate the range of emotions.

- Emphasize to trainees that, if they notice ongoing distress or discomfort after having talked about suicide, they can talk to a trainer in private about this.

Note the trainees' mood at this point. You might ask trainees to reflect on how they are feeling. If the mood is particularly low, engage the trainees in an activity that will help them move on from this session. For example, an energizer might help, but avoid one that requires a lot of energy as this might be challenging for some trainees to engage with. You could just do a stretching or movement activity.

Lesson 6

Summary, home practice and setting up the review 25min

6.1 Summary (5 minutes)

Tell trainees

- Give a brief summary of the topics covered in today's session. Provide an opportunity to clarify any concepts or to answer questions.

6.2 Set home practice (15 minutes)

Learning outcomes: trainees consolidate their learning on assessing thoughts of suicide.

Tell trainees

- Ask trainees to take some time to reflect on what they have learned in this module.
- Ask them to read **Chapter 4 (section - Guidance when assessing thoughts of suicide in clients) in the PM+ intervention manual**, and to review **Appendix A and Appendix D** of the same manual, which contains the PM+ scripts and other important information for monitoring thoughts of suicide.
- Tell them to think of a question to ask about assessing and monitoring suicide in the review section of the next module.
- Tell them that, if they notice uncomfortable thoughts or feelings while doing this home practice, they can write down their thoughts and feelings and discuss them privately with a trainer on the following day.

6.3 Set up the review for next time (5 minutes)

Organize trainees

- Remind the trainees who are to facilitate the review session at the start of the next module.
- Remind them to focus on key information that they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run this review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 4 role-play
PM+ assessments

These instructions are for Module 4, Lesson 5.3 – Trainee role-plays on monitoring thoughts of suicide using PM+ scripts.

Instructions to the person role-playing the PM+ helper

Your role is to use the during-intervention PSYCHLOPS assessment (**Appendix B of the PM+ intervention manual**) to assess progress and any possible reports of self-harm/suicide. Remember to be clear and direct and ask questions about risk of harm. If they share that they are having thoughts of suicide, do the following:

- Ask the client what reasons they may have for dying and living.
- Use the assessment questions to understand if they are at risk of ending their life any time soon.
- Use the **PM+ intervention manual** to guide you.

Instructions to the person playing the role of the PM+ client

MALE SCENARIO

You are a 38-year-old refugee surviving on aid from local religious organization. You are temporarily living with friends who are abusing substances. You feel trapped and angry. You are constantly arguing with the people around you. You have been told that PM+ might help you but you do not think it will make a difference. You feel you need money more than talking sessions. You have had suicidal thoughts, and sometimes you think it would be easier for everyone else if you were to die. But you do not have any plans for how you would kill yourself. You long to reunite with your spouse and three children but it seems hopeless.

FEMALE SCENARIO

You are a 26-year-old hotel cleaner. Since violence in your community broke out two years ago, you have become withdrawn from friends and isolated at home despite living with your mother and two siblings. You fear going out at night even though there are no longer any immediate dangers. Your mother is very concerned about your depressed mood and isolation. You notice that you cannot concentrate on reading and doing things you used to enjoy. You feel tired and just want to sleep for days. You have never told anyone before, but you once attempted to end your life by taking lots of medication and alcohol. Nothing happened except vomiting. That was a year ago and although you constantly think about escaping from your lonely life, you do not have plans to end your life. You want help but you worry about work and questions from your mother.

CLIENT PROMPTS

If the PM+ helper asks you to describe your feelings or thoughts, or if they ask you if you have any thoughts of self-harm/suicide, say “I’m so sad all of the time, it seems that maybe it isn’t worth being alive most days”.

If the PM+ helper asks you to list reasons for living, say that you want to keep living for your family as you do not want to make them worried or sad. Explain that you want to die because you want to escape the loneliness that you are feeling. If asked about plans to end your life in the next two weeks, say you do not have plans and do not want to die as it will hurt your family, say you are just feeling hopeless and suffering.

Module 5

Understanding adversity and the PM+ intervention 5h

No.	Description	Learning outcomes	Advance preparation
Lesson 1. Review and introduction to the module (20 minutes)			
1.1	**Review (15 minutes)**	Trainees revise their knowledge on PM+ assessments and practise facilitating a review session	
1.2	**Introduction to Module 5 (5 minutes)**	Trainees know what to expect from Module 5	Prepare a flipchart or slide with PM+ Session 1 aims (Table 6)
Lesson 2. Teaching PM+ Session 1, What is PM+? (130 minutes)			
2.1	**Small group discussion. What is PM+? (20 minutes)**	Trainees practise explaining what PM+ is to clients	Score sheet for information that helpers should deliver when explaining PM+ to a new client
2.2	**Demonstration activity. Reasons for and challenges to joining PM+ (20 minutes)**	Trainees understand the need to explore a client's motivation towards PM+	Have an ordinary everyday object
2.3	**Group discussion. Introducing motivational interviewing techniques (15 minutes)**	Trainees explore how to motivate people to engage in PM+	Print and share copies of the table Good and less good reasons for joining ***PM+*** **(Chapter 5 in the PM+ intervention manual)**
2.4	**Demonstration activity. Motivating clients to join PM+ (40 minutes)**	Trainees see the motivational interviewing technique being used	
2.5	**Trainees role-play discussing reasons for, and challenges to, joining PM+ (35 minutes)**	Trainees practise using the motivational interviewing technique	

No.	Description	Learning outcomes	Advance preparation
Lesson 3. Teaching PM+ clients how to understand adversity (120 minutes)			
3.1	**Small group discussion. What is adversity? (40 minutes)**	Trainees can name common reactions to adversity	Print the scenario in Annex 10
3.2	**Demonstration role-play. How to deliver "Understanding adversity" (30 minutes)**	Trainees see how to deliver PM+ information on adversity	Print and share copies of culturally adapted case scenarios for the clients (Annex 11)
3.3	**Trainees role-play Individualizing "Understanding adversity" (50 minutes)**	Trainees practise giving individualized responses to adversity	
Lesson 4. Summary, setting home practice and setting up the review (30 minutes)			
4.1	**Summary (15 minutes)**	Trainees can ask any remaining questions Trainees reinforce their knowledge of explaining adversity and the PM+ intervention	
4.2	**Set home practice (5 minutes)**		
4.3	**Set up the review for next time (10 minutes)**		

Lesson 1

Review and introduction to the module 20min

Prepare in advance

Flipchart or slide outlining the aims of PM+ Session 1, and how these are achieved.

1.1 Review (15 minutes)

Learning outcome: trainees revise their knowledge on PM+ assessments and practise facilitating a review session.

Organize trainees

- Nominated trainees lead the review session (up to 10 minutes).
- The trainer answers questions or clarifies misinformation about the previous module's content.
- They should practise creating a safe space where the other trainees feel comfortable at sharing their concerns and thoughts about assessing and managing clients with thoughts of suicide.
- As different concerns and questions are shared, the volunteer trainees should seek to create consensus on ways to manage thoughts of suicide, and on ways to make the actionable steps as culturally appropriate as possible. This should include where and how to refer people with thoughts of suicide.
- Either invite two new trainees to volunteer to lead the next review session, or choose a pair of trainees (e.g. trainer or trainees who complete this review can nominate).

Facilitate a discussion

- If the trainee facilitators have missed something, spend a few minutes reviewing it yourself.
- Take a few moments to ask about the trainees' mood following discussions on suicide. If the mood is low choose an ice-breaker or energizer from Annex 4.

1.2 Introduction to Module 5 (5 minutes)

Learning outcome: trainees know what to expect in Module 5.

Tell trainees

- Tell trainees that the training will now focus on teaching them how to deliver PM+ sessions, starting with Session 1. There are four components to PM+ Session 1, namely:

  - Explaining what PM+ is, and encouraging the client to engage with the programme.
  - Discussing adversity and people's responses to it, in an individualized way.
  - Managing stress.
  - Setting up home practice.

- This module will focus on learning how to explain PM+, how to engage clients with the programme and how to discuss adversity.
- Module 6 will cover the details of teaching the PM+ stress management technique and practising it at home.
- Show Table 6 on a pre-prepared flipchart or slide. Describe the aims of the first PM+ session and how to achieve those aims.

TABLE 6

Session 1 aims, and how they are achieved

PM+ Session 1 aims	How aims are achieved
Build initial rapport with clients to aid engagement with PM+.	• Use of basic helping skills. • Use of motivational interviewing technique (Reasons for, and challenges to, joining PM+).
Improve clients' knowledge of adversity and understanding of how PM+ can assist them.	• Explain the impact of adversity on an individual. • An individualized script on adversity, and how people react to it. • Client receives an explanation of PM+.
Equip clients with a strategy to manage physical arousal related to mental health problems.	• Teaching and rehearsing the Managing Stress technique.

Lesson 2

Teaching PM+, Session 1, What is PM+? 130min

Prepare in advance

Trainees need their copies of the PM+ intervention manual. You need enough flipcharts and markers to give to several small discussion groups in Lesson 2.1.

2.1 Small group discussion. What is PM+? (20 minutes)

Learning outcome: trainees practise explaining what PM+ is to clients.

Organize trainees (3 minutes)

- Divide the trainees into small groups and give each a flipchart and marker.
- Direct the groups to the **PM+ intervention manual, Chapter 5,** *Helping the client to understand and participate in PM+*. Read the helper's script.

Run the small group activity (15 minutes)

- Read the script from the **PM + intervention manual that describes PM+** (trainees can read along with you or just listen).
- Then ask the trainees to close their manuals and place them under their chairs.
- In their small groups, tell the trainees to write down all the important information covered in the text. Ask the group to highlight what they think is the most important information for the client to know. Allow 10–15 minutes for this activity.
- At the end, give each group the score sheet or show it as a slide, and ask them to score their flipcharts.

Score sheet for information that helpers should deliver when explaining PM+ to a new client

Item	Score
My (helper's) name (1 point)	
My role (1 point)	
Everything shared will be kept private (1 point)	
Private except for my supervisor (1 point)	
Private except for harm to you (the client) (1 point)	
We will learn strategies to help you overcome difficulties (1 point)	
I have mentioned the client's own specific problems as part of the statement (1 point)	
These strategies have helped many people in situations similar to your own (1 point)	
There will be five sessions (1 point)	
This session is included in the five sessions (1 point)	
We will meet once per week (1 point)	
Each session will last about 90 minutes (1 point)	
Between our meetings I will ask you to practise the strategies (1 point)	
PM+ can help you to manage practical problems (1 point)	
PM+ can help to improve your activity (1 point)	
PM+ can also help you to reduce your stress (1 point)	
PM+ will work best if you attend every session (1 point)	
Please contact me if you are having difficulty attending sessions rather than avoiding me or not turning up (1 point)	
I will not be angry or upset if you are having difficulties in attending sessions (1 point)	
Do you think you will be able to come to all the sessions? (1 point)	
Total number of points = 20	

Tell trainees (2 minutes)

- It is crucial that clients understand this information when setting up the first PM+ session so that the client is aware of what is involved in the programme.
- At this stage helpers do not need to go into details about the PM+ strategies.
- Explain that, while PM+ helpers could read this script directly from the manual to the client, it is best if they practise delivering it in a more natural way.

2.2 Demonstration activity. Reasons for, and challenges to, joining PM+ (20 minutes)

Learning objective: trainees understand the need to explore a client's motivation towards PM+.

Prepare in advance

You will need an everyday object. Something not quite perfect is best, such as a blunt pencil, a cup with a hole or chip in it, a scruffy bag, etc.

Tell trainees (2 minutes)

- This demonstration activity lets the trainees see and reflect on how we motivate people. It is relevant for discussing reasons for and challenges to joining PM+.
- Explain that, to understand a client's motivation, helpers should explore why the client wants to join PM+ as well as what may make it difficult for them to be part of the intervention.

Organize trainees (3 minutes)

- Ask for a volunteer.
- Sit facing the volunteer in front of the rest of the group.
- Give the volunteer the ordinary object.

Run the demonstration role-play (5 minutes)

- Tell the volunteer that they should try to convince you, in any way possible, that you should keep this object. Give the volunteer a minute to do this.
- After a minute, ask for another volunteer to persuade you in a different way. Repeat this with about three or four volunteers.

Facilitate a discussion (10 minutes)

- Start by asking "What strategies did the volunteers use to get the client to want their object?"

  "Did anyone ask the client how they felt about the object?"

  "Which strategies worked, and which did not?"

- Ensure that the discussion covers the following points:
  - Highlight that the volunteers probably focused on the positive aspect of the object, saying how much they liked it, begging, bribing, etc.
  - Reflect on how those strategies make the person they are trying to convince feel? What impact does it have?
  - Explain that most people will try to convince a person by telling them all the desirable characteristics of the object. These might be true. But if the person does not feel listened to, and their reasons for not wanting the object are ignored or argued with, they are likely to be more defensive and want the object less.
  - Encourage the trainees to think about how this is relevant for discussing PM+ with clients.

2.3 Group discussion. Introducing motivational interviewing techniques (15 minutes)

Learning outcome: trainees explore how to motivate people to engage in PM+.

Prepare in advance

For this lesson you will need copies of the table Good and less good reasons for joining PM+, from **Chapter 5 of the PM+ intervention manual** (or the trainees can use their manuals).

Organize trainees (5 minutes)

- Tell trainees to find **Chapter 5 of the PM+ intervention manual** and the table describing good and less good reasons for joining PM+. It is also available as **Appendix G, Protocol 1, Session 1**.

Tell trainees (10 minutes)

- In the first session, the PM+ helper will ask some of the questions in the table in order to explore what advantages and disadvantages there are for joining PM+.
- Read and review the table with the trainees. Explain that, for many clients, PM+ may not be a very desirable programme to join because of several reasons. For instance:
  - It takes up time.
  - It does not provide medications or money.
  - Talking about emotional problems might be very unusual.
- For many people, PM+ might be a new concept of treatment and therefore it may be difficult for them to give you their reasons for joining at all.

- Explain that this table is using a particular technique to help clients really engage in the programme. There are three components involved in the technique, namely: discussing the challenges of joining; the benefits of joining; and then summarizing, with emphasis on the reasons to join (see the *Key concepts* box).

KEY CONCEPTS

The motivational interview technique

Discuss the challenges to joining the programme and be very understanding of these challenges.

Discuss why joining the programme might be really good for the client. Emphasize that these reasons might be more important than the challenges.

Summarize the main challenges and reasons regarding joining PM+, making sure to emphasize the reasons to join.

2.4 Demonstration activity. Motivating clients to join PM+ (40 minutes)

Learning outcome: trainees see the motivational technique being used.

Organize trainees (1 minute)

- Ask for another volunteer to role-play with you.
- Explain that you are going to show how to use the motivational technique to encourage the volunteer to want the less-than-perfect everyday object from the earlier activity.

Run the demonstration role-play (10 minutes)

- Talk with the volunteer about their reasons for not wanting the object. Discuss the reasons and, if possible, solve them.
- Help the person come up with their own reasons for wanting the object (i.e. do not tell the person why they want the object).
- Summarize all the reasons for wanting and not wanting the object, emphasizing the reasons for wanting the object.

Facilitate a discussion (25 minutes)

- Ask the trainees to reflect on what they have seen. Use these prompts to encourage more reflection:
    - What happened in the role play to motivate the person to join PM+?
    - How can it be used again when clients are having difficulty staying engaged in PM+ (e.g. if someone has stopped coming to the group or is not practising the strategies between sessions or always comes late or leaves early)?

  - How can it be used with someone who has suicidal thoughts, but not a strong desire to end their life (it can explore their challenges and reasons to live)?

- Ask the trainees how basic helping skills could be used to engage a client. Give them a few moments to reflect, then ask them for suggestions. These might include:
  - Basic helping skills show understanding of a client's uncertainty about joining PM+ and validate those feelings rather than being judgemental.
  - Verbal communication can reflect to the client the good and less good reasons so that they can make a clearer decision about whether to join PM+.
  - Praising openness can identify personal strengths in the client in order to improve their confidence in engaging in the treatment.
  - Good basic helping skills also model a collaborative and equal relationship between client and helper.

Tell trainees (4 minutes)

- Finish the discussion by explaining that, even when using these techniques, not everyone is going to be motivated to engage with PM+.
- It is not a helper's job to motivate every client to join PM+.
- There is no negative judgement if someone decides not to join PM+.
- The client always has the choice not to continue with PM+.
- It can be unhelpful for clients to begin PM+ when they are not motivated (e.g. they might cancel appointments, drop out of the intervention, and decide not to see their helper again).
- After this discussion, the helper might recognize that PM+ is not suitable or feasible for the client at this time (perhaps due to obstacles, other significant priorities, etc.). The helper should feel comfortable when giving this opinion to the client. The client may return to seek treatment at a later stage.

2.5 Trainees role-play discussing reasons for, and challenges to, joining PM+ (35 minutes)

Prepare in advance

Trainees will need to use the table *Good and less good reasons for joining PM+*, from **Chapter 5 of the PM+ intervention manual**.

Learning outcome: trainees practise using the motivational interviewing technique.

Organize trainees (5 minutes)

- Divide trainees into pairs and tell them to refer to the table on "Good and less good reasons for joining PM+".

- One trainee should play the helper and one the client, and they should explore whether or not the client will join PM+. After about 10 minutes, the pair should swap roles so that both have a chance to play the helper.
- Ask the trainees to imagine their own scenario when role-playing the client, or to use the scenarios from previous role-plays. They could talk about PM+ not being important to them right now, and all the other significant challenges they might have that would get in the way of their attendance at PM+ sessions.

Run the trainee role-plays (20 minutes)

- Start the first role-play.
- Walk around during the role-plays and observe the trainees.
- After 10 minutes ask the trainees to swap roles and start the role-play again.

Facilitate a discussion *(10 minutes)*

- Bring the full group back together.
- Ask for reflections on how it felt to ask about people's motivations in this way.

Lesson 3
Teaching PM + clients how to understand adversity 120min

Prepare in advance

Ensure that each trainee has a copy of the **PM+ intervention manual** so they can read the information in **Chapter 5** and in **Appendix G**.

Trainees will also need Annex 10 from this document (a scenario for small group discussions on "What is adversity?"), and the question prompts for discussing adversity. The question prompts could be displayed as large posters that everyone can see, or as handouts for small group discussions.

3.1 Small group discussion: What is adversity? (40 minutes)

Learning outcome: trainees can name common reactions to adversity.

Tell trainees (2 minutes)

- When we talk about adversity, we are referring to times in people's lives when they are experiencing difficult, extremely challenging or stressful situations or circumstances.

- Give examples and also ask trainees for examples (this will help you to gauge whether they understand).

Organize trainees (3 minutes)

- Divide the class into small groups.
- Ask each group to discuss the following questions (make sure they can see these questions):
  - What adversity are people in the local community facing?
  - How do people react to this adversity? Consider their feelings, physical reactions, their behaviours, thoughts or ways in which they relate to others.
  - How do people's reactions differ?
  - What do people who experience adversity need to know?

Facilitate small group discussions (25 minutes)

- Allow the groups to discuss these questions for 15 minutes.
- Move around the groups and ensure that the discussions cover the following points.
  - The emotions people feel when facing adversity can be common, but people express these in individual ways. For example, when someone dies, people might grieve differently depending on their personality, situation and their relationship to the person.
  - Common reactions include feelings of depression, anxiety and stress and the associated symptoms discussed in Module 2 (use the lists generated in Module 2 if required).
  - People need to know that their reactions are not unusual and that they can talk openly about their feelings.
  - Emphasize to trainees that using basic helping skills like praising openness and validating feelings will make the client feel more heard and understood, and this can give them comfort.
- Stop the discussion after 15 minutes.
- Then ask trainees to read the suggested scripts for talking about adversity from **Chapter 5 of the PM+ intervention manual**. These are also found in **Appendix G under the heading "Understanding adversity"**.

Organize trainees (10 minutes)

- Now give each trainee Annex 10 as a handout and ask them to read it. It gives a scenario and an example response that is personalized for the client.
- Give the trainees another 5–10 minutes to discuss these responses in their groups.
- Walk around the room listening to the reflections. Be available to answer any questions the trainees may have and/or prompt further reflection.

3.2 Demonstration role-play. How to deliver "Understanding adversity" (30 minutes)

Learning outcome: trainees see how to deliver PM+ information on adversity.

Prepare in advance

Print and share Annex 11.

You and your co-trainer will need to be familiar with the scenario from Annex 11 for this demonstration role-play.

Organize trainees (5 minutes)

- Work with a co-trainer or a volunteer to role-play a helper talking with a client and individualizing the adversity script from the PM+ intervention manual.
- The person playing the role of the PM+ client should use the scenario given in Annex 11. You can share that case scenario with all the trainees before you start the role-play.
- Ask the rest of the trainees to observe how you individualized the script.

Run demonstration role-play (10 minutes)

- You are demonstrating how to be an effective PM+ helper, so ensure that you spend time normalizing the client's feelings and their responses to the adversity they face.

Facilitate a discussion (15 minutes)

- Ask trainees to reflect on the role-play. Use prompts such as:
  - How did the PM+ helper individualize the information about adversity?
  - When might it be difficult to individualize the script?
  - How could you overcome those challenges?
- Finish the discussion by reminding the trainees that it is important to practice and adapt the scripts, so their own communication styles are included and it feels natural to them.

3.3 Trainees role-play individualizing "Understanding adversity" (50 minutes)

Learning outcome: trainees practise giving individualized responses to adversity.

Organize the trainees (5 minutes)

- Ask the trainees to return to the same pairs they were in for the reasons for, and challenges to joining PM+ role-play.

- Tell them to take turns playing the PM+ client and the PM+ helper. They should use a scenario from a previous role-play (Annex 11).
- Ask them to practise individualizing how they deliver the **"Understanding adversity" scripts in the PM+ intervention manual.**

Run the trainee role-plays (30 minutes)

- Run the first role-play for 10 minutes.
- After the role-play, trainees have a few minutes to give feedback.
- Then ask the trainees to swap roles. Again, allow time for feedback after the role-play.
- Walk around the room and listen to as many pairs as you can.

Facilitate a discussion (15 minutes)

- Bring the full group back together.
- Ask the trainees to share their reflections.
- Share any of your own reflections.

Lesson 4

Summary, setting home practice and setting up the review 30min

4.1 Summary (15 minutes)

Tell trainees

- Give a brief summary of the topics covered in Module 5.
- Provide an opportunity to clarify anything that trainees raise and to answer questions.

4.2 Set home practice (5 minutes)

Learning outcome: trainees reinforce their knowledge of explaining adversity, and the PM+ intervention.

Tell trainees/explain to trainees

- Ask trainees to complete any reading for the previous modules.
- Ask trainees to re-read **Chapter 5 and Appendix G in the PM+ intervention manual.**

4.3 Set up the review for next time (10 minutes)

Organize trainees

- Remind trainees who are to facilitate the review session at the start of the next module.
- Remind them to focus on key information they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run this review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 6

Managing stress 3h25min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review and introduction (20 minutes)			
1.1	**Review (15 minutes)**	Trainees can recall knowledge from Module 5 and practise facilitating a review session	Use the pre-prepared flipchart/slide with the outline of PM+ Session 1 aims
1.2	**Introduction (5 minutes)**	Trainees know what to expect of Module 6	
Lesson 2. Managing stress in PM+ (110 minutes)			
2.1	**Recognizing stress (15 minutes)**	Trainees understand that stress can have physical as well as emotional effects	
2.2	**Breathing activity and discussion (25 minutes)**	Trainees learn how the breathing rate affects the body	Use **Appendix G: PM+ intervention protocol**, Session 1, Managing stress
2.3	**Trainees role-play managing stress (40 minutes)**	Trainees practise delivering the stress management strategy	Print and adapt the Module 6 role-play Have copies of trainees' PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 8**
2.4	**Helping clients who are having problems engaging with slow breathing practice (30 minutes)**	*Trainees see and discuss how to use basic helping skills to support clients who are having problems engaging with managing stress*	
Lesson 3. Using home practice in PM+ (20 minutes)			
3.1	**Explaining home practice for PM+ clients (20 minutes)**	Trainees know the importance of setting home practice	

No.	Description	Learning outcome	Advance preparation
Lesson 4. Summary, setting home practice and setting up the review (55 minutes)			
4.1	**Group discussion. Pulling together PM + Session 1 (15 minutes)**	Trainees revise how to deliver PM+ Session 1	
4.2	**Optional: Trainees role-play pulling together a PM+ Session 1 (35 minutes)**		If using the role-play, take the case scenarios from any of the previous role-plays, or ask trainees to think of their own Have copies of trainees' PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 8**
4.3	**Set home practice (5 minutes)**	Trainees practise the Managing Stress strategy and revise delivering elements of Session 1	

Lesson 1

Review and introduction 20min

Prepare in advance

Have ready the prepared flipchart or slide with the outline of the PM+ Session 1 aims that you used in Module 5.

1.1 Review (15 minutes)

Learning outcome: trainees can recall knowledge from Module 5 and practise facilitating a review session.

Organize trainees

- Nominated trainees lead the review session (up to 10 minutes).
- The trainer answers questions or clarifies misinformation about the previous module's content.
- Either invite two new trainees to volunteer to lead the next review session, or nominate a pair of trainees.
- The trainer then leads a reviews of the trainee's home practice: how to individualize scripts and talking about adversity.

1.2 Introduction (5 minutes)

Learning outcome: trainees know what to expect of Module 6.

Tell trainees

- Remind the trainees they are still learning how to deliver Session 1 of PM+.
- Present the core components of Session 1, highlighting the topics to be covered in this module:
    - What is PM+?
    - What is adversity?
    - **Managing stress.**
    - **Setting up home practice.**
- Say that, after explaining PM+ and individualizing adversity, helpers teach the PM+ clients how to understand the effect of stress on the body, how to manage stress, and the importance of home practice.
- Show again the flipchart or slide you prepared for Module 5, displaying Table 6**. Session 1 aims, and how they are achieved.**

Lesson 2

Managing stress in PM+ 110min

Prepare in advance

Trainees will need their **PM+ intervention manual**.

You will need to be familiar with the slow breathing exercise described in **Chapter 6 of the PM+ intervention manual** and you need to be able to time one minute for the breathing exercise in Lesson 2.2.

2.1 Recognizing stress (15 minutes)

Learning outcome: trainees understand stress can have physical as well as emotional effects.

Tell trainees

- Read the subsection called Background in **Chapter 6 of the PM+ intervention manual.** Give them time to do this.
- Explain that, as well as emotional changes, many people can experience physical symptoms when they have faced adversity.
- Physical symptoms of stress can include shallow breathing, racing heart, chest pain, dizziness, headaches, muscle tension and tiredness. These symptoms often result from breathing too quickly.
- Explain that managing stress is a breathing exercise that helps reduce stress. It is taught in Session 1 and practised at the end of every session.
- Consider giving an example. You might explain that someone who has to look after children but is also facing adversity may not sleep well. That can make them physically and mentally less able to concentrate on looking after the children. By using the breathing activity to reduce stress, the person may have more energy and be able to attend better to their daily tasks.

2.2 Breathing activity and discussion (25 minutes)

Learning outcome: trainees learn how the breathing rate affects the body.

Tell trainees (4 minutes)

- The aim of this exercise is to explore what it feels like to express stress as shallow and rapid breathing, and how slowing breathing can affect the body.
- To do this we will show the effects of breathing at three different rates.
- Tell trainees they will count how many times they breathe in and out over a minute.
- Tell them they will do this three times at different rates.
- One breath is counted as one in and one out.

Organize trainees (1 minute)

- Ask trainees to stay seated.
- Ensure you can see a clock and time 1 minute.

Run the activity (5 minutes)

- **First round**: Set a timer for 1 minute and ask the trainees to count how many breaths they took. Record the numbers.
- **Second round:** Set a timer for another minute. You and the trainees now quicken your breathing (breathe fast and shallow). Warn the trainees they may feel like they are doing intense exercise, or they may feel panicky. Emphasize that this is not dangerous and that the body is designed to cope. Make sure you do the breathing with them, so they do not feel silly. After a minute, ask them how many breaths they took, and how they feel.
- **Third round:** Lead the slow breathing exercise as described in **Chapter 6 and in Appendix G of the PM+ intervention manual**. The trainees should count their breaths again.

Facilitate a discussion (15 minutes)

- Use the numbers you have recorded to compare the rates of breathing between the three rounds, and between trainees.
- Explain that a calm rate of breathing is anything under 13 breaths per minute. When people breathe too fast and too shallow (hyperventilate), they are usually breathing above 23 breaths per minute.
- Discuss how it felt to breathe at different rates and ask what symptoms the trainees noticed.
- Especially discuss how they felt when trying to breathe slowly after breathing very fast, and how they felt when they did manage to slow their breathing down.
- Ask the trainees to think how they could manage a situation where a client began showing stress by breathing too fast. Also ask them how they could explain the beneficial effects of slowing breathing in order to manage stress.
- Finish the discussion by recognizing that often trainees (and in the future clients) are unsure how breathing can help with the very complex problems that clients may have. Use the information in the Key concepts box, along with your own experience, to explain the benefits.

KEY CONCEPTS

Slow breathing for managing stress

The aim of the breathing exercise is not to make problems magically go away, but to give clients ways to cope better when they are stressed.

When we are stressed our "fight or flight" mode is activated and our breathing speeds up. If we are not actually running away from a threat then rapid breathing can be more than is needed (we can start to hyperventilate). Breathing too fast for the situation alters the balance of gases in our bloodstream. This can lead to symptoms such as dizziness, feeling faint, numbness and even apparent breathlessness.

Calming the breath can balance out those gases and signal to the body that a high state of alert is not required. This can help manage the physical symptoms of stress.

2.3 Trainees role-play managing stress (40 minutes)

Learning outcomes: trainees practise delivering the stress management strategy.

Prepare in advance:

Print and share copies of Module 6 role-play

Provide copies of the PM+ competency assessment tool to trainees.

Organize trainees (3 minutes)

- Divide trainees into pairs and give them the instructions and prompts in the Module 6 role-play (found at the end of this module).
- Ask them to take turns in leading the slow breathing exercise for managing stress.
- Tell trainees to give each other feedback at the end of their practice session, and remind them they should be following the steps they learned for giving helpful feedback.
- Give them enough time to read the instructions in their PM+ intervention manual before you start the role-play.

Run trainee role-plays (20 minutes)

- Allow 10 minutes for the first role-plays.
- Then ask the trainees to swap roles and run another role-play.
- Observe some of the role-plays and informally rate trainees on their PM+ competency assessment tool (digital or paper). The relevant item for this role-play is **PM+ 8** (introducing and explaining slow breathing).

Facilitate a discussion (15 minutes)

- At the end of the role-plays, bring the group back together and give some generalized feedback, based on your informal ratings.
- Ask trainees to identify the key points they believe are important for teaching the management of stress. Make sure they cover the following points.
  - Give instructions slowly.
  - Give instructions in chunks (e.g. diaphragmatic breathing and slow breathing) and allow time to demonstrate the skill.
  - Have clients practise chunks of instruction (e.g. diaphragmatic breathing and slow breathing) for at least two minutes.
  - Encourage clients to practise managing stress daily at home.

Tell trainees (2 minutes)

- At the beginning of each training day (or at appropriate intervals, such as meal breaks, and depending on the trainees' motivation to practise) volunteers can take turns at leading the group in slow breathing, so they have a chance to practise teaching and leading clients.

2.4 Helping clients who are having problems engaging with slow breathing practise (30 minutes)

Learning outcome: trainees see and discuss how to use basic helping skills to support clients who are having problems engaging with managing stress.

Facilitate a discussion (10 minutes)

- Ask the trainees how they would respond if PM+ clients tell them they are too busy to try the managing stress strategy, or clients found it too difficult, or they do not think it is enough to help them.
- Then review and discuss the helpful hints (**Chapter 6 on Managing Stress in the PM+ intervention manual and Appendix G PM+ Intervention Protocol Session 1 Managing Stress**) for responding to common problems clients have with managing stress.
- Encourage the trainees to think about how they would engage and motivate a reluctant client.

Organize the trainees (3 minutes)

- Ask for a volunteer to role-play a client.
- Ask them to pick one or more of the common problems people have with the slow breathing exercise from the table at the end of **Chapter 6 of the PM+ intervention manual**.

Run a demonstration role-play (10 minutes)

- You play the role of the PM+ helper.
- Role-play how to use basic helping skills to work through the client's problems.
- You can use ideas from the same table. Work with the client's worries and concerns and motivate them to engage with the strategy.

Facilitate a discussion (7 minutes)

- Ask the group for their reflections on the role-play.
- Ask which basic helping skills the helper used to support the client.
- Open up the discussion to answer any questions trainees have on the different problems/concerns that clients might have with this strategy.
- Use the following questions as prompts:
  - How would you respond if a PM+ client says they are too busy to try the strategy on managing stress, or that it is too difficult?
  - How would you respond if clients say they don't think the breathing exercise is enough to help them?
  - Which basic helping skills and motivating techniques might help a reluctant client?
- Brainstorm ideas for how trainees could manage these situations more confidently.
- Ask whether they themselves have reservations about the stress management strategy and talk through any concerns.

Lesson 3
Using home practice in PM+ 20min

3 .1 Explaining home practice for PM+ clients (20 minutes)

Learning outcome: trainees know the importance of setting home practice.

Tell trainees (5 minutes)

- Tell trainees that an important element of PM+ is the client practising each of the strategies between the sessions – called home practice. Cover the points in the *Key concepts* box.

KEY CONCEPTS

Home practice

Home practice puts the strategies into real-world situations where clients can use them to improve their mental health and their practical problems. More practice in the real world means more improvements in mental well-being.

Practice makes implementing the strategies easier. For example, slowing down your breathing can be hard initially but with practice people will improve.

Practising the strategies without the helper gives a client an opportunity to experience difficulties with the strategy that can then be managed in the following session.

Successfully practising without the helper can improve the client's confidence and self-esteem.

- Tell trainees they will be encouraging their clients to practise the strategies between sessions as much as possible. Each strategy might require a different amount of practice.

Facilitate a discussion (15 minutes)

- Start the discussion by explaining that sometimes clients can find it difficult to do home practice.
- Ask trainees why clients might struggle with home practice. Make sure the discussion covers the following points:
    - Clients may have a lot of responsibilities (e.g. work, looking after children) that make it hard for them to prioritize home practice.
    - Clients might simply forget.
    - A client's poor mental health might make it hard for them to practise the strategies outside of the session. For example, clients with depression might not feel motivated to practise; and clients with anxiety might feel too scared to practise.
- Ask trainees to suggest ways to help clients complete their home practice. Make sure the discussion covers the following points:
    - Review the exercise on *Good and less good reasons for joining PM+* and adapt it to include reasons for, and challenges to, completing home practice.
    - Tell clients that experience suggests that the more home practice clients do between sessions, the greater the improvements they make by the end of the programme.
    - Help clients to manage any practical problems that may be getting in the way of completing home practice.
    - Help clients to set up reminders to practise the strategies.

Lesson 4

Summary: setting home practice and setting up the review 60min

4.1 Group discussion. Pulling together PM+ Session 1 (15 minutes)

Learning outcome: trainees revise how to deliver PM+ Session 1.

Tell trainees

- Summarize how all the components discussed in Modules 5 and 6 come together to form PM+ Session 1. The elements, and their timings, are the following:
  - introductions and confidentiality (5 minutes);
  - during-intervention PSYCHLOPS assessment and general review (10 minutes);
  - understanding what PM+ is (20 minutes);
  - understanding adversity (30 minutes);
  - the Managing Stress technique (20 minutes);
  - setting home practice and ending the session (5 minutes).
- Acknowledge that there is a lot to remember in Session 1, but say it is critical to get it right. The way the clients experience Session 1 will influence whether or not they will continue with PM+ for the full course.
- Refer trainees back to the PM+ intervention manual **Chapters 4, 5 and 6 and Appendix G** to re-read any information they want to cover.

4.2 Optional – Trainees role-play pulling together a PM+ Session 1 (35 minutes)

competency in delivering the core components of Session 1.

Organize the trainees (5 minutes)

- In pairs ask the trainees to practise delivering PM+ Session 1 as a role-play.
- Tell the trainees to focus on the “What is adversity?” and the “Managing Stress” sections of PM+ Session 1.
- Ask the trainees to use and adapt scenarios in Annex 10 and Annex 11 or encourage trainees to think of people they know who are living with adversity and showing stress.

Run a trainee role-play (20 minutes)

- Allow trainees to role-play for about 20 minutes.
- Informally rate one or two trainee helpers using their PM+ competency assessment tool (digital or paper). PM+ 8 (introducing and explaining slow breathing) is the most directly relevant item to this role-play, although you may want to rate other items too.

Feedback (10 minutes)

- Provide group feedback or (depending on time) feedback to the pairs, based on your ratings on trainee PM+ competency assessment tools.

4.3 Set home practice (5 minutes)

Learning outcome: trainees practise the Managing Stress strategy and revise the delivery of elements of Session 1.

Tell trainees

- Ask the trainees to practise using the breathing technique for 10 minutes every day during the rest of the training and encourage them to reflect on how they feel as it becomes more familiar.
- If you have not had time to run a role-play on pulling Session 1 together, ask the trainees to practise delivering elements of Session 1 to their family or to other trainees.

Organize trainees

- Remind trainees who are to facilitate the review session at the start of the next module.
- Remind them to focus on key information they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run this review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 6 role-play

Managing stress

These instructions are for Module 6, Lesson 2.3. Trainees role-play managing stress.

Instructions to the person playing the PM+ helper

Your role is to explain to the PM+ client how to use slow breathing techniques and support them to feel comfortable using them in their everyday life when they feel stress. Follow scripts and instructions from the **PM+ intervention manual Chapter 6, Appendix G Session 1, step 5: Managing stress.**

When you deliver the managing of stress as part of Session 1 you should individualize the information and talk about the client's examples of stress or physical tension where you can.

Go through each of the steps slowly and allow time to practise using each step. This is called "chunking". We use this approach so that the information is not too overwhelming for clients.

Make sure you count "one, two, three" at a rate of one second per count. You might use a ticking clock to help.

Instructions to the person playing the PM+ client

FEMALE SCENARIO

You are a 44-year-old mother of five children under 10 years of age. You are feeling extremely stressed. Your husband has to work extra shifts at the factory. This means you have to look after the children seven days a week without any help.

Your mother is also very unwell and you often need to help care for her. You are beginning to feel hopeless about your situation. You are not sleeping well and your body aches all through the day. You describe no longer feeling strong emotions, even love for your children. You find yourself getting angry with them and no longer caring about looking after them properly.

You identify your main problems as managing all the different roles in your life – being a mother, a carer for your own mother and keeping up with household activities. Your second problem is feelings of hopelessness.

MALE SCENARIO

You are a 37-year-old man whose son was killed in a motorbike accident four months ago. You are very anxious about the safety of your other children. You do not let them play outside any longer for fear they might be killed. When you feel anxious, you experience lots of uncomfortable sensations in your body, such as chest pain, headaches, dizziness and aches.

You continue to grieve the loss of your son and find it difficult to be around your other children because this makes you miss your son and become very upset. When your children misbehave, you get very angry with them and tell them that the good son died. You feel irritable all the time and cry uncontrollably. When you feel like this your headaches come back and you feel a lot of tension in your body. You say you also feel tired most of the time.

PROMPTS FOR CLIENT.

When the PM+ helper explains to you how to use the slow breathing technique, express concerns that you are not doing the breathing correctly. For example, say “I don’t think I am doing this right” or “I worry that I won’t know when to use this correctly”.

Module 7

The Managing Problems strategy 7h5min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review and introduction (20 minutes)			
1.1	**Review the components of PM+ Session 1 (15 minutes)**	Trainees facilitate a review of knowledge from previous modules	Flipchart with PM+ Session 1 aims (Table 6)
1.2	**Introduction to the components of PM+ Session 2 (5 minutes)**	Trainees understand what to expect from Module 7	Flipchart with PM+ Session 2 aims (Table 7)
Lesson 2. The Managing Problems strategy in PM+ (315 minutes)			
2.1	**Demonstration role-play: How to begin PM+ Session 2 (35 minutes)**	Trainees learn how to start PM+ session 2	Use **Appendix G PM+ intervention manual** Protocol Session 2
2.2	**Introduce the Managing Problems strategy (5 minutes)**	Trainees know why problem-solving is important to PM+	
2.3	**Present the Managing Problems steps (30 minutes)**	Trainees know the problem-solving steps	Flipchart with Managing Problems steps
2.4	**Demonstration role-play: delivering the Managing Problems strategy badly (70 minutes)**	Trainees can identify unhelpful approaches to delivering the Managing Problems strategy	
2.5	**Group activity. Identifying solvable and unsolvable problems (25 minutes)**	Trainees can distinguish solvable and unsolvable problems	Flipchart with solvable and unsolvable problems
2.6	**Group discussion. Defining the problem (20 minutes)**	Trainees know how to define a problem with a PM+ client	

No.	Description	Learning outcome	Advance preparation
2.7	**Trainees role-play choosing and defining a problem (40 minutes)**	Trainees practise how to define a problem with a PM+ client	Print and adapt Module 7 role-plays. Have copies of trainees' PM+ competency assessment tools ready (paper or online). The relevant items are: PM+ **9, 10 and 11**
2.8	**Group discussion. Brainstorming solutions and using questions to avoid directing the client (30 minutes)**	Trainees practise how to brainstorm solutions with a PM+ client and how to use questions to avoid giving direct advice	Groups will need a large sheet of paper or flipchart
2.9	**Group discussion. Choosing a solution and making a plan with a client (20 minutes)**	Trainees can explore solutions and help clients choose the best, and know the importance of making a realistic plan	Refer to the PM+ intervention manual **Chapter 7 and Appendix E, client handouts, weekly calendar.**
2.10	**Trainees role-play making an action plan (40 minutes)**	Trainees practise how to make realistic action plans with PM+ clients	Have copies of trainees' PM+ competency assessment tools ready (paper or online). The relevant items are: **PM+ 12, 13 and 14**
Lesson 3. Practising with a difficult Managing Problems situation and concluding Session 2 (70 minutes)			
3.1	**Rotating role-play on managing difficult problems (50 minutes)**	Trainees see and practise how to apply the Managing Problems strategy to a difficult problem	Use and adapt the scenario in Annex 12 Have trainees' PM+ competency assessment tools ready (paper or online). The relevant items are: **PM+ 9-14**
3.2	**Trainees role-play ending Session 2 with the Stress Management strategy (20 minutes)**	Trainees practise concluding PM+ Session 2	

No.	Description	Learning outcome	Advance preparation
Lesson 4. Summary, home practice and setting up the review (20 minutes)			
4.1	**Summary (10 minutes)**	Trainees recap Module 7	
4.2	**Setting home practice for managing problems (5 minutes)**	Trainees practise managing problems by applying the strategy to their own lives	
4.3	**Set up the review for next time (5 minutes)**		

Lesson 1

Review and introduction 20min

Prepare in advance

You will need your flipchart showing the Session 1 aims and how these are achieved, and another showing the aims and approaches for Session 2.

1.1 Review the components of PM+ Session 1 (15 minutes)

Learning outcome: trainees facilitate a review of knowledge from previous modules.

Organize trainees (10 minutes)

- Nominated trainees lead the review session (up to 10 minutes).
- The trainer answers questions or clarifies misinformation about the previous module's content.
- Either invite two new trainees to volunteer to lead the next review session, or nominate a pair of trainees.

Trainer-led review (5 minutes)

- Review the trainees' home practice. Ensure that the following are included:
  - finding out how the trainees are adapting and integrating the management of stress into their everyday life;
  - assessing motivation levels by asking what was difficult about the breathing exercise, why things were difficult, and what can be done make it easier;
  - monitoring how well people have understood the strategy.
  - asking how the trainees feel about teaching these strategies to other people.

1.2 Introduction to the components of PM+ Session 2 (5 minutes)

Learning outcome: trainees understand what to expect from the module .

Tell trainees

- Explain that this module will teach how to deliver PM+ Session 2, which has the following components:
  - during-intervention PSYCHLOPS assessment and general review (5 minutes);
  - the Managing Problems strategy (70 minutes);
  - the Managing Stress strategy (10 minutes);

- ending the session (5 minutes).

- Introduce Table 7 to show trainees how PM+ Session 2 aims are achieved.

TABLE 7
PM+ Session 2 aims, and how they are achieved

Session 2 aims	How aims are achieved
Teach the clients a problem-solving strategy to help them adaptively manage practical problems.	• Teaching the steps in the Managing Problems strategy. • Apply the Managing Problems strategy to clients' personal problems.
Support ongoing practice of managing stress.	• Review home practice. • Help manage any difficulties clients had with home practice. • Practise the management of stress at the end of the session.

Lesson 2

The Managing Problems strategy in PM+ 315min

Prepare in advance

You will need the **PM+ intervention manual, Appendix G Protocol, How to start PM+ Session 2.**

2.1 Demonstration role-play: How to begin PM+ Session 2 (35 minutes)

Learning outcome: trainees learn how to start a PM+ session by reviewing home practice.

Organize trainees (5 minutes)

- The aim of this role-play is to demonstrate how to start PM+ Session 2.
- Choose a trainee who has had problems with their home practice to play the role of the PM+ client opposite you. For example, if they said they did not have time to do the home practice or had a negative experience using the strategy such that they felt dizzy. Encourage them to feel comfortable when expressing these problems during the role-play.
- Tell the rest of the trainees to follow along using **Appendix G Protocol of how to start PM+ Session 2.**

Run a demonstration role-play (15 minutes)

- Demonstrate how to start PM+ Session 2 with a general review, the PSYCHLOPS as well as reviewing the client's home practice.

Facilitate a discussion (15 minutes)

- Ask for trainees' reflections on the role-play.
- Ask them to reflect on how confident they feel about facilitating the review and completing the PSYCHLOPS in a timely manner. Ask them if they can think of problems or difficulties they might encounter and think of solutions as a group.
- Answer any concerns the trainees may have.

2.2 Introduce the Managing Problems strategy (5 minutes)

Learning outcome: trainees know why problem management is important to PM+.

Tell trainees

- Explain that adversity can reduce a person's ability to respond well to practical problems.
- They may feel helpless or lack confidence in managing their problems, or possibly their feelings of anxiety or grief will get in the way of managing practical problems effectively.
- The PM+ approach to managing problems is a structured strategy that aims to improve clients' ability to solve or manage practical problems.

2.3. Present the Managing Problems steps (30 minutes)

Learning outcome: trainees know the problem-solving steps.

Tell trainees

- Use your prepared flipchart to review the seven steps of Managing Problems (see the **PM+ intervention manual, Chapter 7 and Appendix G** for the template).
- Briefly set out how to approach each step, as outlined in the *Key concepts* box. Make sure the trainees understand that the steps should be followed in the right order.
- As you cover the steps, emphasize points that aim to prevent common mistakes in delivering Managing Problems

KEY CONCEPTS

The steps in Managing Problems

1. **Listing problems.**
List the client's problems that were identified during assessments and sessions. Give the client enough time to make a list they feel is complete. Categorize problems as solvable problems, unsolvable problems, or unimportant problems that have no impact on the client's priorities or quality of life.

2. **Choose a solvable problem.**
Initially choose an easy, solvable problem that is not too complex. This way the client can learn the steps involved in the strategy and how to apply them. Eventually address bigger problems so the client can manage them with support. Avoid problems that are better handled by other PM+ strategies. For example, "I can't do housework because I feel tired" will best be managed with the Get going, Keep doing strategy.

3. **Define the problem.**
Name and clearly describe the problem. Avoid broad definitions (such as "problems at work") or too narrow definitions (such as specific arguments). Instead, identify the precise issues to be solved (e.g. "disagreements with my boss that are preventing me from performing my job well").

4. **Possible solutions.**
Helpers should prompt clients to think up possible solutions but should not provide direct advice. Empower the clients to think about past successes with similar probs or ask, "What solutions would you recommend to a friend?" Make general questioning suggestions if clients struggle.

5. **Choosing helpful solutions.**
Encourage the client to choose the best solution. Weigh up each option from all sides – i.e. the pros and cons, the chances of success and failure. Then choose the best option.

6. **Making a plan.**
Make a plan and schedule steps in the activities calendar provided in the PM+ intervention manual (Appendix E)Help the client think about how they are going to proceed by specifying a detailed plan – such as when something will happen, under which circumstances, which words they plan to use, how they will react to what others say or do etc. A more specific plan gives the client a greater chance of success.

7. **Review.**
In later review sessions you will discuss whether the solution led to the desired result. Was the plan a success? If it was, you can tackle the next problem. If it was not, go back to Step 4 and choose another solution to try. Many problems are so complex, unfortunately, that several solutions and plans are needed to solve them. But this to be expected. After all, there must be reasons why the client has struggled with the problem for so long. Simple problems would have been solved a long time ago.

2.4 Demonstration role-play. Delivering the Managing Problems strategy badly (70 minutes)

Learning outcome: trainees can identify unhelpful approaches to delivering the Managing Problems strategy.

Prepare in advance

The aim of this role-play is for trainees to appreciate how **not** to deliver the Managing Problems strategy so they can avoid making common mistakes when working with clients.

To do this, trainers will demonstrate how to deliver a Managing Problems session badly.

Prime your trainees to look for possible errors in the demonstration role-play and to highlight if anything was done badly. The most common helper error in delivering Managing Problems is to be too direct or to tell clients what they should do to solve or manage their problems. The role-play will help trainees to see this played out.

Prepare an empty Managing Problems table on a flipchart. Use the "**Steps to managing problems" template from the PM+ intervention manual (in Appendix E)** but keep the left-side column blank blank.

Organize trainees (5 minutes)

- Ask a trainee to volunteer for the client role-play.
- Out of view of the other trainees tell the volunteer trainee to invent some practical problems (e.g. difficulty making a decision).
- Advise the volunteer that it is important not to use their own problems as the purpose of this demonstration is to show trainees how not to deliver this strategy. Tell the other trainees to look at the Managing Problems steps in Appendix G of the PM + intervention manual and to follow your behaviour as you go through the role-play. Tell them to list any mistakes made and note why they are mistakes.

Run the demonstration role-play (35 minutes)

- You will play the role of the PM+ helper. Role-play the Managing Problems steps with the client but demonstrate unhelpful behaviours.
- Be directive and tell the client what you think they should do to address the problem, as well as demonstrating at least two of the following unhelpful behaviours:
    - Skip the first step of listing all the problems, and just start working on the first problem the person mentions.
    - Accept a general, vague definition of the problem (e.g. "I have financial problems").
    - Show judgement of the client's potential solutions.

- Choose the solution you think is best even though it is one that the person is unlikely to be able to do due to lack of resources.
- Don't break the task down into smaller steps.
- Don't make an action plan to address the problem.

Facilitate a discussion (30 minutes)

- Ask trainees for their initial comments and reactions.
- Ask them to explain the unhelpful behaviours they saw and why they were unhelpful.
- Be very clear that the unhelpful behaviours were done on purpose to highlight what not to do.
- You can use the following prompts:
  - Ask the volunteer playing the client:
    - How did they feel during the role-play?
    - How did they feel towards the helper?
    - What would they have preferred their helper to do?
  - Ask the observing trainees:
    - How might they have felt if they were the client in this situation?
    - What was done well or poorly in the role-play?
    - What would they do differently if they were the helper?
- Finish the discussion by emphasizing that being direct or giving advice to the client is the most common error helpers make when delivering the Managing Problems strategy.
- It is normal to want to tell someone what solutions you think they should follow to manage their problems, but this is not helpful for the client as they need to find their own solutions.
- The best way to avoid giving advice is by asking questions. Highlight these recommended questions from the PM+ intervention manual:
  - What would you suggest to a friend with the same problem? Could this work for you?
  - What have you tried in the past?
  - What have others suggested you should do when you have shared this problem with them?

2.5 Group activity. Identifying solvable and unsolvable problems (25 minutes)

Learning outcome: trainees can distinguish solvable and unsolvable problems.

Tell trainees (1 minute)

- This activity will introduce the trainees to Step 1 in Managing Problems.

Organize trainees (8 minutes)

- Ask the trainees to stand up.
- Explain that you will call out different problems.
- If trainees believe these problems are solvable, they should move to one side of the room; if they believe these problems are unsolvable, or the problem is beyond their influence, they should move to the other side of the room.
- Define what is meant by these terms:
  - solvable: includes problems that can be completely or partially solved or where the person has some influence over the problem;
  - unsolvable: problems for which there is no solution that the person can implement, they have no control or influence over the problem.

Run the activity (15 minutes)

- Read the example problems from Table 8. They are not exhaustive, and you can add others that are relevant to the community you are teaching (be sure to mix the examples up to present the solvable and unsolvable problems).
- Once the trainees have decided if the problems are solvable or unsolvable, ask them to explain their decisions.
- Ask them whether aspects of the problem might be solvable or unsolvable. For example, being diagnosed with an incurable and chronic illness (such as asthma, cancer, or an allergy) may be unsolvable. However, how a person manages their symptoms, risks or pain could be improved and could make the problem partly solvable.

TABLE 8

Solvable and unsolvable problems

Solvable	Unsolvable (or better suited to other PM+ strategies)
• Unemployment • Lack of money for specific needs (e.g. school fees, rent) • Arguing with partner • Pain management • Difficulty attending to children's needs • Conflict with a neighbour, family member or colleague (although a client can change only their own response, not other people's behaviour) • Managing domestic tasks/duties	• There are no opportunities for paid work • Government policies • War • Broader poverty issues (e.g. living in a slum, difficult or cramped living conditions) • Death of a loved one (though how a person manages grief could be solvable, but might be better suited to the Strengthening Social Support strategy) • Chronic or terminal illness • Refugee or displacement status

Tell trainees (1 minute)

- Listing problems is Step 1. Step 2 involves the helper and client choosing the problem they want to tackle first. This should be based on the priorities and wishes of the client.

2.6 Group discussion. Defining the problem (20 minutes)

Learning outcome: trainees know how to define a problem with a PM+ client.

Facilitate a discussion (15 minutes)

- Step 3 is to define the problem.
- Ask the trainees why, once a problem has been selected as solvable and is the priority problem for the client to manage, does it need to be defined?
- Lead the discussion towards realizing that defining a problem makes it easier to break it down into smaller manageable and actionable steps. Explain that, once the problem is defined, its solutions are easier to find.
- You could talk through some of the following examples:
  - War is unsolvable, but there are definable and solvable parts of the problem, such as how we respond to dangers and how we manage war-related stress and worry.
  - Problems of having frequent verbal arguments with a partner are generally at least partly solvable. If we define the problem as too many verbal arguments, we can take steps to have fewer (though we can only modify our own response to the causes of the arguments).
  - Poverty and low opportunities for paid work seem unsolvable. But if we define the problem as the stress caused by unemployment, we can take steps to manage stress or improve someone's social support. If we define the problem as not being ready or in a good position to find work, we might solve that by preparing a CV for a job application or asking for support to seek paid work.
  - Chronic or terminal illness may be unsolvable. But if the defined problem is family well-being after someone's death, perhaps plans can be made to improve the situation. And if the problem is defined as pain or discomfort from illness, we can act to make conditions as comfortable as possible.
  - Problems with studying (e.g. school, university) can often be solvable. If the problem is not having time or space we can explore how to better manage other tasks, juggle priorities, or ask for more support from family and friends.

Tell trainees (5 minutes)

- Finish the discussion by covering the important points of defining a problem as presented in the *Key concepts* box.

KEY CONCEPTS

Choosing a problem

Define the elements of the problem that the client can control or influence to some extent.

Keep the explanation of the problem as specific and as brief as possible.

Try not to include more than one problem – they are easier to tackle when separated out.

If a problem has many parts, break it down and define each part separately.

2.7 Trainees role-play choosing and defining a problem (40 minutes)

Learning outcome: trainees practise how to define a problem with a PM+ client.

Prepare in advance

Print and share Module 7 role-plays

Have copies of trainees' PM+ competency assessment tools.

Organize trainees (5 minutes)

- Divide trainees into groups of three persons and assign each person to be a helper, client or observer.
- Give each group the role-play instructions and prompts in the Module 7 role-plays
- Tell trainees playing the role of the PM+ helper that they will use **Appendix G in their PM+ intervention manual, Session 2: Steps 1–3 of Managing Problems.**

Run the trainee role-play (20 minutes)

- Be ready to rate trainees informally using their PM+ competency assessment tools (digital or paper).

Each trainer aims to assess 1–2 role-plays. The relevant items for this role-play are PM+ 9 (listing and distinguishing solvable and unsolvable problems), PM+ 10 (choosing a problem) and PM+ 11 (defining the problem).

Facilitate a discussion (15 minutes)

- Bring the group back together and ask trainees to reflect on the challenges of Step 3.
- Give any group feedback based on your observations.

2.8 Group discussion. Brainstorming solutions and using questions to avoid directing the client (30 minutes)

Learning outcome: trainees practise brainstorming solutions with a PM+ client and how to use questions to avoid giving direct advice.

Tell trainees (5 minutes)

- Explain that this exercise will focus on Step 4 of Managing Problems, which is brainstorming as many potential solutions as possible.

Facilitate small group discussions (5 minutes)

- Divide the group into teams of 3–5 people, and read out the following problem:

  A client has a number of medical expenses and bills to pay but is feeling overwhelmed and does not know how to deal with them all. He has put off looking at the bills because it makes him feel anxious but he also knows that he needs to start dealing with the problem.

- Tell trainees they have five minutes to think of as many potential solutions as they can.
- One person in the group can write the solutions on a large piece of paper or flipchart.
- Tell them that solutions could be helpful or unhelpful, silly or sensible – with a brainstorming activity the aim is to get lots of ideas before deciding which might work.
- Tell the trainees that this is something they will do with clients.

Facilitate a large group discussion (20 minutes)

- Bring the group back together and ask the different groups to share their solutions.
- Compare and contrast the solutions.
- Then lead a discussion which asks the trainees to reflect on the type of questions they can use as PM+ helpers to encourage clients to think of multiple solutions.
- Emphasize again that the most common error PM+ helpers make is that they give direct advice or suggest potential solutions to the client.
- Instead, it is important that the trainees spend time thinking of questions they can use to encourage clients to think of their own solutions.
- Invite trainees to suggest questions that would help draw out some of the possible solutions they have just listed.
- Encourage broad questions that do not propose the specific solution. For example, it is better to say “Who could you ask for help?” rather than “Can you ask your mother to help with this?” In this way, the clients think up their own solutions. You can refer to the questions below (from the PM+ intervention manual) to prompt trainees if needed.
- You can use these other examples of broad questions if needed:
  - What would you suggest to a friend with the same problem? Could this work for you?
  - What have you tried in the past (even if it has not worked)?

- What have others suggested you do when you have shared the problem with them?
- Is there any support you can get from friends or family that might help manage this problem?
- Could you talk to someone about this problem? Who might that be?

2.9 Group discussion. Choosing a solution and making a plan with a client (20 minutes)

Learning outcome: trainees can explore solutions and help clients choose the best and know the importance of making a realistic plan.

Tell trainees (2 minutes)

- The next step (Step 5) is to choose the most helpful solution.
- As PM+ helpers they will help the client do this by discussing each option from several viewpoints, considering the pros and cons, and the chances of success or failure (which may also depend on resources available to the client).
- However, it is the client who will choose the best solution. Sometimes, it might be OK to choose two options – such as two small solutions – so long as the action plan (Step 6) remains simple and feasible to carry out.

Facilitate a discussion (15 minutes)

- Start by explaining that the next task is to practise making that action plan (Step 6).
- But first ask trainees to consider the *following* questions.
  - Think back to a time when you were struggling to solve a problem. How did you manage it? Did you make a plan?
  - Did the plan work?
  - Was it complicated?
  - Why might a plan with plenty of steps be more likely to succeed than a plan with only one or two steps?

Tell trainees (3 minutes)

- Finish the discussion by saying that clients with more steps in their plan:
  - will feel less overwhelmed;
  - will be more prepared;
  - will feel a gradual sense of success at having actioned even small steps of their plan.
- Talk through the following example with the trainees:

  A client defines their problem as financial debt to a telecoms company, so they cannot use their phone.

A plan with one step might be: speak to the telecoms company and ask for an extension to pay the bill.

However, the client may find problems with this plan, such as: "I cannot afford credit to phone them" or "I cannot connect to them during business hours" or "I am too afraid to call".

A plan with multiple steps will seem more achievable will and make the client feel less overwhelmed. A plan might have steps such as:

1. Speak to a family member to get ideas about what to say to the telecoms company.
2. Check and obtain mobile credit. If there's no money for credit, ask to borrow someone else's phone.
3. Plan how to save enough money to clear the bill – how long will it take?
4. Decide how long you will need as an extension for payment.
5. Write down what to say to the telecoms company, so you have this to refer to when you call.
6. Plan for one lunch break during work to phone the company. Specify the day (using the weekly calendar handout for clients, in Appendix E of the PM+ intervention manual. If unable to reach the company, try during the lunch break the next day until you get through.

- Ask for the trainees' reflections and answer any queries.

2.10 Trainees role-play making an action plan (40 minutes)

Learning outcome: trainees practise how to make realistic action plans with PM+ clients.

Prepare in advance

Print and share Module 7 role-plays.

Have copies of trainees PM+ competency assessment tools available.

Organize trainees (5 minutes)

- Ask trainees to return to the groups they were in previously for the role-play on choosing and defining problems. Ask them to swap roles but to use the same problems and scenarios.
- Tell the person role-playing the PM+ helper to use the **PM+ intervention manual, Appendix G:** Managing Problems, steps 4, 5 and 6.

Run the trainee role-play (15 minutes)

- Have trainees' PM+ competency assessment tools (digital or paper) ready and informally rate their role-play demonstrations (assess one or two role-plays per trainer).
- The relevant items are PM+12 (brainstorming solutions), PM+ 13 (choosing helpful strategies) and PM+ 14 (developing an action plan for the solution).
- Allow a few minutes at the end for the pairs to give feedback to each other.

Facilitate a discussion (15 minutes)

- Provide group feedback based on your informal observations and the PM+ competency assessment tools.
- Ask for reflections from the trainees and answer any concerns they may have.

Tell trainees (5 minutes)

- Explain to trainees that Step 7 (reviewing the action plans) is a continual process that starts when the client begins PM+ Session 3.
- As part of this step, helpers will ask clients to reflect on the efficacy of their action plans, their solutions and how the nature of their problems is changing.
- Explain to the trainees that they will have an opportunity to practise this at the beginning of the next module.

Lesson 3

Practising with a difficult Managing Problems situation and concluding Session 2 70min

3.1 Rotating role-play on managing difficult problems (50 minutes)

Prepare in advance

Prepare a copy of the Managing Problems worksheet on a flipchart. Familiarize yourself with the challenging scenario in Annex 12 before the activity.

Learning outcome: trainees see and practise how to apply the Managing Problems strategy to a difficult problem.

Tell trainees (5 minutes)

- The aim of this rotating role-play is for trainees to work together to complete a Managing Problems worksheet from **Appendix E of the PM+ intervention manual** (ensure all trainees can see a copy of this work sheet) for a client presenting with a more difficult problem.
- Give the trainees time to familiarize themselves with the worksheet again.

Organize the trainees (5 minutes)

- Ask your co-trainer or a volunteer to role-play the person with a difficult case. Invite them to read aloud the scenario from Annex 12. Remind trainees that Managing Problems is ideal for dealing with practical problems.
- Explain that you will start the role-play as the PM+ helper and support the client to list solvable and unsolvable problems.
- Once you have completed that step (approximately 3–5 minutes), pause the role-play and ask another trainee to swap with you and become the helper.
- Allow them to role-play Step 2 for (3–5 mins), then pause the role-play and ask for another trainee to take over as the helper. Continue this way until all of the steps are completed.
- When trainees are playing the helper, they can say "Stop!" and ask for advice from other trainees. However, they should try to keep the role-play as natural as possible.

Run the rotating role-plays (20 minutes)

- Start the role-play as the PM+ helper.
- Once you are no longer in the role-play, use the trainees' competency assessment tools to help you make some informal observations. The most directly relevant items to this role play are PM+ 9 (listing and distinguishing solvable and unsolvable problems), PM+ 10 (choosing a problem), PM+ 11 (defining the problem), PM+12 (brainstorming solutions), PM+ 13 (choosing helpful strategies) and PM+ 14 (developing an action plan for the solution). You may also want to rate items PM+ 1-6 (i.e. those related to basic helping skills).
    - Ensure that the trainees work through the Managing Problems steps in the role-play and write the client's responses for each step into the worksheet on the flipchart, so that everyone can see.

Facilitate a discussion (20 minutes)

- Ask trainees to reflect on and discuss any challenges the trainees' experienced in delivering Managing Problems with this more difficult case.
- Provide group feedback.
- Finish the discussion by focusing on how well the trainees understand how to support the client to find practical solutions in the face of emotional problems.

3.2 Trainees role-play ending Session 2 with the Stress Management strategy (20 minutes)

Learning outcome: trainees practise concluding PM+ Session 2.

Tell trainees (6 minutes)

- Explain that at the end of PM+ Session 2 it is important to use the Managing Stress strategy.

- This is to encourage its ongoing use, but also to recognize how difficult the Managing Problems strategy can be for people.
- Review the benefits of using stress management by telling trainees the key concepts they learned in Module 6.

Organize trainees (3 minutes)

- Ask for a volunteer to guide the group through the Stress Management strategy.
- Tell the volunteer to use the instructions in the PM+ intervention manual (and follow any group/trainee decisions made in earlier modules to adapt the instructions to fit the local context).

Run the role-play (10 minutes)

- Join the trainees in the Stress Management strategy.

Tell trainees (1 minute)

- To fully conclude PM+ Session 2, trainees should set home practice for their clients.

Lesson 4

Summary, home practice and setting up the review 20min

Prepare in advance

You will need enough Managing Problems worksheets to hand out to trainees.

4.1 Summary (10 minutes)

Tell trainees

- Give a brief summary of the topics covered in Module 7, allowing time to answer questions and clarify issues that trainees ask about.

4.2 Setting home practice for Managing Problems (5 minutes)

Learning outcome: trainees practise managing problems by applying the strategy to their own lives.

Organize trainees

- Give each trainee a Managing Problems worksheet and tell them the home practice is to work through the sheet (Steps 1–6) for one of their own problems.
- Explain that they do not need to share their worksheet with anyone else if they do not want to.
- However, say that you will be asking for a volunteer to share an action plan that they are struggling with, so they might not want to choose their most intimate problem.
- They can support each other as they work through the steps if they wish to.
- They can ask the trainers for help if they have difficulties.

4.3 Set up the review for next time (5 minutes)

Organize trainees

- Tell the trainees that you will be leading the review at the beginning of Module 8.
- However, you still need two trainee volunteers who will role-play how to review Managing Problems in Module 8, Lesson 2.
- Explain that one trainee will play the role of the PM+ helper (with support from you and the other trainees) and will use the steps outlined in the **PM+ intervention manual, Appendix G** (the section for Session 3, Review Managing Problems). Encourage the volunteer to get familiar with this as part of their preparation.
- The other volunteer will play the role of a client who is struggling with managing problems.
- For their preparation this person can use their own problems OR invent a problem and how a client may struggle with the strategy.

Module 7 role-plays

Managing Problems

These instructions are for the trainee role-plays from Module 7, Lesson 2.6. *Trainees role-play choosing and defining a problem.*

Instructions for the person role-playing the PM+ helper

Use and adapt the suggested dialogues in the PM+ intervention manual (Appendix G Session 2, Step 1–3 of Managing Problems) to introduce the Managing Problems strategy to the client. Support them to choose and define a solvable problem that they are facing. If your client feels that their problems are too big or too many, then support them to choose and define one problem that they think is in their control to solve.

Instructions to the person playing the role of the PM+ client

FEMALE SCENARIO

You are unemployed and living with your husband and four children at your in-laws' home in very cramped conditions. You and your family were forced out of your home as you could no longer pay the rent. Your in-laws have very strict rules about what you and your children are allowed to do in the house. Your children are often getting into trouble for being too loud or disruptive. They also expect you to complete all of the household chores and cooking for the family. Some days, you are unable to do this adequately as well as spend time with your children, which causes arguments between you and your in-laws. You are feeling increasingly sad about your living situation and the effect it is having on your children. You and your husband are also beginning to argue about his parents.

As you discuss the problems with the helper, identify several problems: problem of cramped living conditions; problem of in-laws expectations on children; problem of in-laws expectations on you; arguments with your husband about these problems.

But be most worried about the effect your in-laws are having on your children and so decide this is the problem you would like to work on first.

Problem: Manage my in-law's expectations on my children and their behaviour in the house.

MALE SCENARIO

You are a 37-year-old man married with five children under 10 years of age. You have been working at the same factory for years but it was closed down six months ago. Since then you have not been able to find permanent work.

You did not finish school and are disabled from an injury you sustained in an ongoing conflict in your country, which restricts your mobility. This has made it difficult for you to find work. Your wife has tried to sell materials at the markets to support the family financially. However, you have started to have troubles paying bills, such as rent, electricity and food.

You identify two large problems: unemployment and financial problems. While you need to find work, you decide that you first need to manage the smaller problems related to paying the bills.

Problem: Manage my finances so I can pay the rent and electricity bill that is due in two weeks.

CLIENT PROMPTS

If the PM+ helper asks you to think of the problems you are facing at the moment, focus on the elements of the problem that are difficult to solve. Use culturally appropriate body language and communication skills to explain that you feel hopeless or out of control and that you don't think you could solve any of your problems.

Instructions for Module 7, Lesson 2.9. Trainees role-play making an action plan (35 minutes)

Instructions to the person role-playing the PM+ helper

Using Appendix G of the PM+ intervention manual, Managing Problems, Steps 4, 5 and 6. It is your role to help the PM+ client come up with possible solutions to their solvable problem. Choose a solution and make a specific action plan to help them solve their problem.

Instructions to the person playing the PM+ client

Use the same scenario and problems as in the previous role-play.

Client Prompts: If the PM+ helper asks you to list possible solutions, take some time to focus on unrealistic or unhelpful solutions. However, if the PM+ helper helps you to realize why these are unrealistic, then accept the explanation and give some more practical solutions.

Once you have selected a practical and realistic solution, the PM+ helper will ask you to create an action plan. Spend some time focusing on the barriers to implementing that plan and all the reasons why it could fail. However, if the PM + helper encourages and motivates you to try, then engage with the plan and be enthusiastic.

Module 8

Get going, Keep doing 5h15min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review and introduction (45 minutes)			
1.1	**Review information from previous modules (35 minutes)**	Trainees recall components of PM+ Session 1 and 2	Prepared flipcharts (Table 6 and Table 7)
1.2	**Introduction to PM+ Session 3. Get going, Keep doing (10 minutes)**	Trainees know what to expect in Module 8	Prepared flipchart with Session 3 aims (Table 9)
Lesson 2. PM+ Session 3 (160 minutes)			
2.1	**Volunteer role-play. Reviewing a client's home practice for the Managing Problems strategy (35 minutes)**	Trainees see how to review the Managing Problems strategy with a client	**PM+ intervention manual, Chapter 8 and Appendix G (Session 3)**
2.2	**Present the rationale for Get going, Keep doing (20 minutes)**	Trainees know that low mood causes inactivity, and that this can self-reinforce in a cycle. Get going, Keep doing aims to break this inactivity cycle	
2.3	**Components of the Get going, Keep doing strategy (20 minutes)**	Trainees know the steps in Get going, Keep doing	
2.4	**Small group discussions on breaking down tasks into steps for Get going, Keep doing (65 minutes)**	Trainees identify how to break an activity down into smaller steps	Flipchart page with example of a broken-down activity
2.5	**Group discussion on the differences between the Managing Problems strategy and Get going, Keep doing (20 minutes)**	Trainees recognize the difference between Managing Problems and Get going, Keep doing	Flipchart showing discussion questions

No.	Description	Learning outcome	Advance preparation
Lesson 3. Pulling together PM+ Session 3 (85 minutes)			
3.1	**Trainees role-play using the Get going, Keep doing strategy (55 minutes)**	Trainees practise using the Get going, Keep doing strategy	Print and adapt Module 8 role-play Have the trainees' PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 15**
3.2	**Trainees practise Managing Stress and reflect on ending session 3 (30 minutes)**	*Trainees practise concluding PM+ Session 3*	
Lesson 4. Summary, home practice and setting up the review (25 minutes)			
4.1	**Summary (10 minutes)**		
4.2	**Set home practice of the Get going, Keep doing strategy (10 minutes)**	Trainees practise individualizing Get going, Keep doing	Blank sheets with the titles: Enjoyable activities and Tasks/Chores (2 columns); Steps to take; Action plan
4.3	**Set up the review for next time (5 minutes)**		

Lesson 1

Review and introduction 45min

Prepare in advance

You will need a flipchart or slide showing the Session 3 aims, and how these are achieved (Table 9).

1.1 Review information from previous modules (35 minutes)

Learning outcome: trainees recall components of PM+ Session 1 and PM+ Session 2.

Facilitate a discussion (15 minutes)

- Ask trainees to recap their knowledge of PM+ Session 1 and 2.
- Take some time to see how the trainees are understanding and responding to the flow and rhythm or each PM+ session.
- Check with them to see how competent they would feel in delivering these sessions.
- You could bring out the dartboard activity from Module 1 and compare how people felt then compared to now and stimulate some discussion and reflection.
- This is also an opportunity to answer any questions or correct any misconceptions that the trainees may have.

1.2 Introduction to PM+ Session 3. Get going, Keep doing (10 minutes)

Learning outcome: trainees know what to expect in Module 8.

Tell Trainees

- Use your flipchart to outline the components of PM+ Session 3. The components are:
  - PSYCHLOPS and general review (5 minutes)
  - Managing Problems (35 minutes)
  - Get going, Keep doing (35 minutes)
  - Managing Stress (10 minutes)
  - Ending the session (5 minutes).
- Take a moment to reflect on the timings given for the session, noting that Managing Problems is given the same amount of time as the new Get going, Keep doing strategy.
- Explain that Session 3 provides the first opportunity to explore how a client is finding the Managing Problems strategy. This means time is needed to review and understand the strategy and to support the client to overcome any challenges.

- Session 3 also includes Managing Stress to help clients continue practising this strategy.
- Display your prepared flipchart or slide showing the Session 3 aims and talk through how they are achieved (Table 9).

TABLE 9

The aims for PM+ Session 3, and how these are achieved

Session 3 aims	How aims are achieved
Support ongoing practice of managing stress and managing problems.	• Review home practice. • Help manage any difficulties the client had with home practice. • Continue with Managing Problems (with the original problem or a new problem).
Educate the client about low mood (or depression) and inactivity.	• Present and explain the inactivity cycle.
Teach the client a strategy that improves mood by increasing engagement in pleasant activities and tasks.	• Teach the steps of Get going, Keep doing. • Apply Get going, Keep doing to the client's life.

Lesson 2

PM+ Session 3 160min

Prepare in advance

For Lesson 2.1 your chosen volunteers from the end of Module 7.

2.1 Volunteer role-play. Reviewing a client's home practice for the Managing Problems strategy (35 minutes)

Learning outcome: trainees see how to review the Managing Problems strategy with a client.

Tell trainees (1 minute)

- Remind trainees that, after the general review and PSYCHLOPS, PM+ Session 3 starts with the helper asking the client to review how they have been implementing the Managing Problems strategy at home.

Organize the trainees (4 minutes)

- Ask the trainees chosen at the end of the previous module to organize themselves as the PM+ helper and client.
- Tell the person playing the PM+ helper to use the script in **Appendix G of the PM+ intervention manual**, Step 7 Managing Problems, to review the client's progress.
- Tell this person they can ask you or other trainees for help if they need to but encourage them to keep the role-play as realistic as possible.

Run the role-play (30 minutes)

- Allow the role-play to continue for up to 20 minutes.
- Sit behind the trainee playing the helper to provide any subtle coaching that is needed.
- Allow 10 minutes to answer any questions that trainees have about reviewing Managing Problems with clients.

2.2 Present the rationale for Get going, Keep doing (20 minutes)

Learning outcome: trainees know that low mood causes inactivity, and that this can self-reinforce in a cycle. Get going, Keep doing aims to break this cycle of inactivity.

Prepare in advance

You may wish to prepare a flipchart or slide to show the inactivity cycle graphically for Lesson 2.2. You could also display the prompt questions.

Tell trainees (3 minutes)

- In PM+ Session 3 the helper explains to the client that adversity can lead people to become stuck in a cycle of inactivity and low mood.
- Emphasize that a client who is feeling low will probably say, "I will do xxx when I feel like it".
- Unfortunately, withdrawing from activities causes the low mood to continue. The worse a person feels, the harder it is for them to start doing things again, and the cycle continues.

Organize trainees (10 minutes)

- Ask trainees to turn to **Chapter 8 in the PM+ intervention manual** and read the Background section.
- Trainees could read this to themselves, or you could invite a volunteer to read it aloud.
- Ask the trainees to take some time to consider the prompt questions below (display them):
  - Think about times when you have felt low in mood.
  - Did your activity levels change?

  - Did you stop doing certain things?
  - Did you stop seeing people or attending events? Do you know why you stopped?
  - How did you feel?
- Trainees do not need to share anything unless they want to.

Tell trainees (7 minutes)

- Draw the inactivity cycle on a flipchart or ask trainees to look at the one in their **PM + intervention manual, Appendix E: Client handouts**.
- Explain that the Get going, Keep doing strategy aims to break the cycle of inactivity.
- It improves the client's mood by encouraging the client to get involved again in pleasant and task-oriented activities in spite of their mood.
- You will need to communicate to clients that they need to "do first, and the motivation or positive feelings will follow" rather than wait until they feel better to start an activity.
- Explain that, supporting the client to "do first" involves the clients setting themselves very small goals that they can achieve, and they can experience positive benefits from achieving.
- Experiencing some success in doing even a small part of a task is really helpful for improving the client's mood, sense of achievement and confidence.
- Choosing simple tasks and breaking them into small steps is one way of ensuring success for your clients.

2.3 Components of the Get going, Keep doing strategy (20 minutes)

Learning outcome: trainees know the steps in Get going, Keep doing.

Tell trainees (10 minutes)

- Talk through the steps of the Get going, Keep doing strategy as they are in the *Key concepts* box.

KEY CONCEPTS

- **Step 1. Explore pleasant activities and tasks or chores.** Emphasize the importance of both. Enjoyable activities encourage pleasant feelings. Getting tasks done gives a sense of achievement, which can also be very helpful in lifting a person's mood. Refer the trainees to Chapter 8 of the PM+ intervention manual for examples of both types of activities that a helper can suggest.
- **Step 2. Choose one pleasant activity and/or task.** Emphasize that helpers should not randomly choose an activity for the client. Instead, they should ask the client "What did you do before, that you no longer do?", "What do you need to start doing again?" or "Is there something new you have always wanted to do, or think you now need to do?".
- **Step 3. Break the chosen activity down into small manageable steps.** When a person is depressed, activities and tasks can seem too big or too difficult. Therefore break them down to make them feel less overwhelming.
- **Step 4. Help the client to schedule steps, tasks and activities.** Suggest putting reminders in a telephone, writing tasks into a calendar, planning activities to coincide with other routines, such as mealtimes etc. The weekly calendar in the Appendix E of the PM+ intervention manual is a useful tool.

Organize trainees (10 minutes)

- Ask the trainees to go to **Appendix G in the PM+ intervention manual** and read the protocols for Session 3.
- Give them some time to read through the scripts and ideas, then answer any questions or concerns.

2.4 Small group discussions on breaking down tasks into steps for Get going, Keep doing (65 minutes)

Learning outcome: trainees identify how to break an activity down into smaller steps.

Prepare in advance

Prepare a display of "Meeting friends for coffee" activity broken down into small steps. You will also need enough flipcharts or large pieces of paper for each group.

Tell trainees (1 minute)

- Explain to trainees that the hardest part of Get going, Keep doing is breaking an activity down into small, manageable steps, so everyone is going to practise this.

Organize trainees (4 minutes)

- Divide trainees into groups of four and explain that they will practise this now.
- Each group should move to a separate space in the room. Give each group a flipchart and markers.

Facilitate small group discussions (10 minutes)

- Give the trainees an example activity such as *Getting to this training day*. It should be something that everyone has completed recently.
- They have 10 minutes to write down as many steps as were needed, in the correct order, to complete the activity.
- Say that it does not matter how small each step is. In fact, the smaller the better. The aim is to be the group with the most steps.

Facilitate a large group discussion (20 minutes)

- Keep the trainees in their groups but bring the groups back together and ask one person from each group to present their steps to everyone.
- Acknowledge the group with the most steps.
- Summarize the activity for the group, pointing out that when someone is depressed it is much easier for them to commit to taking small steps than to completing something that seems challenging. You could say:

  "Imagine you were feeling very depressed and had difficulty getting started in the morning. Which would be easier for you to do: come to training today or just do the first step on your list? Of course, just doing the first step would be easier. Even if you don't finish the list, you might feel better for having done something. Then, when you feel a bit better still, you might decide you can do some more steps, and by the end of the week you might have been able to get yourself to training. But if you didn't break this task into small steps, you may never have come, it might just have seemed too hard."

- Allow trainees to ask any questions about how to break an activity down into small steps.
- Now explain that they will return to their small groups and practise breaking down an activity into steps.

Facilitate small group discussions (10 minutes)

- Give each group one of the following activities (or choose something else that is suitable):
  - meeting a friend for coffee;
  - going for a walk;
  - washing my clothes;
  - cooking dinner;
  - doing some painting;

  - fixing the flat tyre on my bicycle;
  - reading and responding to my work emails.
- Show this example in a prominent place to guide trainees' discussions.

MEETING MY FRIEND FOR COFFEE

- list from the reason for meeting my friend (fun, laughter, get support, etc.);
- list my friends and decide which one I want to meet;
- call or message my friend to arrange a time or place to meet;
- plan how I will get there and whether I need money or help from others;
- on the night before the meeting, plan what I am going to wear and lay the clothes out ready for the next day;
- on the day of seeing my friend, do stress management if I get anxious;
- on the day of the meeting, put clothes on that I've laid out, follow my plan for getting to the place on time.

Facilitate a large group discussion (20 minutes)

- Bring the group back together.
- Give one person from each group two minutes to present their steps to the whole class.
- Acknowledge the group with the most steps.
- Allow trainees to ask any questions about how to break an activity down into small steps.

2.5 Group discussion on the differences between the Managing Problems and the Get going, Keep doing strategies (20 minutes)

Learning outcome: trainees recognize the difference between Managing Problems and Get going, Keep doing.

Prepare in advance

For this lesson you will need to prepare a flipchart or slide showing the discussion questions on the differences between the Managing Problems and the Get going, Keep doing strategies.

Facilitate a discussion (20 minutes)

- Display these questions on a flipchart or slide.

  1. How do the Managing Problems and the Get going, Keep doing strategies differ?
  2. What does each strategy aim to do?

3. How might a client's action plan for each strategy vary?
4. What can a PM+ helper do to avoid mixing up these two strategies?

- Start a discussion by telling the trainees that a common challenge for PM+ helpers is to understand the difference between Managing Problems and Get going, Keep doing.
- Ask for trainees to contribute their thoughts about why the two strategies are so often confused. Use the questions as prompts.
- Make sure the discussion draws out the following *Key concepts*.

KEY CONCEPTS

Managing Problems solves practical problems

This strategy helps the client to follow steps to solve their practical problems – learning to identify what the problem is, to break it down into manageable steps and to begin resolving it.

Get going, Keep doing breaks the inactivity cycle, improving mood and functioning

This strategy helps to break the inactivity cycle which happens when people's mood is low and they lose interest in, or the energy to do, various activities. Get going, Keep doing helps the client to improve their functioning and become active in chores, jobs or recreational interests.

They can work together

There is no doubt that when clients begin to manage problems and get going after a period of inactivity, the two strategies will work hand in hand to improve the client's mood, situation and well-being.

But the strategies remain distinct

The two strategies still have separate purposes. PM+ helpers need to be careful to ensure that they are supporting the client by offering them the best strategy for a specific difficulty.

Lesson 3

Pulling together PM+ session 3 85min

Prepare in advance

You will need the Module 8 role-play ready to give to trainees for Lesson 3.1. Consider also having some spare copies of the client handouts from **Appendix E of the PM+ intervention manual** so that trainees can use these in their role-plays.

Have copies of trainees' PM+ competency assessment tools ready.

3.1 Trainees role-play using the Get going, Keep doing strategy (55 minutes)

Learning outcome: trainees practise using the Get going, Keep doing strategy.

Organize trainees (5 minutes)

- Ask trainees to form pairs and give them the handouts from the Module 8 role-play.
- Explain that they will now deliver the Get going, Keep doing strategy using this scenario.
- The role-play should last for about 35 minutes so that trainees can see how long it would take to deliver this strategy to a real client.

Run trainee role-play (35 minutes)

- Have trainees' PM+ competency assessment tools (digital or paper) ready and informally rate one or two role-plays per trainer. The relevant item for this role play is PM+ 15 (Get going, Keep doing).
- At the end of the role-play allow the trainees five minutes to reflect together.

Facilitate a discussion (15 minutes)

- Bring the group back together for reflection
- Trainer provides informal feedback from role-plays they observed
- Allow time for trainee's to ask questions

3.2 Trainees practise Managing Stress and reflect on ending PM + Session 3 (30 minutes)

Learning outcome: trainees practise concluding PM+ Session 3.

Organize trainees (2 minutes)

- Ask the trainees to stay in their pairs but swap roles.

Run trainee role-plays (10 minutes)

- Those trainees that are now playing the PM+ helper will lead the other in Managing Stress.

Facilitate a discussion (18 minutes)

- Ask the trainees to come back together as a big group and reflect on how to pull together PM+ session 3.
- This should include: reflecting on the role-play where a volunteer trainee supports a volunteer client to review their Managing Problems home practice; considering how Get going, Keep doing can be introduced; and the importance of continuing with the Stress Management strategy.
- You could use the following questions as prompts:
  - What do you think of the flow of the session?
  - Do you think clients may have problems moving between Managing Problems and Get Going, Keep Doing?
  - How could you ensure that the transition is smooth?
  - How does it feel now that you have more experience in running the Stress Management strategy?

Lesson 4

Summary, home practice and setting up the review 25min

Prepare in advance

Prepare three pieces of paper for each trainee:

- one with two columns, titled *Enjoyable activities* and *Chores/tasks*;
- one titled *Steps to take*;
- one titled *Action plan*.

4.1 Summary (10 minutes)

Tell trainees

- Provide a brief summary of the topics covered in today's session.
- Give trainees an opportunity to ask questions or request clarification.

4.2 Set home practice of the Get going, Keep doing strategy (10 minutes)

Learning outcome: trainees practise individualizing Get going, Keep doing.

Organize trainees

- Explain that trainees will practise the Get going, Keep doing strategy on themselves at home so they can see what it is like to be a client.
- Give everyone their three pre-prepared pieces of paper. Explain that on the paper with the two columns they should write down:
  - enjoyable activities they used to do but that they are no longer doing or that they would like to start;
  - chores or tasks they need to do but have not been able to do recently or have put off doing.
- Explain that the Steps to take sheet is where they will write down
  - an enjoyable activity and the steps they can do to plan to do that activity in the next few days;
  - a chore or task and the steps they can plan to do that task in the coming days.
- Explain that the sheet entitled Action plan is where they should write down:
  - an action plan that specifies when they will complete at least the first steps of the enjoyable activity and the task or chore.
- Say that, if they feel comfortable, trainees will be asked to share their action plans with the group at the start of the next module.

4.3 Set up the review for next time (5 minutes)

Organize trainees

- Choose two trainees (or ask for volunteers) to facilitate the review session at the start of the next module.
- Remind them to focus on key information that they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run the review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 8 role-play

Get going, Keep doing

These instructions are for Module 8, Lesson 3.1

Instructions to the person role-playing the PM+ helper

Use the PM+ intervention manual to introduce the rationale for the Get going, Keep doing strategy. Appendix G has the protocol. Appendix E has handouts to give to the client. Engage the PM+ client with the strategy and make a plan that fits their needs.

Instructions to the person role-playing the PM+ client

FEMALE SCENARIO

You are a 30-year-old woman who looks very untidy (messy hair and dirty clothes). You look tired and speak very softly. You have been trying to become pregnant with your husband for many years without success.

You say this has put a lot of strain on the family. Your parents-in-law have been especially harsh on you and you have begun to blame yourself too. In the last month you have noticed that you are unable to concentrate on anything and you forget things all the time.

You find it hard to socialize now because you cannot keep up with conversation and become easily confused. You also do not want people to talk about why you have not had children yet. When you do socialize, you find that you get terrible headaches and will sleep for hours afterwards. Recently you have stopped seeing anyone except your mother who you see once a week.

MALE SCENARIO

You are a 45-year-old man whose wife died of cancer about nine months ago. Since then you have been losing weight rapidly and find it difficult to sleep.

You have become quite withdrawn from your social life and do little around the home or care for yourself. You continue to go to work because you have to, but you find little pleasure in doing this. You describe feeling as if your sense of pleasure in life died with your wife. You miss her terribly but you are content that she is no longer suffering.

CLIENT PROMPTS

If the person role-playing the helper asks you to think of different activities, focus on activities that you feel you must do (such as chores) and not activities that you enjoy.

At the beginning of the role-play be quite vague about activities that you enjoy. However, as the helper asks you more questions, be open to exploring which activities you enjoy and why.

You could express frustration with the strategy – e.g. by saying “You make it sound so easy. But if it were easy I would have done these activities before. It is much harder than this.

Module 9

Strengthening Social Support

4h45min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review and introduction (35 minutes)			
1.1	**Review home practice and the main components of sessions 1–3 (30 minutes)**	Trainees facilitate a review of home practice	Flipcharts with Table 6, Table 7 and Table 9
1.2	**Introduction to PM+ Session 4 (5 minutes)**	Trainees know what to expect of Session 4.	Flipcharts with the aims and components of sessions, 1–4
Lesson 2. PM+ Session 4 (150 minutes)			
2.1	**Trainees role-play how to review PM+ strategies with a client (60 minutes)**	Trainees practise implementing PM+ reviews	
2.2	**Group discussion exploring the Strengthening Social Support strategy (20 minutes)**	Trainees can name different types of social support and know how to access them	**Chapter 9 in the PM+ intervention manual**
2.3	**Trainees role-play delivering Strengthening Social Support (70 minutes)**	Trainees practise how delivering the Strengthening Social Support strategy	Adapt and print Module 9 role-play instructions Trainees need their copies of the PM+ intervention manual Have trainees PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 16**

No.	Description	Learning outcome	Advance preparation
Lesson 3. Coping with challenges to Strengthening Social Support (75 minutes)			
3.1	**Small group discussions. Challenges to engaging in Strengthening Social Support (25 minutes)**	Trainees recognize types of challenges clients may have with Strengthening Social Support	Use the scenario in Annex 13
3.2	**Trainees role-play engaging a reluctant client with Strengthening Social Support (50 minutes)**	Trainees practise how to engage a reluctant client with Strengthening Social Support	Use Module 9 role-play instructions Have trainees' PM+ competency assessment tools ready (paper or online). The relevant item is: **PM+ 16**
Lesson 4. Summary, home practice and set up the review (25 minutes)			
4.1	**Summary (10 minutes)**	Trainees reflect on how to pull together and conclude PM+ session 4	
4.2	**Setting home practice of Strengthening Social Support (10 minutes)**	Trainees practise individualizing the Strengthening Social Support strategy on themselves	Trainees will need paper for their home practice
4.3	**Set up the review for next time (5 minutes)**		

Lesson 1

Review and introduction 35min

Prepare in advance

You will need a flipchart or slide with the aims of Session 4 and how these are achieved (Table 10).

You may wish to refer back to the corresponding flipcharts or slides you prepared to explain sessions 1–3.

1.1 Review home practice and the main components of Sessions 1–3 (30 minutes)

Organize trainees (10 minutes)

- Nominated trainees lead the review session (up to 10 minutes).
- The trainer answers questions or clarifies misinformation about the previous module's content.
- Either invite two new trainees to volunteer to lead the next review session, or nominate a pair of trainees.
- The trainer then leads a review of the trainees' home practice questions such as "What did you find difficult about this strategy?" or "What might your future clients find difficult about this strategy?" or "What can be done to support clients?" This can help prompt discussion.

1.2 Introduction to Session 4 (5 minutes)

Learning outcome: trainees know what to expect of Session 4.

Tell trainees

- Set out the components of PM+ Session 4 for the trainees. These are:
  - PSYCHLOPS and general review (5 minutes)
  - Managing problems (20 minutes)
  - Get going, Keep doing (20 minutes)
  - Strengthening Social Support (30 minutes)
  - Managing stress (10 minutes)
  - Ending the session (5 minutes).
- Display and read through Table 10 with the trainees, so that they know what to expect from the session.

TABLE 10

Session 4 aims, and how these are achieved

Session 4 aims	How aims are achieved
Support ongoing practice of Managing Stress, Managing Problems and Get going, Keep doing (for an enjoyable activity).	• Review home practice. • Help manage any difficulties a client had with home practice. • Continue with Managing Problems (with the original problem or a new problem). • Encourage practice of Get going, Keep doing (for an enjoyable activity).
Apply Get going, Keep doing with a task-oriented activity.	• Revisit the steps for Get going, Keep doing with a task-oriented activity.
Educate clients on how adversity has an impact on their supports and how to strengthen these.	• Define types of social support. • Teach the steps of the Strengthening Social Support strategy.
Teach clients a strategy that helps them to strengthen their social supports.	• Apply the Strengthening Social Support strategy to a client's life.

Lesson 2

PM+ Session 4 150min

Prepare in advance

Trainees will need their copies of the PM+ intervention manual.

2.1 Trainees role-play how to review PM+ strategies with a client (60 minutes)

Learning outcome: trainees practise implementing PM+ reviews.

Organize trainees (5 minutes)

- Divide the trainees into at least three small groups and give each group one of the following tasks to role-play:
  - running a PSYCHLOPS assessment and review home practice of managing stress;
  - review home practice of managing problems;
  - review home practice of Get going, Keep doing.
- From within the groups, ask for a trainee to play the role of the PM+ helper and another to play the role of the client; the other trainees will be observers and are there to support the PM+ helper.

- Those people playing the role of the client should report having difficulties using the PM+ strategies at home. They can use a scenario from previous role-plays or create their own.
- Those people playing the role of the helper should be reminded to use basic helping skills and the motivational interviewing technique to engage their client and work through the problems. You can also use the scripts and protocols provided in **Appendix G of the PM+ intervention manual.**

Run trainee role-plays (30 minutes)

- Allow 30 minutes for them to perform the role-plays.
- Walk around the room observing the role-plays and be available to give advice and support when requested.

Facilitate a discussion (25 minutes)

- Bring the group back together and reflect on the role-plays, including any difficulties they encountered or that was observed.
- Provide feedback.

2.2 Group discussion exploring the Strengthening Social Support strategy (20 minutes)

Learning outcome: trainees can name different types of social support and know how to access them.

Tell trainees (1 minute)

- Session 4 will introduce the topic of social support, how adversity affects social support and the ways people can seek this support.

Facilitate a discussion (10 minutes)

- Ask what "social support" means to the trainees. If they need prompting, mention:
  - spending time with others such as when sharing a meal;
  - talking with someone you trust about problems and concerns;
  - getting help to complete a task; or
  - getting information about where to get practical help.
- Discuss where people can obtain social support. Examples should include family and friends, but also state services, and support from institutions (such as departments of social services, local community-based organizations, employers, and organizations campaigning for workplace support).

- Ask the trainees what they would do if they needed social support. They will probably say they would reach out to family or friends but encourage them to think further about how they might explore local opportunities.

Tell trainees (9 minutes)

- Strengthening Social Support is the final strategy introduced in PM+.
- It focuses on the following types of social support. Talk through the supports listed in the *Key concepts* box, perhaps listing them on a flipchart or showing them as a slide.

KEY CONCEPTS

- Emotional support from a trusted other
- Support from an organization/agency
- Practical support
- Social connection (being with others without talking about problems)
- Helping or supporting others.

- Direct the trainees to **Chapter 9 in the PM+ intervention manual,** and ask them to read along as you explain that the steps to delivering the Strengthening Social Support strategy are:
  - educating the client about the importance of strengthening social support;
  - exploring which social support the client would like to strengthen (remembering to consider all the different types); and
  - helping the client to decide how they want to strengthen their social support and creating an action plan for this.

2.3 Trainees role-play delivering the Strengthening Social Support strategy (70 minutes)

Learning outcome: trainees practise how to deliver the Strengthening Social Support strategy.

Prepare in advance

You will need to prepare the instructions for the Module 9 role-play on delivering the Strengthening Social Support strategy.

Have copies of trainees' PM+ competency assessment tools ready.

Organize trainees (10 minutes)

- Divide trainees into pairs and give each pair the instructions for the Module 9 role-play. They can also use the protocol in **Appendix G of the PM+ intervention manual**.
- Explain that each person will have a turn playing the role of helper and client. Role-plays will be stopped after 15 minutes. Trainees can have five minutes to have a break or to give feedback, and then swap roles and role-play for another 15 minutes.

Run trainee role-plays (40 minutes)

- Walk around the room to observe the role-plays. You can give individual feedback as needed. Have trainees' PM+ competency assessment tools ready so you can informally rate trainees. The relevant item for this role-play is PM+ 16 (strengthening social support).

Facilitate a discussion (20 minutes)

- Bring the class back together and give some group feedback based on your observations.
- Invite trainees to share their observations and discuss the challenges they experienced, or to ask questions.

Lesson 3
Coping with challenges to Strengthening Social Support 75min

Prepare in advance

Depending on the size of your class, you may need to adjust the time allotted for Lesson 3.1 and Lesson 3.2.

Print and share copies of Annex 13.

You may wish to prepare a flipchart of the main points trainees should include in their scenario in Lesson 3.1.

3.1 Small group discussions. Challenges to engaging in Strengthening Social Support (25 minutes)

Learning outcome: trainees rrecognize types of challenges clients may have with strengthening social support.

Tell trainees (1 minute)

- This activity helps trainees to consider which kinds of challenges clients may have with the Strengthening Social Support strategy.

Organize trainees (4 minutes)

- Divide trainees into groups of three.
- Give the groups the scenario in Annex 13 and tell them to read the sections on trust and sexual assault in **Chapter 9 of the PM+ intervention manual**.

Facilitate small group discussions (15 minutes)

- Tell each group they have 15 minutes to think of and describe a scenario for a client who would find Strengthening Social Support to be a challenging strategy.
- Suggest that the groups can use the scenario in Annex 13 and **Chapter 9 in the PM+ intervention manual** to give them ideas.
- Tell trainees that their scenarios should include the following:
  - the main reason for the difficulty;
  - a history of what might have contributed to the difficulty;
  - how the client might act during the session;
  - how the helper could adapt Strengthening Social Support to help this client engage with the strategy.

Facilitate a large group discussion (10 minutes)

- Bring the group back together.
- Briefly explore the scenarios (ask a volunteer to read out the scenario for the group), focusing on how the helpers could support a client who is struggling to connect with social supports.
- Finish the discussion by paying particular attention to the points in the *Key concepts* box. The trainees may choose to adapt their scenarios as a result of the discussions.

KEY CONCEPTS

Strengthening Social Support

- Strengthening Social Support can be adapted to suit the client, but the strategy should still cover all different types of support (e.g. emotional support, support from organizations and institutions, practical support, support from peers and family and seeking for social connections).
- The helper should try to ensure, as much as is possible, that the client can trust the person or organization they seek support from.
- The strategy should be practised in a gradual way, with tasks broken down into small steps so the client gradually confronts the situation (i.e. trusting someone, seeking support from someone).

3.2 Trainees role-play the engagement of a reluctant client with Strengthening Social Support (50 minutes)

Learning outcome: trainees practise how to engage a reluctant client with strengthening social support.

Organize trainees (2 minutes)

- Ask trainees to stay in their groups of three, with one person becoming the helper, one the client and one an observer.
- The groups should then role-play their scenario from the previous exercise, drawing on the protocol suggested in **Appendix G of the PM+ intervention manual** for the Strengthening Social Support strategy.

Run trainee role-plays (30 minutes)

- Have trainees' PM+ competency assessment tools ready, and informally rate one or two role-plays per trainer. PM+ 16 (strengthening social support) is the most directly relevant item to this role-play, although you may want to rate other items too.

Facilitate a discussion (17 minutes)

- Ask trainees to reflect on adaptations they made to the strategy to support this client.
- They should identify any challenges they faced and any final changes they would make. List these on a flipchart.
- Ask one person from each group to share their list of challenges and any adaptations they made (for no more than two minutes each). Give feedback on these adaptations, to ensure that trainees have followed the PM+ protocol.

Lesson 4

Summary, home practice and setting up the review 25min

Prepare in advance

Supply paper for the home practice task.

4.1 Summary (10 minutes)

Learning outcome: trainees reflect on how to pull together and conclude PM+ Session 4.

Tell trainees

- Give a brief summary of the topics covered in PM+ Session 4, and provide an opportunity for trainees to ask questions.
- Remind the trainees how you would pull together PM+ Session 4 by starting with a PSYCHLOPS assessment and general review, then moving to a review of Managing Problems and Get going, Keep doing home practice, and then introducing the Strengthening Social Support strategy.
- Ask for a volunteer to run a Stress Management exercise to demonstrate the ending of PM+ Session 4.

4.2 Setting home practice of Strengthening Social Support (10 minutes)

Learning outcome: trainees practise individualizing the Strengthening Social Support strategy on themselves.

Organize trainees

- Ask the trainees to use the Strengthening Social Support strategy on themselves as their home practice so that they can experience what it is like to be a client.
- Give everyone a piece of paper and ask them to write down headings that they should complete:
    - my social supports (as many as possible);
    - supports I want to strengthen and why;
    - how I could strengthen those supports;
    - an action plan (perhaps on the other side of the paper).

- Tell trainees that, at the beginning of the next module, they will be asked to share their social supports and action plans if they feel comfortable at doing so.

4.3 Set up the review for next time (5 minutes)

Organize trainees

- Remind trainees who are to facilitate the review session at the start of the next module.
- Remind them to focus on key information they have learned from this module. Provide assistance if they require it.
- Encourage them to plan how they will run this review in their own time. They will be in charge of sourcing materials if they need any but, if possible, the trainer can help them.

Module 9 role-play

Strengthening Social Support

These instructions are for Module 9, Lesson 2.3.

Instructions to the person role-playing the PM+ helper

Explain why the Strengthening Social Support strategy is important for the client. Use the dialogues and scripts in Appendix G of the PM+ intervention manual. Make sure that you individualize the strategy to suit your client.

Instructions to the person role-playing the PM+ client

FEMALE SCENARIO

You are a 44-year-old mother who lives in a very poor district. You attended the clinic reporting that you feel irritable all the time. You describe constantly feeling angry for no reason and you become upset talking about the effect this is having on your children. You say you often yell at them and even sometimes hit them.

You also describe being physically agitated, often forgetting to eat, and having significant problems with sleeping. Your anger and agitation are beginning to interfere with your functioning – you find that you just do not have any interest in doing the things that used to give you pleasure. You continue to work to make a living but say you need to put a lot of effort into maintaining your motivation to do this. You are particularly worried about your friends and family seeing you this way, so you have stopped seeing them recently.

MALE SCENARIO

You are a 23-year-old man studying economics at university. You are the oldest of four children and the only male. You explain that your father puts a lot of pressure on you to achieve high marks so you can secure a good job to help support the family and bring honour to the family name. You say that you always had this pressure in school and usually performed well.

However, more recently, you have become very anxious about assignments and examinations – and especially doing oral presentations. You received a poor mark on an oral presentation six months ago which affected your confidence, and you have felt anxious since then. You have another presentation coming up and have been worrying about it every day. You feel sick and sometimes becomes panicky when you try to write your presentation. You know you cannot avoid doing it but you don't know how to get through it.

CLIENT PROMPTS

If the person role-playing the PM+ helper asks you to identify social supports in your life, say: "Why is this important? At the moment I don't have the time or energy to spend time with people."

At the beginning of the strategy, find it difficult to focus on any positive social supports. However, if the PM+ helper guides you to identify why some supports are positive, then be convinced by them.

Module 10

Staying well and looking forward 4h20min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Review and introduction (30 minutes)			
1.1	**Review components of PM+ sessions 1–4 (20 minutes)**	Trainees practise facilitating a review	Flipcharts listing the aims of PM+ sessions 1–4
1.2	**Introduction to PM+ Session 5 (10 minutes)**	Trainees understand what to expect in PM+ Session 5	Flipchart of the components of Session 5 and Table 11 showing the aims
Lesson 2. Helping a client to stay well, to imagine helping others and to look to the future (165 minutes)			
2.1	**How to introduce the aims of Session 5 to a client (25 minutes)**	Trainees know how to introduce the concept of staying well	**PM+ intervention manual Recovering and staying well. Appendix G PM+ intervention protocols for Session 5**
2.2	**Demonstration role-play on talking about staying well (50 minutes)**	Trainees see how to deliver the staying well part of PM + Session 5	
2.3	**Small group discussions on how imagining helping others can reinforce the PM+ strategies (50 minutes)**	Trainees understand that talking about how to help others can reinforce the PM+ strategies	Print and/or adapt scenarios from Annex 14
2.4	**Small group role-plays on encouraging a client to look to the future (40 minutes)**	Trainees explore how to use the discussion on looking to the future	Print and/or adapt scenarios from Annex 15

No.	Description	Learning outcome	Advance preparation
Lesson 3. Ending PM+ (35 minutes)			
3.1	**Group discussion on ending the PM+ intervention (20 minutes)**	Trainees know how to end the PM+ Intervention	
3.2	**Group discussion: When PM+ can and cannot be extended beyond five sessions (15 minutes)**	Trainees know when and how PM+ can be extended	
Lesson 4. Summary and preparation for PM+ skills-building (30 minutes)			
4.1	**Summary (10 minutes)**		
4.2	**Preparation for PM+ skills-building sessions (20 minutes)**	Prepare the trainees for the Module 11 skills-building sessions	

Lesson 1

Review and introduction 30min

Prepare in advance

It will help the Review lesson if you have the flipcharts outlining Sessions 1–4 to hand.

Have a flipchart or slide showing the components of Session 5 and Table 11, showing the session's aims and how these are achieved.

1.1 Review components of PM+ Sessions 1–4 (20 minutes)

Learning outcome: trainees practise facilitating a review.

Organize trainees

- Nominated trainees lead the review session (up to 10 minutes).
- Trainer answers questions or clarifies misinformation about the previous module's content.
- Trainer then leads a review of the trainees' home practice on strengthening social support. Ensure that trainees reflect on how easy or hard it was to use the Strengthening Social Support strategy and fit it into their home life.

1.2 Introduction to PM+ Session 5 (10 minutes)

Tell trainees

- Explain the components of PM+ Session 5:
  - PSYCHLOPS and a general review (20 minutes)
  - discussion and education on how to stay well (30 minutes)
  - imagining how to help others (20 minutes)
  - looking to the future (15 minutes)
  - ending the programme (5 minutes).
- Tell trainees that Session 5 takes steps to end the intervention, and point out that these are as important as the steps taken to start the intervention.
- Talk through Table 11 to explain how the aims of Session 5 are achieved.

TABLE 11
Session 5 aims, and how these are achieved

Session 5 aims	How aims are achieved
Discuss how the client can maintain the gains they have made, and not relapse into the same mental health problems.	• Introduce the concept of staying well • Use scenarios to help the client to consider how PM+ strategies might help other people or situations. • Help the client to prepare for future stressors .
Support the client in setting future goals.	• Set realistic goals with the client and review how to use PM+ strategies to support these.

Lesson 2

Helping a client to stay well, to imagine helping others and to look to the future 165min

Prepare in advance

Trainees will need their PM+ intervention manual.

Print out the scenarios in Annex 14 to give to the trainees for their discussions and role-plays.

2.1 How to introduce the aims of Session 5 to a client (25 minutes)

Learning outcome: trainees know how to introduce the concept of staying well.

Organize trainees (10 minutes)

- Ask trainees to read **Chapter 10 in the PM+ intervention manual** and the part of the protocol in Appendix G that covers Session 5.
- You could ask them to read these sections to themselves or have someone volunteer to read them aloud.

Facilitate a brief discussion (15 minutes)

- Start the discussion by reinforcing the steps that trainees should take to introduce the concepts in Session 5:

  1. Praise the client for reaching the end of PM+.
  2. Explain that the client can expect symptoms to return, especially in times of significant stress, but the symptoms are likely to be less severe, less long-lasting and more manageable than before.
  3. Remind the client that they can respond to future symptoms of emotional ill-health by practising the PM+ strategies and using these to stay well.
  4. Reinforce the client's understanding of the PM+ strategies that they have learned (using the technique of thinking how to help others).
  5. Help the client to look to the future.

- Invite any thoughts and comments/reflections from the trainees.
- Explain that these steps will be explored in more detail in the rest of this lesson.

2.2 Demonstration role-play on talking about staying well (50 minutes)

Learning outcome: trainees see how to deliver the staying well part of PM + Session 5.

Organize trainees (2 minutes)

- Ask your co-trainer or a volunteer to role-play the client (they can use any of the previous scenarios) but tell them to act anxiously about finishing treatment so that you can demonstrate how to manage this anxiety.
- Tell the rest of the trainees that, as they watch the role-play, they should follow **Appendix G of the PM+ intervention manual, Session 5, Step 2**.
- Explain that you will not role-play Step 1 and they should assume that the session started with a general review and PSYCHLOPS.

Run demonstration role-play (30 minutes)

- Demonstrate how you would deliver Step 2 on "Introducing stay well" to the client. Use the protocol notes in **Appendix G of the PM+ intervention manual**.
- Invite feedback from the trainees and give them an opportunity to ask questions.

2.3 Small group discussions on how imagining helping others can reinforce the PM+ strategies (50 minutes)

Learning outcome: trainees understand that talking about how to help others can reinforce the PM+ strategies.

Tell trainees (5 minutes)

- After introducing Staying well to the clients, the PM+ helper should move on to Step 3 on "Imagining how to help others".
- Direct trainees to **Step 3, Appendix G. Session 5 Protocol**.
- Helpers should run this activity with their client in order to explore how well the clients have understood the different strategies.
- Explain that the trainees are going to practise it now. Ask them to close their manuals.

Organize trainees (5 minutes)

- Divide the trainees into at least four small groups.
- Give each group the scenarios from Annex 14.
- Give each group a large piece of paper.

Facilitate small group discussions (20 minutes)

- Ask the trainees to read out the case scenarios, discuss them and write down:
  - the strategies they believe are most helpful for the client in the scenario;
  - the problems that each strategy will help the client to address;
  - how helpful each strategy will be.

Facilitate a discussion (20 minutes)

- Bring the whole class back together.
- Ask one person from each group to present their responses to the whole class (allow up to 2 minutes per group).
- Discuss the responses and clarify any mistakes or misunderstandings about each strategy.
- Compare the strategies chosen by the group with the strategies described in **Appendix F of the PM+ intervention manual.**
- Finish the discussion by reminding trainees that aim of this step is to explore how well the clients understand the different strategies. Asking them to apply the strategies to other people's situations will enable the PM+ helpers to see if they can be flexible and understand the nuances and how to adapt and integrate the strategies into everyday life. The assumption is that, if they can do if it for other people, they can do it for themselves.

2.4 Small group role-plays on encouraging a client to look to the future (40 minutes)

Learning outcome: trainees explore how to use the looking to the future discussion.

Prepare in advance

Trainees will need their PM+ intervention manuals.

Print out the scenarios in Annex 15 to give to the trainees for their discussions and role-plays.

Tell trainees (2 minutes)

- After introducing the topic of Staying well, explore how confident the clients are at using the strategies (Step 3: Imagining how to help others).
- Step 4 asks PM+ helpers to support clients to look to the future.

Organize trainees (3 minutes)

- Tell trainees to stay in the same small groups.
- Read out the helper script that introduces and delivers the "looking to the future" technique from **Chapter 10 and Appendix G of the PM+ intervention manual.**
- Give the groups a few minutes to organize themselves so that someone plays the PM+ helper, someone else the client and the rest are observers. The person playing the client can choose one of the scenarios described in Annex 15.

Run trainee role-plays (15 minutes)

- Walk around the room observing as many role-plays as possible in order to give generalized feedback.

Facilitate a discussion (20 minutes)

- After the end of the role-plays, ask trainees to reflect on the role-plays and discuss any challenges they foresee.
- Ask for their views on the "staying well" and "looking to the future" techniques.

Lesson 3

Ending PM+ 35min

Prepare in advance

Trainees need their PM+ intervention manuals.

3.1 Group discussion on ending the PM+ intervention (20 minutes)

Learning outcome: trainees know how to end the PM+ intervention.

Facilitate a discussion (18 minutes)

- Lead a discussion on how to draw Session 5 to a close and end the PM+ intervention for the client.
- Ask trainees to contribute ideas on how they would have this conversation and ensure that the discussion reflects the following actions:
    - Start by thanking and congratulating the client.
    - Wish the client luck with their recovery.
    - Remind the client to keep working on their strategies.
    - Discuss the post-intervention assessment.
    - Consider arranging a follow-up meeting a few months later (check whether the client has plans to move or will stay in the area).
- Finish the discussion by encouraging trainees to reflect on culturally appropriate ways they could consider adapting the actions to fit their context – i.e. are there any traditional customs or ways to thank people and wish them luck in the future?

Tell trainees (2 minutes)

- Tell the trainees that the PM+ helper could do the post-intervention assessment within Session 5 if there is time.
- However, normally the post-intervention assessment is done 1–2 weeks after the last session.
- This enables a more accurate picture of how the client is coping and adapting the strategies in their everyday lives.
- Remind the trainees that instructions for post-assessment questionnaires can be found in Chapter 4 of the PM+ intervention manual.

3.2 Group discussion: When PM+ can and cannot be extended beyond five sessions (15 minutes)

Learning outcome: trainees know when and how PM+ can be extended.

Tell trainees (5 minutes)

- Tell them that it can be difficult to end the intervention when a client is still in distress after five sessions.
- In such a situation, the best thing to do is for the helper to discuss the client's situation with a supervisor and to focus on what the client actually needs.
- The helper and supervisor might agree to support the person in continuing to practise the PM+ strategies independently but with regular follow-up meetings. For instance, a client who has taken longer to feel comfortable trusting their helper, and only begins to show improvement in later sessions, may simply need more time.
- However, if the client is demonstrating severe distress or talking about thoughts of suicide then they must be referred to a mental health specialist.
- Similarly, if the client has engaged well in PM+ but there has not been much change to their distress, then more sessions are unlikely to help. Again, a referral to a mental health specialist would be needed.

Facilitate a discussion (10 minutes)

- Invite trainees to ask questions about these decisions or raise challenges they expect they might have.
- Encourage trainees to use supervision sessions to discuss clients who may benefit from additional PM+ sessions.
- Finish the discussion by reminding trainees that it can be really hard to end help, especially if people are still going through distressing times in distressing situations.
- It is not possible for the PM+ helper to make the clients' lives better or to make them feel better. Instead, the PM+ helper's role is to give clients the PM+ tools and to support them to use these tools in their everyday lives. This way the clients can become their own helpers.
- Explain it is essential to look after the health and well-being of the helper. Explain that trainees will explore this further in Module 12.

Lesson 4

Summary and preparation for the Module 11 skills-building sessions 30min

Prepare in advance

You will need the overview of PM+ from the PM+ intervention manual for the summary discussion.

You will need to assign trainees to their roles and sessions ahead of Module 11, and therefore you must have prepared Module 11 before delivering the final lesson in Module 10. Refer to *Part A. General instructions for delivering the modules* (see section on Preparing for the extended role-plays in Module 11 earlier in this manual, as well as the instructions in Module 11 itself.

4.1 Summary (10 minutes)

Tell trainees

- Provide an overview of the PM+ sessions with trainees.
- Give them time to ask questions. Clarify any concerns they have about delivering the intervention as a whole or as individual sessions.

4.2 Set up expectations for Module 11 (20 minutes)

Tell trainees (5 minutes)

- Explain that the next step in the training is to build trainees' skills and confidence by spending a day practising the delivery of PM+.
- Tell the trainees that the role-plays in Module 11 will be longer (90 minutes), in order to give everyone an opportunity to practise pulling a whole session together.
- Explain that Sessions 1–3 will be role-played.
- Trainees will be working in groups of two or three, with one person role-playing the helper for the full 90 minutes. (If you have recruited independent people to role-play the clients, the other trainees will observe the role-play. If you do not have external clients, one of the trainees will role-play a client while the other observes.)
- Be clear that these roles will change as the groups move between sessions, so everyone will play a helper for one full session.
- Remind trainees that you and your co-trainer(s) will also be observing the role-plays, in order to provide feedback. If additional trainers are to be involved, let the trainees know now.

Organize trainees (15 minutes)

- Assign trainees to the groups you have decided on and tell each trainee in which session they will role-play the helper and in which one they will role-play the client.
- Ask them to prepare their client scenarios (they can use any of the scenarios used previously in the training) and to revise their helper role for the sessions they have been assigned before the next training day. Tell them they are expected to use the scripts in **Appendix G of the PM + intervention manual.**
- Tell the trainees that within Module 11 they will each get at least 10 minutes of personalized feedback from a trainer, which will draw on the Module 11 role-plays but also on observations of other role-plays during the training. Tell them this is also an opportunity to ask questions about anything they need to have clarified.
- Finally, remind trainees that the assessments are not intended as a way to pass or fail them. Rather, the aim is to support their learning, recognize their strengths and direct their attention to any weaknesses.

Module 11

Skills-building 6h40min

No.	Description	Learning outcome	Advance preparation
Lesson 1. Trainees role-play delivering PM+ sessions 1–3 (up to 8 hours)			
1.1	**Introduce how and why competency-based assessments will be used (45 minutes)**	Trainees are reminded what to expect from Module 11 and are reassured that assessments are supportive	Refer to Part A How to use PM+ competency assessments Have a timetable ready to display
1.2	**Skills-building role-plays (5 hours 15 minutes**	Trainees gain confidence in delivering a whole PM+ session	
1.3	**Trainees reflect on the skills-building day and trainers give group feedback (40 minutes)**	Trainees reflect on the skills-building day and know where they still need support in supervision	

Lesson 1

Trainees role-play delivering PM+ Sessions 1–3 up to 8h

Prepare in advance

This module is a full day of skills-building with accompanying competency assessments. As such, it requires more preparation than most of the other modules. You must already have done the preparation set out in *Part A. General instructions for delivering the modules (See section:* Preparing for the extended role-plays in Module 11*)*. You must also have assigned the trainees' roles at the end of Module 10 and given them time to prepare.

You will already have allocated the trainees into groups of three. Make sure you have enough space for the groups to work separately, and that you and any co-trainers can move around easily to observe.

Have your timetable for the role-plays ready to display on a flipchart or as a slide.

You should have a list of trainees. Use it to divide the trainees between the trainers. Decide who will be responsible for observing each trainee, and also who will give each trainee their personalized feedback. You can tick off the names as you go, so that no-one is missed.

Remember that, when using the PM+ competency assessment tool, it may not be possible to assess trainees against all of the competencies. The idea is that the role-plays observed throughout the training and during this skills-building day should help to build up a good picture of your trainees' strengths and areas for improvement that can continue to be supported during the supervision process.

During the role-plays, make notes to prepare for the group feedback session at the end of the day.

1.1 Introduce how and why competency-based assessments will be used (40 minutes)

Learning outcome: trainees are reminded what to expect from this module and are reassured that assessments are supportive.

Tell trainees

- Explain that a skills-building day is an essential part of any PM+ training.
- Remind trainees that they will be working in groups of three, delivering PM+ sessions 1–3, with trainers observing.

- Explain that at some point in the day the trainers may take the trainees who are not actively part of the role-play aside in order to give them personalized feedback about their progress with the training.
- Reassure everyone that they will not be given a pass or fail grade. Instead, the feedback will be based on the trainee's PM+ competency assessment tools. These are a way to ensure that PM+ is delivered to a high standard and according to the PM+ protocols.
- Remind them that the PM+ competency assessment tool helps both the trainee and trainer/supervisor to understand which elements of the PM+ intervention the trainee is comfortable in delivering (i.e. areas of strength) and also any areas where more practice is needed (i.e. areas for improvement).
- Answer any questions or concerns that trainees may have.
- Also remind the trainees that the feedback time is also an opportunity to ask questions and discuss any concerns about delivering PM+ effectively. Trainees should see the feedback time as an opportunity to say how they want to be supported during in-field practice and supervision.
- Emphasize that trainees should not feel pressured by the PM+ competency assessments but should see them as guides to learn about their own strengths and weaknesses as PM+ helpers and to refine their skills when needed.

1.2 Skills-building role-plays (6 hours 40 minutes)

Learning outcome: trainees gain confidence in delivering a whole PM+ session.

Organize trainees

- Show your pre-prepared timetable (see Table 1, Part A), which should include breaks. Remind trainees they will all take turns at being the helper.
- Tell the trainees that after each 90-minute role-play, there will be 15 minutes for the group to reflect and give each other feedback.
- Check that trainees know which session they are delivering and remind them that they are expected to use the scripts in **Appendix G of the PM + intervention manual.**
- Disperse the groups around the training area and tell them to start with the first role-play.
- Remember to signal the end of each session and the time for lunch and other breaks.

Run the final role-plays

- During the role-plays, walk around and observe quietly, without commenting or interfering unless trainees have completely misunderstood the instructions. Make notes on the trainees' PM+ competency assessment tools. Plan to spend 10–20 minutes observing each trainee.
- Over the course of the day, give every trainee at least 10 minutes of personalized feedback based both on the Module 11 role-plays and on their competency PM+ competency assessment tools from earlier in the training. You may need to take people aside during the

sessions that they are observing, or fit individual meetings into breaks and lunchtime etc. Individualized feedback should give the trainees a clear picture of their strengths and any areas for improvement to be supported during supervision.

Tips for giving individual feedback

- Emphasize strengths or things helpers did well.
- Discuss areas of weakness and correct any mistakes that were made. Avoid just telling helpers how to correct these mistakes. Instead, ask them to suggest ideas for how to improve.
- Invite trainees to give themselves feedback on their strengths and weaknesses.
- Invite trainees to consider their competency assessments and where they want to improve as they do their in-field practice cases.
- Remind trainees that competency assessments feed into ongoing support and training during supervision. Discuss what helpers should expect to continue learning through their supervision sessions.

1.3 Trainees reflect on the skills-building day and trainers give group feedback (40 minutes)

Learning outcome: trainees reflect on the skills-building day and know where they still need support in supervision.

Facilitate a discussion (up to 40 minutes)

- Be creative – you can think of different and engaging ways to ask for trainee feedback.
- However, give trainees the opportunities to reflect on:
  - what they found easy or difficult and what they need more practice doing;
  - (individually) which competencies they want to strengthen during in-field practice and supervision; and
  - how they plan to work on these.
- Provide feedback to the whole group, based on some of the trends you saw during observations and from the PM+ competency assessment tools, but without revealing names.
- Also without revealing names, discuss any common weaknesses or mistakes that trainees made, and ask the group to identify ways in which these could be corrected.
- Finish the discussion on a positive note so that the trainees walk away feeling motivated and competent.
- Finally, congratulate the trainees on their efforts during the skills-building day, and tell them that the next (and final) module will discuss supervision.

Module 12

Supervision 3h15min

No.	Description	Learning outcome	Advance preparation
Review and introduction (30 minutes)			
1.1	**Review of the skills-building sessions (25 minutes)**		
1.2	**Introduction (5 minutes)**	Trainees know what to expect from the module	
Lesson 2. Supervision explained (60 minutes)			
2.1	**Group discussions on why supervision is important to delivering PM+ (30 minutes)**	Trainees know the role that supervision plays in delivering PM+	
2.2	**Group discussion on how to engage with supervision (30 minutes)**	Trainees know how to engage with supervision in their local setting	Print the example and blank supervision forms for each trainee (Annex 16)
Lesson 3. Self-care to manage a helper's own distress (60 minutes)			
3.1	**Reflection and small group discussions (60 minutes)**	Trainees understand the importance of self-care and how to manage personal distress through supervision	Flipchart paper and pens
Lesson 4. Concluding the training (45 minutes)			
4.1	**Concluding the training (45 minutes)**	Trainees understand how to engage with the next steps of delivering PM+	

Lesson 1

Review and introduction 30min

Prepare in advance

Have some feedback ready to give the group after the Module 11 role-plays.

1.1 Review (25 minutes)

Tell trainees

- Start by congratulating the trainees on their efforts during the skills-building session.
- Give positive feedback and a general review of the skills-building day. Ask trainees to share any reflections they may have.

1.2 Introduction (5 minutes)

Tell trainees

- In this final module the trainees will learn about supervision and why it is important to PM+.
- Tell them that the group will discuss different ways in which supervision can be delivered; the importance of engaging with supervision in order to become effective PM+ helpers; the benefits of supervision; and some of the challenges that could arise.
- Explain that this session requires trainees to take ownership of their supervision plan and the ways they can build on their PM+ competency assessments during supervision.

Lesson 2

Supervision explained 60min

Prepare in advance

You may want to prepare the *Key concepts* box as a slide or handout for Lesson 2.1.

You will need blank and example supervision forms Annex 16 ready for each trainee for Lesson 2.2.

2.1 Group discussions on why supervision is important to delivering PM+ (30 minutes)

Learning outcome: trainees know the role supervision plays in delivering PM+.

Organize trainees (2 minutes)

- Divide the trainees into two groups and give them flipcharts and markers.

Tell trainees (8 minutes)

- Tell the groups that supervision within PM+ is not the same as day-to-day management by a supervisor at work. Instead, it is an essential part of good practice. It helps protect and safeguard both the helper and the client. In PM+ supervision forms part of ongoing training.
- Go through the points in the *Key concepts* box with the trainees.

KEY CONCEPTS

Supervision

Supervision is an important part of providing people with effective psychological treatment.

Everyone working in mental health and psychosocial support should participate in supervision, whatever level they work at. Supervision helps protect the client, the helper and the organization implementing the programme.

Supervision that is conducted regularly and well:

- ensures that the quality of PM+ is maintained;
- helps helpers to keep within their personal and professional boundaries;
- helps helpers to feel well-supported and confident in managing difficult client situations, and therefore stops them feeling burnt out, overwhelmed or exhausted.

Helpers should always discuss the risks of self-harm or suicide with a supervisor, and supervisors should always be available to discuss an urgent issue.

Supervision is part of the ongoing training for PM+ helpers. It refines the skills learned in this training and gives helpers access to their supervisor's greater experience.

Facilitate group discussions (20 minutes)

- Tell one group they have 10 minutes to discuss and note down the responsibilities and characteristics of a good supervisor.
- Tell the other group to discuss the responsibilities of a good supervisee (i.e. the PM+ helper).
- After 10 minutes, ask groups to swap their flipcharts and spend five minutes reviewing the other group's ideas. They should add any new points they think of and select the three points they feel are the most important.
- Ask one person from each group to present the top three ideas they selected (give them up to five minutes each).
- Then go through the full list of responsibilities from each group and identify any that are not relevant or appropriate to PM+ supervision. Explain why.

2.2 Group discussion on how to engage with supervision (30 minutes)

Learning outcome: trainees know how to engage with supervision in their local setting.

Tell trainees (10 minutes)

- If possible, now is a good time to talk through in as much detail as possible how supervision of the trainees will happen. For instance:
  - Name the supervisors and which supervision groups the trainees will be in.
  - Explain how the two clients for the in-field training will be chosen and how the trainees will get in touch with them.
  - Explain how the close supervision will be implemented (usually biweekly one-to-one meetings) to discuss how the two clients are engaging and progressing with the intervention.
  - Discuss whether supervision will be face-to-face or online, where it will take place, when it will take place, how long it will last, and how to contact your supervisor etc.
  - Explain how the frequency, modality and duration of supervision sessions may change over time (e.g. with time, move from one-to-one to group supervision, or from in-person to online supervision).
- Explain that the point of this activity is to get the trainees to engage with supervision and to consider how they can fit it into their professional lives.
- If these details are not yet confirmed, you should explain when they will be and how the trainees will be informed.

Facilitate a discussion (20 minutes)

- Ask trainees to reflect on the format that a supervision session may take.
- Ensure that trainees include the following points in their reflections:
  - Supervision often involves further role-plays in which trainees can practise specific skills or areas of PM+ delivery with their supervisors.
  - For case scenarios and discussion of clients' progress, distribute the completed example and the blank template supervision forms (Annex 16) and give trainees the chance to review them. Ask trainees to reflect on these templates and consider how they could be adapted and used to support their development.
  - Offer individual support to each supervisee to keep them motivated, engaged and enjoying offering PM+.
  - Provide a space to support the well-being of the PM+ helper to avoid career fatigue and burn-out.
- Tell the trainees they may also find it helpful to meet with other helpers to discuss their work in a form of peer supervision, but that this must follow all the normal confidentiality rules.

- Meeting with other helpers can help manage helpers' stress and can sometimes deal with relatively minor issues that arise in the job.
- Remind trainees that everyone, no matter what level they work at, takes part in supervision and it is normal for helpers to give feedback to their supervisors and trainers. You could ask for informal feedback during this exercise, or you could give the trainees an opportunity to give you anonymous written feedback if this is more comfortable.

Lesson 3

Self-care to manage a helper's own distress 60min

Prepare in advance

You will need a flipchart and markers to record the discussion. You may want to give trainees paper to record their personal reflections.

3.1 Reflection and small group discussions (60 minutes)

Learning outcome: trainees understand the importance of self-care and how to manage personal distress through supervision.

Tell trainees (17 minutes)

- Explain to the trainees that listening to and working with people who have experienced a lot of adversity can be tiring and distressing.
- It is not uncommon for people to feel affected by, or even overwhelmed by, repeatedly hearing about adversity.
- Tell trainees that for the next five minutes they can quietly ask themselves the following:
  - What are the signs that I am beginning to feel distressed or overwhelmed?
  - How do I currently manage any distress?
- After five minutes, ask the trainees whether they would like to share any personal reflections. Do not force people to talk.
- List common signs of feeling distressed or overwhelmed on your flipchart. Take suggestions from the trainees, but ensure that the list includes:
  - feeling stressed;
  - having a low mood;
  - sleep problems;
  - anxiety;

  - anger or irritability;
  - feelings of hopelessness.

- Now ask trainees to reflect on what they could do to prevent feeling overwhelmed as a result of delivering PM+. List suggestions on the flipchart. They should include:
  - Organize regular check-ins with colleagues and supervisors.
  - Schedule adequate breaks, such as talking with colleagues, doing slow breathing or a similar strategy to manage stress, or doing an enjoyable activity.
  - Avoid having too many PM+ clients at any one time.
  - Know when and how to ask for help from a supervisor (e.g. when thinking about a client's situation is intruding on other tasks or activities).
- Emphasize to the trainees that it is better to ask a supervisor for guidance too often than not often enough.

Organize trainees (3 minutes)

- Divide the trainees into small groups.
- Ask for half the group to represent supervisors and the other half of the group to represent PM+ helpers.

Facilitate small group discussion (25 minutes)

- Ask the people representing the PM+ helpers to explain to the supervisors how they might feel after delivering PM+ regularly for a number of months and what support they might want and need from the supervisor or from peers.
- Ask the people representing the supervisors to create different plans and ideas to support the well-being of PM+ helpers (PM+ helpers can also feed in ideas).

Facilitate a group discussion (20 minutes)

- Bring the full class back together for a brief discussion.
- Ask different groups to share, compare and contrast the different supervision plans.
- By the end of the lesson, ensure that trainees know what they could do to protect themselves from feeling overwhelmed and what kind of support they would need from their supervisors.

Lesson 4

Concluding the training 45min

4.1 Concluding the training (45 minutes)

Prepare in advance

Bring back the dartboard activity from Module 1 and ask trainees to do the activity again to compare how they felt at the beginning of the course compared to the end.

You may wish to prepare a fun activity or game to end the training, or perhaps arrange some form of recognition for completing the course, such as a meal or other event.

Tell trainees

- Thank trainees for taking part in the training and congratulate them on completing it.
- Review what the training aimed to achieve (such as getting people ready to help their community), and the progress the class has made – especially areas of significant improvement.
- Highlight any areas that remain a challenge and encourage trainees to monitor their progress with these. Remind them to continue to talk with other helpers and supervisors about challenging issues while maintaining confidentiality. Tell trainees to focus particularly on areas they still need to develop when they begin their practice cases.
- Repeat the dartboard exercise from Module 1 to measure whether the trainees feel more confident as a result of the training. On a large sheet of paper, draw a circle with six equal segments, and in each write one of the following statements:

  I know PM+ (I am familiar with PM+ and its strategies).

  I can do PM+ (I am confident delivering PM+ strategies to others).

  PM+ clients (I know who is and who is not suitable for PM+).

  PM+ helper (I understand the role of a PM+ helper).

  Helping skills (I can use basic helping skills to help someone feel comfortable sharing difficult personal information in PM+).

  Following the manual (I understand why it is important to keep to the PM+ manual when delivering PM+).
- Ask trainees to mark on the poster where they feel in relation to each statement. Closer towards the centre means they feel increasingly confident. Towards the edge means they feel less confident.
- Compare their confidence now with the dartboard they created on the first day, and make a positive comparison that highlights progress. You can also remind trainees they are still new

to PM+ and they will gain more confidence (and receive more training) by seeing clients and attending supervision.

- While you have the trainees gathered around the poster, ask each in turn to say one word that reflects how they feel about the training. Use this as a short evaluation to ensure that everyone is feeling OK with their progress.

Tell trainees that the training is over. You might want to end with a round of applause!

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Part C:

Annexes

Annex 1

The PM+ competency assessment tool

Citation: The World Health Organization, The Problem Management Plus (PM+) Competency Assessment Tool, WHO, Geneva, 2025. https://equipcompetency.org/en-gb. (note this version does not include the two competencies related to Group PM+.)

Foundational helping and problem management plus competencies

1 .Non-verbal communication (ENACT #1)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Engages in other activities (e.g. answers mobile phone, completes paperwork) ☐ Laughs at client ☐ Uses inappropriate facial expressions ☐ Inappropriate physical contact	☐ Allows for silences ☐ Maintains appropriate eye contact ☐ Maintains open posture (body toward client) ☐ Continuously uses supportive body language (head nod) and utterances (uh huh) ☐ None of the above	☐ Completes all basic helping skills ☐ Varies body language during the session in relation to client's content and expressions

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

2. Verbal communication (ENACT #2)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Interrupts client ☐ Asks many suggestive or leading closed-ended questions (e.g. "You didn't really want to do that, right?") ☐ Corrects client (e.g. "What you really mean…") or uses accusatory statements (e.g. "You shouldn't have said that to your husband") ☐ Culturally and age-inappropriate language and terms	☐ Uses open-ended questions ☐ Summarizing or paraphrasing statements ☐ Allows client to complete statements before responding ☐ None of the above	☐ Completes all basic helping skills ☐ Encourages client to continue explaining (e.g. "Tell me more about…") ☐ Clarifies statements in first person (e.g. "I heard you say", "I understood…") ☐ Matches rhythm to client's, allowing longer or shorter pauses based on client

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

3. Explain and promote confidentiality (ENACT #3)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Forces client to disclose to helper or others ☐ Describes confidentiality inaccurately (e.g., "I will only tell your family") ☐ Promises full confidentially without exceptions ☐ Minimizes client's concerns about confidentiality (e.g. "It doesn't matter if anyone else hears us")	☐ Explains concept of confidentiality ☐ Lists exceptions for breaking confidentiality for self-harm or harm to others ☐ Explains why it can be important to break confidentiality ☐ None of the above	☐ Completes all basic helping skills ☐ Details the referral process related to confidentiality and exceptions ☐ Asks questions to assess client's understanding of confidentiality ☐ Topics of discussion are appropriate to confidentiality of setting

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

4. Rapport-building and self-disclosure (ENACT #4)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Dominates session describing a personal experience ☐ Minimizes client's problems by describing how the helper has dealt with this ☐ Asks unnecessary embarrassing personal questions ☐ Discusses confidential information of other clients	☐ Introduces self and explains role ☐ Makes casual, informal conversation ☐ Asks for client's introduction (e.g. What the client prefers to be called) ☐ Shares general experience related to the client (e.g. about one's community/region) ☐ None of the above	☐ Completes all basic helping skills ☐ Asks client's reflection on information that the helper has shared ☐ Checks in on client's comfort (e.g., offers seat, preffered language)

Check the level that best applies (only one level should be checked)

☐ Level 1	☐ Level 2	☐ Level 3	☐ Level 4
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

5. Exploration and normalization of feelings (ENACT #5)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Makes statements that client's response is unusual or atypical for others in similar situations (e.g. "People don't usually react in this way") ☐ Minimizes or dismisses client's feelings or emotions ☐ Forces client to describe emotions	☐ Appropriately encourages client to share feelings ☐ Explains that others may share similar symptoms, reactions and concerns, given similar experiences ☐ Asks client to reflect on the experience of sharing emotions ☐ None of the above	☐ Completes all basic helping skills ☐ Explores potential reasons for hesitance to share emotions ☐ Comments thoughtfully on client's facial expression to encourage emotional expression ☐ Validates emotional responses while reframing potentially harmful emotional reactions

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

6. Demonstrate empathy, warmth and genuineness (ENACT #6)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Critical of client's concerns ☐ Dismissive of client's concerns ☐ Helper's emotional response appears inappropriate, fake or acting	☐ Is warm, friendly and genuine throughout session ☐ Continuously shows concern or care for the client (e.g. "That sounds sad. Can you tell me more about it?") ☐ Asks question to identify what emotions the client was feeling (e.g. "I wonder if you felt sad or angry when this happened") ☐ None of the above	☐ Completes all basic helping skills ☐ Asks client to reflect on empathic statements from helper (e.g. "What did you think when I said you sounded sad?")

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

7. Assessment of harm and developing a response plan (ENACT #7)

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Does not ask about self-harm ☐ Lectures client with religious or legal arguments against self-harm (e.g. "This is sin" or "This is against the law") ☐ Expresses disbelief (e.g. accuses client of discussing self-harm to get attention; states that others would not actually harm the client or the client's children) ☐ Encourages client not to tell anyone else about self-harm or harm to others	☐ Asks about self-harm or harm to others, or explores harm if raised by client ☐ Asks about current intent, means or prior attempts ☐ Asks about risk and/or protective factors ☐ None of the above	☐ Completes all basic helping skills ☐ If indicated, helps client to develop safety plan (e.g. coping strategies and help-seeking)

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

8. Introducing and explaining slow breathing

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Forces client to practise breathing strategy ☐ Criticizes client on performance (“That’s all wrong; you need to do it this way”) ☐ Rushes through practice or uses a loud or harsh voice ☐ Gives nonspecific unhelpful suggestions (e.g. “Try breathing better”)	☐ Explains benefit of slow breathing ☐ Demonstrates technique, including appropriate hand placement (e.g. on stomach) ☐ Guides client through breathing practice, checking comfort level at each step ☐ Paces breathing steps with counting for inhale, holding and exhale ☐ None of the above	☐ Completes all basic helping skills ☐ Reassures client that this does not need to be done perfectly (e.g. “This takes practice; you will know what feels comfortable for you”) ☐ Praises client for practising and normalizes any difficulties or challenges with the practice ☐ Adapts strategy as needed to ensure client’s comfort ☐ Uses metaphors that are understandable to the client (e.g. balloon metaphor)

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

9. Listing and distinguishing solvable and unsolvable problems

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Blames client for problems (e.g. "You made this problem for yourself") ☐ Says all of client's problems are too difficult or cannot be solved ☐ Tells client what their problems are without client's input ☐ Incorrectly explains what are solvable and unsolvable problems	☐ Supports client to list their own problems ☐ Uses a brainstorming process to identify additional problems ☐ Explains the concept of solvable and unsolvable problems ☐ Helps client identify which of their problems are solvable and unsolvable ☐ None of the above	☐ Completes all basic helping skills ☐ Helps client reframe unsolvable problems into solvable problems ☐ Discusses how addressing solvable problems can improve the client's well-being

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

10. Choosing a problem

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Chooses problem for the client ☐ Allows client to choose unsolvable problem ☐ Criticizes client's choice of the most important problem (e.g. "That problem isn't important; focus on something else") ☐ Fails to identify a single problem to focus on	☐ Works with client to rank or prioritize which solvable problems could be addressed ☐ Works with client to select a single specific problem that is solvable ☐ Asks for client's feedback on the selected problem (e.g. "Do you feel comfortable to start working on this problem") ☐ None of the above	☐ Completes all basic helping skills ☐ Helps client explore how their life would be different if the selected problem were addressed ☐ Helps client identify alternative problem if barriers are encountered addressing the selected problem

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

11. Defining the problem

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Starts defining the problem without having made selection of a single problem ☐ Provides a complex or confusing definition of problem that does not have clear actions to take ☐ Tells client the problem is hopeless ☐ Minimizes the problem (e.g. "Solving this problem is easy. Let's focus on something more challenging")	☐ Establishes a specific brief definition of the problem ☐ The definition includes a description of things that can be changed and for which the client can take action ☐ None of the above	☐ Completes all basic helping skills ☐ If a problem has many parts, breaks it down and deals with each part separately. ☐ Asks the client to imagine what their life would be like if the problem were solved

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

12. Brainstorming solutions

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Directly gives solutions to the client rather than have client generate own solutions ☐ Makes judgements about brainstormed solutions (e.g. "That is good" or "That is bad") ☐ Asks suggestive or leading questions about solutions (e.g. "That isn't really how you want to solve it, is it?")	☐ Encourages client to think of as many possible solutions as they can ☐ Encourages client to think of solutions they can do by themselves as well as those with support from others ☐ None of the above	☐ Completes all basic helping skills ☐ Uses the friend strategy (e.g. "What would you tell a friend if they had this problem?") ☐ Prompts client to think of existing personal strengths, resources and social support in order to brainstorm more solutions

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

13. Choosing helpful strategies

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Chooses a solution that is unrealistic or potentially harmful (e.g. "You should run away from home") ☐ Tells client what solution to choose ☐ Is critical of client for chosen solution (e.g. "I would have chosen something else. What you chose may not work")	☐ From the list of potential solutions, helps client choose solutions that are helpful to influence the problem ☐ Helps client discuss the advantages and disadvantages of the different listed solutions ☐ Helps client consider what solutions are achievable within the client's financial, personal and social resources ☐ None of the above	☐ Completes all basic helping skills ☐ Explores how client has solved similar problems in the past and discusses what did or did not work ☐ Explores alternative solutions that could be used if selected solution becomes too difficult

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

14. Developing an action plan for the solution

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Tells client what steps to take to achieve the solution ☐ Discourages client by identifying barriers for carrying out the action plan (e.g. "Your wife will never let you do this") ☐ Criticizes the client's proposed action steps for the solution (e.g. "You aren't thinking through all the steps") ☐ Dismisses concerns raised by the client about the action plan (e.g. "That's easy to do. I've seen lots of people do it. Stop worrying")	☐ Helps client create an action plan with specific steps ☐ Helps client pick the day, time and location for when to do steps in the action plan ☐ Discusses what resources would be needed to carry out the actions (e.g. transportation, childcare, a supportive friend) ☐ Maintains a positive and encouraging attitude throughout discussion of the action plan ☐ None of the above	☐ Completes all basic helping skills ☐ Suggests ways the client can remind themself to carry out the plan (e.g. notes, calendar, telephone reminders, connecting it with other activities) ☐ Discusses alternative strategies if barriers are encountered when carrying out the action plan ☐ Helps client create plan for managing distress during the action plan (e.g. You can use your breathing exercise before talking to your relative) ☐ Helps the client order the action steps if multiple interconnected actions are needed

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

15. Get going, Keep doing

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Makes activity schedule without client's input ☐ Schedules activities that are vague, unachievable or potentially harmful to improve mood ☐ Blames client for lack of motivation, energy, or will-power to implement Get going, Keep doing activities	☐ Explains the inactivity cycle ☐ Helps the client brainstorm enjoyable activities that improve their mood ☐ Helps the client to choose an activity that is enjoyable and is achievable within the next week ☐ Helps the client schedule when (days and times) they will complete the activity or task in the next week ☐ None of the above	☐ Completes all basic helping skills ☐ Helps the client to break down the activity or task into very small and manageable steps ☐ Discusses reminders the client can use to help complete their activity ☐ Connects enjoyable activity with other events or commitments ☐ Reviews potential barriers or challenges to doing the enjoyable activity

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

16. Strengthening Social Support

Check all behaviours demonstrated in each category.

Unhelpful or potentially harmful behaviours	Basic helping skills	Advanced helping skills
☐ Discourages client from contacting other people about their problem ☐ Tells client whom to contact rather than following client's preferences ☐ Criticizes client for difficulties in communicating with family, friends or community members	☐ Explains meaning and benefits of strengthening social support ☐ Supports client to identify multiple sources of social support ☐ Works with client to select whom from the social support network would be helpful ☐ Encourages client to contact social support persons ☐ None of the above	☐ Completes all basic helping skills ☐ Schedules action plan with client, including when and whom to reach out to for support ☐ Discusses overcoming barriers and challenges when contacting persons for social support ☐ Role-plays with client how to ask selected person for support ☐ Recommends skills and techniques to support relationship goals

Check the level that best applies (only one level should be checked)

☐ **Level 1**	☐ **Level 2**	☐ **Level 3**	☐ **Level 4**
Any unhelpful behaviour	No basic skills or some but not all basic skills	All basic skills	All basic helping skills plus any advanced skill

Notes:

Annex 2

Example training schedule

This schedule is an example for a training run over 10 consecutive working days (i.e. over two weeks), assuming six hours of actual training time per day. It is possible to adapt this depending on the local context (e.g. spreading the training over more days).

If trainees have extensive experience and knowledge of delivering other psychological interventions, or if a pre-training assessment has confirmed competency, the training can be adapted. For example, for trainees with knowledge of common mental health conditions and basic helping skills, the corresponding modules may be reduced and more emphasis could be placed on PM+ strategies. Case scenarios and scenes can be adapted for context. Adult learners learn best when they can bring their own experiences to the training.

Week 1

Day 1	Day 2	Day 3	Day 4	Day 5
Module 1	Module 3 (end)	Module 4	Module 5	Module 6 (end)
Module 2			Module 6 (start)	Module 7 (start)
Module 3 (start)				

Week 2

Day 6	Day 7	Day 8	Day 9	Day 10
Module 7 (end)	Module 8 (end)	Module 9 (end)	Module 11	Module 12
Module 8 (start)	Module 9 (start)	Module 10		Close and end training
		Prepare for Module 11		

Annex 3
Training checklist

The checklist below covers some common tasks involved in setting up PM+ training.

1. Venue

☐ Check access to venue, including nearby bus or train stations, parking etc.
☐ Check access to toilets.
☐ Check that the temperature and lighting in the training room are suitable and that the room can be darkened if using a projector or screen.

2. Setting up the room

☐ Consider how to set up the room to encourage participation, comfort and learning activities.
☐ Ensure that the furniture in the room is moveable to make space for energizers and some training activities.
☐ Make sure there is enough space to conduct multiple role-plays at once (e.g. there is space for several small groups of trainees) or that additional rooms are available.
☐ Place a clock visible to all.

3. Materials

☐ Print copies of training handouts and have manuals for trainees.
☐ Pens or pencils.
☐ Whiteboard or flipchart with stand.
☐ Markers.
☐ Computer and projector if using slides or videos.

4. Other

☐ Prepare scenarios for demonstration role-plays. They should describe a typical client who might present at your local setting.
☐ Allocate sessions to trainees for the Module 11 role-plays and form trainees into groups of three.
☐ Prepare scenarios and or arrange for independent persons to help deliver Module 11.
☐ Prepare to provide snacks, water, tea and coffee or meals as appropriate.
☐ Consider whether you need another person to support you with time management, to organize mealtimes, or to write key points from discussion groups on the board or flipchart.

Annex 4
Additional resources for trainers

Welcome, ice-breaker and energizer ideas

Energizers can be used to break the training up and keep the trainees engaged. They can also be used to review learning outcomes continuously. Here are a few ideas:

Pair reviews

Trainees form pairs and each discuss four things that they learned from the previous day/exercise. Each pair should tell the full group one thing they learned. Trainees should try to avoid saying something that has already been mentioned.

Clapping name game

Everyone starts with the rhythm of clapping (hand – knee). To this rhythm the leader starts with their name and then calls out someone else's name. This continues to the rhythm – only skipping one clap at a time.

Fact or fiction

Ask everyone to write down three things about themselves, two of which are true. They read them out and the group votes on which are true or false.

Tall stories

The trainer starts a story but ends with "…suddenly…". The next person has to add to the story with his or her own sentence that ends the same way. Continue the story until everyone has contributed. If you can, tape it and play it back at the end.

Breathing

Ask trainees to lead breathing strategies or other relaxation strategies.

Brainstorming against the clock

This is often best for late in the training, when trainees have learned many things. Have them come together in groups of three or four with a large sheet of paper and markers. Give them five minutes to list all the specific things they have been taught. Bring the class back together to review which group has come up with the most learning points (i.e. points not covered by other groups). You could offer a prize to that group – e.g. to lead the next energizer activity, or be first in line for morning tea.

Annex 5

Handout: how to give feedback during PM+

Step 1: Give positive feedback first

Be specific with your praise and avoid saying only "It was good".

Instead, describe what was positive, e.g. say:

- "You used the basic helping skills appropriately".
- "You had good eye contact and you were able to reflect back what the client was saying in a way that normalized their feelings".

Give an example of when you noticed the ideal behaviour. For example, you might say:

- "When the client was reluctant to engage in 'Get going, Keep doing' you reflected back her concerns in a gentle and understanding way. Then you repeated the steps for this strategy in a way that made sense to her and helped her see that this strategy may be helpful for her. You then reassured her that you would help her increase her activity slowly."

Describe how a behaviour helped the client. For example, you might say:

- "By demonstrating basic helping skills and going over the steps for Get going, Keep doing, the client felt listened to and that their concerns were valid. It also helped them to understand and engage in the strategy."

Step 2: Provide feedback about areas to improve (only if necessary)

If there is an area where you believe the person needs improvement, or if you need to make a correction of some kind, do so gently using your basic helping skills.

Be specific, give an example, and discuss how this may have affected the client (or explain this is not how PM+ should be delivered). For example, you might say:

- "When you told the client what to do I noticed that she started to close down and stop talking to you as openly. Did you notice that? Why do you think that happened?"

Invite the person to respond. For example, you might say:

- "What do you think? Do you agree or disagree with this comment?"

Annex 6

Handout describing basic helping skills

Offer this handout to trainees to help them understand and use basic helping skills. Print and cut in order to give one skill to each group for role-plays.

Keeping confidentiality

This is a very important part of building trust with clients. They need to know that, when they speak openly about personal things, information is going to remain confidential and private. This is especially true for survivors of intimate forms of traumatic experiences and even more so when there is stigma about the events (e.g. in the case of sexual assault). Clients need to understand that your interactions are confidential, but within limits:

- If the client is at imminent risk of harming themselves or someone else you must [TRAINER TO ADD RELEVANT INFORMATION based on local context].
- Supervision requires you to talk about clients and their progress, but supervisors and services work within the same confidentiality rules.

Communicating concern

Try to understand, as best you can, each client's situation, including the emotions they are experiencing. But it is also important that you do not become too involved in a client's feelings and take them on as your own. This can cause you to feel stressed and over-burdened by your work.

Statements that may show you are concerned include:

- That sounds like it was very challenging/upsetting/frightening (and so on) for you.
- I can see in your face how painful this was for you.
- You have experienced many difficulties.
- You went through a lot.
- I can hear how sad/frightening this was for you.

Verbal communication

The way we speak to a client influences how they feel. Here are some good verbal techniques:

- asking open-ended questions rather than questions that appear to expect a particular answer;
- leaving enough time for the client to finish their reply fully before responding;
- encouraging the client to tell you more;
- showing that you are listening by saying “uh-huh”, “ok”, “I see” or just “mmm”;
- using appropriate language such as culturally- and age-appropriate terms, and avoiding technical words;
- not interrupting;
- summarizing or paraphrasing to check you have understood what was meant, but without appearing to correct or judge a client.

Non-verbal communication

Non-verbal skills communicate to the client that you are listening to them and can also be a way of communicating concern. These include keeping culturally appropriate eye contact, culturally appropriate nodding of your head and, in most cultures, keeping your posture open (e.g. avoiding crossing your arms and sitting with a stiff position or turning away from the client). Sometimes showing similar emotions of your client also shows that you are hearing what they are saying and sympathize with how they are feeling. This might mean expressing sadness on your face when they express sadness. It is important to remember that there can be wide cultural variations in all these actions.

Praising openness

To help a client feel comfortable talking about personal, difficult or embarrassing topics, try to thank or genuinely praise them. You might use local proverbs to encourage the client – e.g. “You double happiness and halve sorrow by sharing what’s on your mind.” You can also praise the client’s efforts to engage in the PM+ strategies and to get better.

Some example phrases that praise openness:

- Thank you for telling that to me.
- You were very courageous in sharing those intimate feelings with me.
- Although it may have been hard to talk about that with me, I think it will be very helpful for your recovery.
- I can see that you are really trying to practise managing stress regularly.

Validating and normalizing

Clients may feel embarrassed at sharing problems with strangers. They might think no one else feels the same way as they do. They may also think that talking about emotions or personal problems is a sign that they are becoming ill, going crazy or that they are weak. Throughout PM+ you can dispel these myths by explaining that many people have similar issues and experiences – i.e. normalizing their problems and feelings. You can validate their reactions as understandable. Normalizing and validating feelings and experiences can lessen shame and build trust.

Some examples of validating and normalizing expressions include:

- You have been through a very difficult experience and it's not surprising that you would be feeling stressed.
- What you have just described is a common reaction people have in these situations.
- Have other people in the group experienced similar reactions/problems?
- Many people I have worked with have also described feeling this way.
- The reactions you have described are very common.
- I am not surprised that you are so scared.

Putting aside personal values

Demonstrating basic helping skills means respecting clients' values and beliefs even if you disagree with them. This can be challenging. However, you should not judge your clients, no matter what they might say to you. You should not allow your personal beliefs or values to influence how you respond to them.

For many, having someone listen without judgement is a new experience that can build trust.

Not giving advice

Don't directly tell people what to do. For example, don't say "You should talk to your boss about the problem." However, it would be OK to say: "Do you think you could talk to your boss about this problem? Would that be possible?"

We are often tempted to advise people, especially if they are feeling hopeless and showing signs of depression. But giving advice can foster dependence instead of helping them manage their own symptoms and recovery.

If a client seems stuck and you are tempted to give advice, ask them what they would advise a loved one in the same situation. This allows them to see things from a fresh perspective, without you explicitly telling them what to do.

Remember: giving advice is different from giving clients important or helpful information. Giving information is something you should do. For example, you might want to tell a client about legal services or other community organizations that could be helpful.

Annex 7

Scenarios for small group discussions on how to give feedback

(FOR USE IN MODULE 3. BASIC HELPING SKILLS)

1. Nadim completed Session 2 with his client. He was obviously well prepared. He covered all the material he was meant to in the session and had all his handouts ready. However, he forgot to complete Step 1 of Managing Problems. In fact, he didn't discuss the problems his client was experiencing at all, but rather asked him to choose the most burdensome problem. At several times, the client clearly wanted to talk about his problems in more detail, but Nadim moved the conversation on in order to complete the Managing Problems strategy. At this point, he did not use his basic helping skills well. However, Nadim did get through the entire session on time and his client was grateful for helping him manage his problem.
Practise giving Nadim feedback.

2. Although Jen did her best in the role-plays during training, it was obvious that she was nervous with her client, probably as this was her first time doing any counselling. Her facial expression, posture and tone of voice all reflected that she felt stressed. Also, she did not seem to understand very well the key concepts that she had to explain.
Practise giving Jen feedback.

3. Kemo is teaching his client the Managing Stress strategy. He was very clear in explaining Managing Stress and obviously understood the key concepts of the strategy well. He used a nice, gentle tone throughout the session and was warm and caring towards his client. However, when it came to teaching the client the steps to managing stress, he did not spot that his client was having difficulties with slow breathing, instead just doing shallow breaths from this chest. Rather, he moved onto the next step of learning to slow the breathing. He also only invited the client to practise slow breathing for less than a minute, before ending the session.
Practise giving Kemo feedback.

Annex 8

Scenarios for demonstrating PM+ assessments

(THESE SCENARIOS ARE FOR USE IN MODULE 4, LESSONS 2.3 AND 3.2.)

Scenario 1

INFORMATION FOR THE CLIENT

You are 26 years old and working at a local hotel as a cleaner. Since the violence in your community two years ago, your life has not returned to normal. You have become withdrawn from your friends and, even though you still live with your mother and two siblings, you simply come and go from the home each day and rarely speak to them. You never leave the home at night, fearing dangers that are not really there anymore. Your mother has expressed serious concern about your isolation and depressed mood. Even though you are still able to go to work each day, you are no longer able to concentrate on your reading, which you used to enjoy. These days you seem to sleep a long time. You have begun to wonder if life is really worth living anymore. You've never told anyone before that you once attempted to take your life (nothing happened except for vomiting and diarrhoea). That was a year ago and you haven't tried since. You haven't thought more about how you might end your life, but the thoughts of escaping your loneliness are with you all the time. You would like to get help, but you don't know how this will affect your work schedule; and you don't want your mother to know – she would ask you questions you cannot bear to answer.

INFORMATION FOR THE HELPER

The main problems identified at pre-assessment are: loneliness, tired all of the time, client has no energy and is unable to enjoy anything. The client's relationship with her mother feels overwhelming, and she feels that the mother is too involved.

Information from Session 1. The client had lots of doubts about whether PM+ can help and feels that some of the adversity is created by her mother. The client wants you to help her mother too. The client practised using the Stress Management strategy at home and saw a benefit.

Scenario 2

INFORMATION FOR THE CLIENT

You are 38 years old and living as a refugee. You are not allowed to work. You depend on the local religious organizations for support and daily meals. You are living with friends but their behaviour of constantly drinking and taking drugs is becoming unbearable. You feel trapped and unable to know what else to do. On most days you feel angry and frustrated. You notice you are arguing with people more than ever. Even though you have been told that this programme might help you, you are not certain that it will make much difference. You feel that you need money more than talk. You have had thoughts about ending your life – sometimes you think this might be the only way out. But you have never tried before and you do not have plans to end your life any time soon. You still hope that one day you might be reunited with your partner and three children, but right now that seems hopeless.

INFORMATION FOR THE HELPER

The main problems identified at pre-assessment were fighting with people, feeling angry and trapped, not working, having no money and nowhere to go.

Information from Session 1. The client engaged well with Session 1 in theory but thinks that the problems are too big and too many for him to be helped. He struggled with stress management strategies at home but engages well in the session.

Annex 9

Actions following assessment for suicidal thoughts

Table 12 will help trainees understand how they should respond when assessing a client's thoughts on suicide (for use in Module 4, Lesson 5.2). It is a slightly more detailed version than the corresponding section in the PM+ intervention manual.

If helpers are ever concerned about the welfare of a client, they should always contact their supervisor.

Question 1: "In the past month have you had serious thoughts or plans to end your life?"

Question 2: "What actions have you taken to end your life?"

Question 3: "Do you plan to end your life in the next two weeks?

TABLE 12

Actions following assessment for suicidal thoughts

Answer to question	PM+ helper response
1. Client answers NO to question 1.	• You do not have to do anything.
2. Client answers YES to question 1, but NONE to question 2 and NO to question 3.	• Tell your supervisor. • Monitor the client's thoughts in every session (ask them if they are continuing to have these thoughts and whether they want to act on them).
3. Client answers YES to question 1, but NO to question 3 and reports that they have previously tried to end their life or they have taken an action, despite not having a plan.	• Contact your supervisor as soon as possible after the session to alert them. • Monitor your client's thoughts in every session (ask them if they are continuing to have these thoughts and whether they want to act on them).
4. Client answers YES to question 1 and YES to question 3 or the person is extremely agitated, violent, distressed or not communicating.	Your client has, or may have, a plan to end their life and is considered at imminent risk of suicide. Respond immediately: • always contact your supervisor; • create a secure and supportive environment; • remove all means of self-harm if possible; • do not leave the person alone – have carers or staff stay with the person **at all times**; • if possible, offer a separate, quiet room while waiting for your supervisor and any further help; • attend to the client's mental state and emotional distress with basic helping skills.

Annex 10

A scenario for small group discussions on *What is adversity?*

(FOR USE AS A HANDOUT IN MODULE 5, LESSON 3.1)

The scenario

The client is a 63-year-old man whose wife recently died. He feels lonely every day and says that he misses her. He has lost his appetite and no longer wants to do the things he used to, such as going for walks or gardening, because these activities remind him too much of his wife. He has stopped socializing and prefers to stay at home on his own. He says he feels very sad and that his mind is often full of thoughts of her.

An example of an individualized response

Thank you for sharing with me how hard things have been for you since your wife died. I would like to spend a few moments talking to you about this sadness and how PM+ might help you find new ways to cope with your grief.

When people face strong emotions, they can respond in many ways. What you have described to me – about not being in the mood to eat or to be active with things like walking and gardening – shows me how much you have been feeling your wife's death. Your emotions give me important information. They tell me just how much you cared for your wife as well as how lonely you feel without her. At the same time, when you tell me how closing down like this is not normal for you, that signals that we need to try some new things that might help you feel more able to face the world – including being around other people. You see, when our emotions are this strong they stop us doing what we would normally do. We might avoid social situations or not look after ourselves. It is not a sign of weakness, but it tells us just how big a change this has been for you and that you are not sure how to manage this change.

I would like to encourage you to think that it is not hopeless. Many people go through these sorts of emotions when they grieve. There are ways I can help you manage some of these problems. I also want to encourage you because we can find ways to help you without needing you to forget your wife. You have been brave to share these problems with me already, so I think you would be strong enough to go through this PM+ programme as well. I think this intervention could help you feel more able to cope with your wife's death.

Annex 11

Demonstration role-play on how to deliver *Understanding adversity*

(FOR USE IN MODULE 5, LESSON 3.2.). NOTE: PLEASE CULTURALLY ADAPT THESE BEFORE USE.

[ ] is a 40-year-old woman. She is married and has three young children, all under 12 years of age.

She has been unemployed for the past two years. The school where she worked was closed down when severe community violence made it unsafe for children and teachers to attend. It has not been reopened and she has not found another job.

When the violence was at its worst, [ ] witnessed many horrible things, including assaults and rapes. She was raped herself by a stranger who was intoxicated at the time. But she has not told anyone this before. She does not disclose this in the assessment.

In the few months following the violence, [ ] was very anxious about her and her family's safety. Like many other women, she stayed in her home most days and only left when she really needed to. Initially she experienced a lot of symptoms of stress and anxiety but understood this was normal. These symptoms improved a little when things began to settle in the community. However she is still afraid of going back to the village where the school is. This was the place where she was exposed to the worst of the violence. She understands that this avoidance is unreasonable.

[ ]'s unemployment is taking its toll on her family as well as on her emotional well-being. She is beginning to show symptoms of depression – such as low mood, extreme tiredness and feelings of hopelessness about her future. She is also beginning to isolate herself again.

Information from the assessment

Behaviour in the assessment:

- mood is flat;
- very slow to give responses;
- problems concentrating;
- teary when talking about her problems;
- poor eye contact, looks down at her hands frequently.

[ ] says the school she worked for is still closed even though it had been two years since the violence. She is still without a job. She said this is beginning to cause significant problems for her and her family. She describes feelings of depression – feelings such as hopelessness and worthlessness, constant low mood, a lack of energy to do things, and a lack of pleasure in doing things she used to enjoy.

Being unemployed for so long is also causing practical problems. For instance, some weeks the family has to skip meals and bills are not paid because there is not enough money from her husband's wage. He is working longer hours and is also beginning to get very frustrated with the situation. He is becoming more and more angry. He has also started drinking more heavily and regularly gets drunk. When he is drunk, he becomes physically violent towards [ ]. This she does not disclose initially.

These practical problems also harm her emotional well-being. She is constantly thinking about not having a job but she no longer feels confident to find one, even if it is a less demanding job. She describes feeling trapped in a cycle of problems, and this is making her feel even more sad and guilty.

Stress symptoms:

- [ ] feels tension and aching throughout her body and feels very drained physically most of the time.
- She has panic symptoms (heart racing, heart pounding, shaking) when she goes into the village near the old school. She also has these symptoms when she notices that her husband is drinking a lot.

Practical problems:

- There are financial problems because [ ] is not earning any money to help support the family.
- Her youngest child is sleeping in the same bed as her and this is making her husband angry.

Inactivity:

- [ ] feels hopeless and depressed – especially because of much her life has changed.
- She is not motivated to get out of bed, will stay in bed most of the day, and says that she is tired because of feeling so worried every time her husband leaves for work.
- She no longer does any housework.
- She does not do anything enjoyable except visit her mother every so often when she feels guilty for not seeing her.

Social supports:

- Her mother is [ ]'s main social support but [ ] finds it difficult to visit her due to anxiety.
- [ ] no longer engages in the community.
- She is very suspicious that others in the community look down on her or will pity her.

Annex 12

Scenario for demonstrating how to manage difficult problems

(FOR USE IN MODULE 7, LESSON 3.1)

This scenario can be read aloud. Trainees do not need individual copies.

[……..] is a 40-year-old woman who feels stressed about her life situation. Her only child was badly injured in a car accident one year ago. He suffered a significant brain injury and is now partly paralysed. Her husband does little of the caring because he is at work or prefers to go out with his friends drinking. Her mother shares the role of caring for the son when [……..] needs to go out to work. She is able to find work relatively easily but is often away from home for most of the day. She talks about missing her son, saying “He is not the same person anymore”.

She feels sad occasionally and will sometimes withdraw from enjoyable activities because she feels guilty about leaving her son. But it is her stress that causes the most bother for her. She feels tense in her body all the time, is unable to sleep or concentrate, and she finds that she often makes mistakes. She has problems with her teeth and is constantly grinding them. The two main problems she identified were feeling stressed and depressed about her son’s condition.

On discussing how to manage her stress better she reveals an additional problem that is adding to her experience of stress: she cannot make her mind up about whether to consent to a new drug that is being trialled at the hospital for her son. She is worried that it might make him worse as it is unknown, although the doctors think it could work.

Annex 13

A scenario for small group discussions on clients who might find Strengthening Social Support a challenging strategy

(FOR USE IN MODULE 9, LESSON 3.1)

A 21-year-old female student shares with you (the helper) that she has feelings of worthlessness and low mood. She is very quiet and reserved in the sessions and you discover that she has very few friends. She does not go out with her peers or do things that would be expected of someone her age. She spends a lot of time studying and staying at home.

When it comes to Strengthening Social Support, she tells you that she does not trust anyone outside of her family since a boyfriend sexually assaulted her and then spread awful rumours about her. This happened three years ago. After the event, no one wanted to be her friend and they did not believe that she was sexually assaulted. Only her family believed and supported her. She tells you that she cannot trust others with personal information any longer.

You discuss with her how it might be good to begin to spend time with her peers and gradually learn to trust people again. You both agree that this is possible but that it is not a good idea for her to share personal information with people straight away. You decide to start very gradually. Your client believes she could begin by talking to a girl who sits next to her in class and asking her a question about their upcoming assignment. Therefore, you jointly decide to start with this.

Annex 14

Case scenarios for small group discussions on how imagining helping others can reinforce the PM+ strategies

(FOR USE IN MODULE 10, LESSON 2.3)

Scenario 1

[……..] is a man in his thirties who lives in a very dangerous village with a lot of violence. His wife was killed six months ago and he is now left to care for his three children on his own. His mother has recently moved in with him because he is refusing to leave the house and is not caring well for his children. He no longer sees his friends. Most days he does not get out of bed and has lost a lot of weight from not eating regularly. He feels very lonely. But he does not know how to go about helping himself.

Scenario 2

[……..] is a 30-year-old woman, struggling with an ongoing fight with her husband about sharing housework. She is fearful of the conflict getting worse and causing problems in her marriage. She complains of pain throughout her body and that she cannot sleep. She says she cannot stop thinking about the problem and she does not know what to do.

Scenario 3

[……..], a woman in her fifties, was attacked by some young men when she was coming home from shopping. She feels very frightened that she might be attacked again and for the last month has avoided going out of her house. She is also starting to isolate herself and not see her friends.

Scenario 4

[……..] is a young woman whose husband continues to be affected by his experience of imprisonment, even though it was some years ago. Her husband's mood has been getting worse. He feels sad most days and finds it difficult to go to work. This is causing stress for [……..] and she has noticed that she no longer wants to spend time with her husband or her friends. She does not enjoy the things she used to, such as knitting and going for walks. She feels very hopeless about her and her husband's situation and does not know what to do to improve it.

Annex 15

Scenarios role-plays on looking to the future

(FOR USE IN MODULE 10, LESSON 2.4)

Scenario 1

A 43-year-old man has attended all sessions regularly. He found managing stress very helpful for reducing his headaches and tension. His main problem was unemployment and paying the rent. He has made some changes that have helped him obtain casual work; however, this did not solve his problem completely. There are still times when he is unable to pay the rent. He has begun attending community events more regularly and has found one person he trusts to talk with about his problems. His mood has improved, although there are times when he still feels very stressed. This usually happens when he is unable to secure work or pay the rent on time.

Scenario 2

A 37-year-old woman was initially reluctant to attend PM+ sessions. She said she already knew about managing stress and this was something that had not improved her mood in the past. However, after Session 2 she became more enthusiastic about PM+. She managed to successfully address her problem with her neighbour, which had been causing her to avoid leaving her house. She noticed her mood improve after this. She improved her activity through Get going, Keep doing and was slowly seeing her friends more often. She had some difficulties trusting others, but wanted to start talking to one friend about personal matters in the future.

Annex 16
Supervision forms

(FOR USE IN MODULE 12)

This annex gives an example of a completed supervision form for a client who is progressing well with PM+ (Table 13). There is also an empty form that you can photocopy and distribute to your supervisees (Table 14).

Supervisors should give their supervisees a regular supply of blank forms so that they can complete a form for each client in order to monitor the client's progression and their confidence in delivering PM+ before each supervision session. This form can be used as the base for any discussions in supervision sessions. If needed, you can adapt these forms to reflect the PM+ helpers' culture and language.

TABLE 13

An example of a completed supervision form

	Helper's notes (Confidential - do not include the client's name)
Client's gender and approximate age	*Man in his mid-forties .*
Brief description of background (e.g. family, living situation)	*Married, 4 children at school. Works full-time at local factory.* *Anxiety attack 6 months ago. He thought he was having a heart attack but doctors say he is healthy and there is nothing medically wrong with him.*
Brief description of main problems	*Anxious all the time, worries about having another attack, especially in front of colleagues at work.* *He restricts his activities, such as exercise, to prevent having another attack.* *He is having difficulties in his marriage – a lot of arguments.*

	Helper's notes (Confidential - do not include the client's name)
Main emotional health symptoms (e.g. how did the client present in the last session, and what positive or negative emotions did they report?)	*Fear of having another attack.* *Has a lot of physical symptoms of anxiety – racing heart, dizziness, tiredness.* *Anger outbursts at wife and also children.* *Does not go out much anymore or do fun things.*
Which PM+ sessions have you completed with this client so far?	*Session 1 and Session 2.*
What PM+ strategies have you completed with the client?	*Education about PM+ and adversity.* *Managing Stress.* *Managing Problems – having arguments with wife; he came up with four potential solutions. He then decided to tell his wife that every time he begins to feel angry he will go into his bedroom and pray and then talk to her when he is feeling more in control.*
What questions or difficulties do you have with this client that you would like help with (e.g. about a particular problem, a PM+ strategy, a practical issue or a personal issue)?	*He often finds it difficult to talk about his feelings in the session. He says that he feels weak because he often feels scared and gets upset easily.*
What feedback did you get from the supervision group about this problem?	*Normalize the client's feelings. Use the Good and Less Good reasons table to discuss the costs and benefits of talking about emotions.*
Given this information, what will you plan to do differently with this client?	*Praise his openness when he does talk about emotions in session. I will not push the client to talk about emotions when he does not feel comfortable to do so.*
What other things did you learn from this supervision session (e.g. what advice was given to other PM+ helpers that you might find helpful)?	*Do not give solutions to clients when completing Managing Problems. They need to come up with their own as they are more likely to implement those.*

TABLE 14

Template supervision form

	Helper's notes (Confidential - do not include the client's name)
Client's gender and approximate age	
Brief description of background (e.g. family, living situation)	
Brief description of main problems	
Main emotional health symptoms (e.g. how did the client present in the last session, and what positive or negative emotions did they report?)	
Which PM+ sessions have you completed with this client so far?	
What PM+ strategies have you completed with the client?	
What questions or difficulties do you have with this client that you would like help with (e.g. about a particular problem, a PM+ strategy, a practical issue or a personal issue)?	
What feedback did you get from the supervision group about this problem?	
Given this information, what will you plan to do differently with this client?	
What other things did you learn from this supervision session (e.g. what advice was given to other PM+ helpers that you might find helpful)?	

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